

County of Monterey EMS System Policy



Policy Number: 1020

Effective Date:

Review Date:

EMS ADVISORY COMMITTEES

I. PURPOSE

To define the structure and roles of the EMS committees that advise the Board of Supervisors, EMS Director, and EMS Medical Director.

II. POLICY

- A. Advisory committees, composed of EMS system constituents, shall convene to review EMS system matters relevant to their scope of responsibility and recommend actions to the EMS Director and EMS Medical Director concerning matters of policy, procedure, and protocol.
- B. The EMS Agency Medical Director, as mandated by California Health and Safety Code, Section 1797.220, provides medical control and assures medical accountability throughout the planning, implementation and evaluation of the EMS System. The EMS Agency Medical Director retains the final decision through his/her medical authority for the EMS system.
- C. The EMS Director, as mandated by the Monterey County Code of Ordinances, Section 15.40.030, shall be responsible for overseeing the EMS Agency as it serves as the lead agency for the EMS system in the County and shall be responsible for coordinating all system participants in the EMS area. The EMS Agency shall plan, coordinate, monitor and evaluate the implementation of the EMS system.

III. EMERGENCY MEDICAL CARE COMMITTEE

- A. The County of Monterey Board of Supervisors establishes bylaws for the Emergency Medical Care Committee (EMCC). The bylaws formally establish the purpose, membership, structure, and rules for the EMCC. These bylaws, as amended from time to time, are incorporated here by reference.

IV. ADDITIONAL COMMITTEES, TASK FORCES, AND WORKING GROUPS

- A. The EMS Director may appoint ad hoc task forces, quality assurance committees, or working groups as needed.
- B. The EMS Director shall determine the membership and terms of these task forces, committees, and working groups.

V. OPERATIONS WORKING GROUP

- A. Purpose: The Operations Working Group (OPS) shall serve as an advisory group to the EMS Agency's Director on operational issues involving prehospital and emergency medical services.
- B. Appointment Process: Appointed by the EMS Director.
- C. Term of Office: Two years, commencing on the first of July, unless the member is replaced. The terms of the first six members listed below shall expire on odd-numbered years, the terms of the balance of the members listed below shall expire on even-numbered years. With the members' assent, the EMS Director may automatically reappoint members to a subsequent term.
- D. Alternates: Members wishing to have an alternate may do so by submitting a written request to the Chair, giving the name of the proposed alternate member. The proposed alternate must belong to and represent the same group, agency, or entity represented by the member. The EMS Director may appoint the proposed alternate member as appropriate.
- E. Officers: The Chair and Vice-Chair are elected annually by the members. The term of the Chair and Vice-Chair is for one year.
- F. Meeting Frequency: OPS generally meets every other month or by request of the Chair or the EMS Director.
- G. Location: As set by agenda.
- H. Membership:
 - 1. One fire chief representative from an incorporated city within Monterey County.
 - 2. One fire chief representative from a fire protection district within Monterey County.
 - 3. One representative from a law enforcement agency within Monterey County.
 - 4. One Monterey County Base Hospital Coordinator.
 - 5. One representative from the County's primary ambulance contractor.
 - 6. One representative from a fire-based ambulance provider.
 - 7. One representative from the County of Monterey Emergency Communications Department.
 - 8. One representative from an EMS aircraft provider.
 - 9. One field (EMT or paramedic) representative from the contracted ambulance provider.
 - 10. One field (EMT or paramedic) representative from a fire-based ambulance provider.
 - 11. EMS Agency Medical Director (ex-officio, non-voting member).

VI. CLINICAL CARE COMMITTEE

- A. Purpose: The Clinical Care Committee (CCC) shall serve as an advisory group to the EMS Agency's Director and Medical Director on medical control and other medical issues.
- B. Appointment Process: Appointed by the EMS Director.
- C. Term of Office: Two years, commencing on the first of July, unless the member is replaced. The terms of the first seven members listed below shall expire on odd-numbered years, the terms of the balance of the members listed below shall expire on even-numbered years. With the members' assent, the EMS Director may automatically reappoint members to a subsequent term.
- D. Alternates: Members wishing to have an alternate may do so by submitting a written request to the Chair, giving the name of the proposed alternate member. The proposed alternate must belong to and represent the same group, agency, or entity represented by the member. The EMS Director may appoint the proposed alternate member as appropriate.
- E. Officers: The Chair of the CCC is the EMS Agency Medical Director.
- F. Meeting Frequency: The CCC generally meets every other month or by request of the Chair or the EMS Director.
- G. Location: As set by agenda.
- H. Membership:
 - 1. One emergency-department physician from Community Hospital of the Monterey Peninsula.
 - 2. One emergency-department physician from Mee Memorial Hospital.
 - 3. One emergency-department physician from Natividad.
 - 4. One emergency-department physician from Salinas Valley Health.
 - 5. The Registered Nurse who is designated as the Base Hospital Coordinator from Community Hospital of the Monterey Peninsula.
 - 6. The Registered Nurse who is designated as the Receiving Hospital Coordinator from Mee Memorial Hospital.
 - 7. The Registered Nurse who is designated as the Base Hospital Coordinator from Natividad.
 - 8. The Registered Nurse who is designated as the Base Hospital Coordinator from Salinas Valley Health.
 - 9. One paramedic representing field staff of the contracted ambulance provider.
 - 10. One paramedic representing field staff of paramedic service providers other than the contracted ambulance provider.
 - 11. One Monterey County-certified Emergency Medical Technician, representing the first-responder agencies.
 - 12. One representative from the County's primary ambulance contractor.

13. One representative from the authorized air ambulance providers.
14. One representative from a law-enforcement agency that provides prehospital emergency medical services, nominated by the Monterey County Chief Law Enforcement Officers' Association.

VII. CLINICAL QUALITY IMPROVEMENT COMMITTEES

- A. Division 9 (Prehospital Emergency Medical Services) of Title 22 (Social Security) of the California Code of Regulations authorizes and requires the local EMS agency to develop clinical standards and quality improvement processes for the EMS system including specialty care systems related to trauma, stroke, and ST-elevation myocardial infarction (STEMI). In furtherance of this, the EMS Director has established the following Clinical Quality Improvement Committees. Meeting attendance is only open to committee members, appointed by the EMS Director, who must sign a confidentiality agreement and guests, approved by the EMS Director, who must sign a confidentiality agreement. Clinical Quality Improvement Committee meetings are not open to the public. Persons who violate the provisions of the confidentiality agreement may be removed from the respective Clinical Quality Improvement Committee by the EMS Director.
 1. Continuous Quality Improvement Technical Advisory Group (CQI TAG).
 2. ST-Elevation Myocardial Infarction (STEMI) Quality Improvement Committee.
 3. Stroke Quality Improvement Committee.
 4. Trauma Evaluation and Quality Improvement Committee (TEQIC).
- B. The EMS Director appointed the standing Clinical Quality Improvement Committees to advise the EMS Director, the EMS Agency Medical Director, and/or the other advisory committees on subjects related to specialty care programs and prehospital clinical care. With the concurrence of the EMS Director, the Chair of a Clinical Quality Improvement Committee may appoint ad hoc working groups. Such working groups shall address topics within the scope of that committee.
- C. Continuous Quality Improvement Technical Advisory Group (CQI TAG)
 1. Purpose: The CQI TAG conducts reviews of cases and assesses the operations and quality of clinical care provided in the prehospital setting, emphasizing the period between the reception of the call at the primary public safety answering point (PSAP), through the first responder and ambulance provider to the emergency department. Additionally, the CQI TAG receives reports from the TEQIC, Stroke, and STEMI QI Committees and reports these to the Clinical Care Committee (CCC).
 2. Members Appointed by: EMS Director, in consultation with the EMS Agency Medical Director and Staff.
 3. Terms of Office: Two years, commencing on the first of July, unless the member is replaced.
 4. Meeting Frequency: The Committee meets no less than quarterly or by request of the Chair, EMS Medical Director, or EMS Director.

5. Committee reports to: Clinical Care Committee.
6. Location: As set by agenda.
7. Quorum: No quorum requirement.
8. Membership:
 - a. One (1) representative from the County of Monterey EOA ambulance services provider.
 - b. One (1) representative from the Monterey County of Monterey EOA ambulance services provider's dispatch center.
 - c. One (1) Representative from the County of Monterey Emergency Communications Center.
 - d. One (1) Base Hospital Coordinator from Natividad.
 - e. One (1) Base Hospital Coordinator from Salinas Valley Health.
 - f. One (1) Base Hospital Coordinator from Community Hospital of the Monterey Peninsula.
 - g. One (1) ED Physician/Nurse Manager from Mee Memorial Hospital.
 - h. One (1) representative from each of the ALS Fire Departments operating in Monterey County.
 - i. One (1) representative from each of the BLS Fire Departments operating in Monterey County.
 - j. One (1) representative from each of air ambulance provider operating in Monterey County.
 - k. One (1) representative from the California Highway Patrol Air Operations in Monterey County.
 - l. One (1) representative from a paramedic training program located in Monterey County.

D. ST-Elevation Myocardial Infarction (STEMI) Quality Improvement Committee

1. Purpose: The STEMI Quality Improvement Committee reviews STEMI system care and advises the Monterey County EMS Agency on STEMI system policy, organization, training, and equipment.
2. Members Appointed by: EMS Director, in consultation with the EMS Agency Medical Director and Staff.
3. Terms of Office: Two years, commencing on the first of July, unless the member is replaced.
4. Meeting Frequency: The Committee generally meets quarterly or by request of the Chair, EMS Medical Director, or EMS Director.
5. Committee reports to: The Continuous Quality Improvement Technical Advisory Group (CQI TAG).

6. Location: As set by agenda.
7. Quorum: No quorum requirement.
8. Membership:
 - a. One (1) STEMI/ Chest Pain Coordinator from Salinas Valley Health.
 - b. One (1) Interventional Cardiologist/Cardiac Cath Lab representative from Salinas Valley Health.
 - c. One (1) STEMI/Chest Pain Coordinator from Community Hospital of the Monterey Peninsula.
 - d. One (1) Interventional Cardiologist/Cardiac Cath Lab representative from Community Hospital of the Monterey Peninsula.
 - e. One (1) representative from Natividad.
 - f. One (1) representative from Mee Memorial Hospital.
 - g. One (1) representative from the County of Monterey EOA ambulance services provider.
 - h. One (1) representative from each of the ALS Fire Departments operating in Monterey County.
 - i. One (1) representative for all BLS Fire Departments operating in Monterey County.
 - j. One (1) representative from each Air Ambulance provider operating in Monterey County.
 - k. One (1) representative from a San Benito County EMS provider agency or hospital.

E. Stroke Quality Improvement Committee

1. Purpose: The Stroke Quality Improvement Committee reviews stroke system care and advises the Monterey County EMS Agency on stroke system policy, organization, training, and equipment.
2. Members Appointed by: EMS Director, in consultation with the EMS Agency Medical Director and Staff.
3. Terms of Office: Two years, commencing on the first of July, unless the member is replaced.
4. Meeting Frequency: The Committee generally meets quarterly or by request of the Chair, EMS Medical Director, or EMS Director.
5. Committee reports to: The Continuous Quality Improvement Technical Advisory Group (CQI TAG).
6. Location: As set by agenda.
7. Quorum: No quorum requirement.
8. Membership:

- a. One (1) Stroke Coordinator/ Designee from Salinas Valley Health.
- b. One (1) Stroke Coordinator/ Designee from Community Hospital of the Monterey Peninsula.
- c. One (1) base hospital coordinator from Salinas Valley Health.
- d. One (1) base hospital coordinator from Community Hospital of the Monterey Peninsula.
- e. One (1) representative from Hazel Hawkins Memorial Hospital.
- f. One (1) representative from Natividad.
- g. One (1) representative from Mee Memorial Hospital.
- h. One (1) representative from the County of Monterey EOA ambulance services provider.
- i. One (1) representative from each ALS Fire Department operating in Monterey County.
- j. One (1) representative for all BLS Fire Departments operating in Monterey County.
- k. One (1) representative from each air Ambulance provider operating in Monterey County.
- l. One (1) Clinical and Education Specialist from Santa Cruz and San Benito County EOA ALS provider.

F. Trauma Evaluation and Quality Improvement Committee (TEQIC):

- 1. Purpose: The Trauma Evaluation and Quality Improvement Committee (TEQIC) reviews trauma system care and advises the County of Monterey EMS Agency on trauma system policy, organization, training, and equipment. Its goals are the evaluation and administration of the trauma system including system vulnerabilities, the development of policy and/or approaches to related issues such as major trauma and burn-related prehospital care, injury surveillance, trauma transfers, repatriation, and long-term outcomes.
- 2. Members Appointed by: EMS Director, in consultation with the EMS Agency Medical Director and Staff.
- 3. Terms of Office: Two years, commencing on the first of July.
- 4. Meeting Frequency: The Committee generally meets quarterly or by request of the Chair, the EMS Medical Director, or EMS Director.
- 5. Committee reports to: The Continuous Quality Improvement Technical Advisory Group (CQI TAG).
- 6. Location: As set by agenda.
- 7. Quorum: No quorum requirement.
- 8. Membership:

- a. One (1) representative from the County of Monterey EOA ambulance services provider's dispatch center.
- b. One (1) representative from the County of Monterey EOA ambulance services provider field operations/quality improvement.
- c. One (1) representative from each ALS Fire Department operating in Monterey County.
- d. One (1) representative for all BLS Fire Departments operating in Monterey County.
- e. One (1) representative from the County of Monterey Coroner's Office.
- f. One (1) representative from Monterey County Law Enforcement.
- g. One (1) representative from CALSTAR.
- h. One (1) representative from Mercy Air.
- i. One (1) representative from the Natividad Transfer Center.
- j. One (1) surgeon or ED physician representing Salinas Valley Health.
- k. One (1) Surgeon or ED physician representing Community Hospital of the Monterey Peninsula.
- l. One ED Physician or nurse representative from George L. Mee Memorial Hospital.
- m. Trauma Program Manager from Natividad.
- n. Trauma Program Medical Director from Natividad.
- o. One (1) Emergency Department physician from Natividad.
- p. Base hospital coordinator from Natividad.
- q. One (1) representative from the San Benito County EMS Agency.
- r. One (1) representative from Hazel Hawkins Memorial Hospital.
- s. One (1) Clinical and Education Specialist from the EOA provider in Santa Cruz and San Benito Counties.
- t. One (1) medical director for the Santa Cruz and San Benito County EMS Agencies.

END OF POLICY

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TRAUMA SYSTEM

I. PURPOSE

- A. To identify the role and responsibilities of the County of Monterey EMS Agency as they relate to the trauma care system.
- B. To ensure that trauma patients are treated at an appropriate facility, regardless of geopolitical boundaries, and to facilitate trauma care coordination with neighboring systems.
- C. To establish limitations on direct-to-consumer advertising by trauma centers that is intended to influence patient flow.
- D. To promote patient flow in a manner that meets patients' clinical needs while considering the economic impact of patient flow decisions.
- E. To mitigate the expense to the County of Monterey EMS Agency for the implementation and management of the trauma system.
- F. To establish the service areas for trauma patients in the County of Monterey.

II. TRAUMA CARE SYSTEM ORGANIZATION AND MANAGEMENT

- A. **County of Monterey EMS Agency Role and Responsibilities** - As the lead agency for the County of Monterey Emergency Medical Services (EMS) system, the County of Monterey EMS Agency is responsible for planning, implementing, and managing the trauma care system. These responsibilities include:
 - 1. Assessing needs and resource requirements;
 - 2. Developing the system design, including the number of Trauma Center(s), and determining patient flow patterns;
 - 3. Assigning roles to system participants, including designation of the Trauma Center(s);
 - 4. Working with the designated Trauma Center(s) and other system participants, and with neighboring EMS systems, on outreach and mutual aid services;
 - 5. Development of a trauma data system, including a trauma registry at the Trauma Center, trauma data collection from non-Trauma Centers, and prehospital data collection;
 - 6. Monitoring the system to determine compliance with appropriate state statutes and regulations, LEMSA policies, procedures, and contracts, and taking corrective action as needed;
 - 7. Public information and education;

8. Overseeing quality assurance processes;
9. Reviewing clinical cases and unusual occurrences involving the EMS trauma system; and
10. Evaluating the impact of the system and revising the system design as needed.

B. Designated Trauma Center(s) Requirements

1. To obtain and maintain designation as a Level II Trauma Center, a hospital must comply with all standards in the California Code of Regulations Title 22, Division 9, Chapter 6.1, Article 3. Trauma Center Requirements §100137.01. Level I and Level II Trauma Centers, the standards contained within the current version of the American College of Surgeons' Resources for Optimal Care of the Injured Patient, and meet the requirements specified in the County of Monterey EMS System Policy 5060 (Trauma Center Standards).

III. Trauma Service Area

- A. The Trauma Service Area is the entirety of the County of Monterey.

IV. Trauma Marketing and Advertising

- A. The following shall guide the approval of the inclusion of the terms "trauma" and "Trauma Center" in marketing and advertising for Trauma Centers within the County of Monterey:
 1. Shall provide accurate information;
 2. Shall not include false claims;
 3. Shall not be critical of other providers; and
 4. Shall not include financial inducements to any provider or third parties.
- B. Titles may include the word "trauma" in staff position titles.
- C. The request to advertise and/or incorporate the term "Trauma Center" in promotional materials shall be made in writing to the County of Monterey EMS Agency. The County of Monterey EMS Agency shall respond in writing within thirty (30) days of receipt of the written request. No use of the term "trauma", "Trauma Center", or similar terminology may be used without authorization by the EMS Agency.

V. Trauma Center Coordination with Health Systems

- A. The County of Monterey EMS Agency field triage policy does not consider the patient's insurance status in determining the destination of patients who meet trauma triage criteria. Since patients who require trauma center services may not require this level of service once they have been stabilized, the Trauma Center should consider the need to return patients who are insured by HMOs and other managed health care organizations to their payer's network at a medically-appropriate time.

- B. The Trauma Center shall make a good faith effort to negotiate agreements with HMOs and other managed health care organizations regarding payment, repatriation of patients, and other related factors.
- C. The Trauma Center shall cooperate with HMOs and other managed health care organizations in their efforts to identify the appropriate level of care for their members, including non-emergency treatment and the location of services provided.
- D. Nothing in this policy is intended to suggest that the Trauma Center should limit the treatment provided to members of HMOs and other managed health care organizations in violation of the Emergency Medical Treatment and Active Labor Act (EMTALA), any other state or federal law, or good medical practice.

VI. Trauma System Fees

- A. Trauma Center Application Fee: A Trauma Center application fee will be established to cover the costs associated with the designation process. These costs may include contract costs for plan development, Requests for Proposal (RFP) development, review of proposals, out-of-area site team costs, and legal reviews and agency costs in excess of the costs associated with the day-to-day trauma system regulation. The Trauma Center application fee will be assessed for hospitals applying for Trauma Center designation. Fees paid that are in excess of actual costs will be returned to applicants.
- B. Trauma Center Designation Fee:
 - 1. The County of Monterey Board of Supervisors will establish a Trauma Center designation fee. This fee covers the cost of monitoring the operation of the trauma care system in compliance with state trauma care systems, regulations, and regional policies. The fee will be based on the time requirements of the trauma medical director, trauma coordinator, and other staff activity dedicated to trauma issues as well as associated overhead and program support costs.
 - 2. The County of Monterey EMS Agency will provide the designated Trauma Center written notice of any increase in the designated fee at least 180 days (6 months) prior to the effective date of the increase with an explanation for the increase and the basis on which it was calculated.
 - 3. If the amount is not agreeable to the designated Trauma Center and resolution of the amount cannot be reached prior to the effective date of the change, or any later date as mutually agreed upon in writing by the parties, then either party may terminate the Agreement without penalty. A written notice of 180 days must be made to the other party to terminate the Agreement. If the Agreement is terminated, the designation fee in existence at the time notice is given will be prorated until termination.

VII. Mutual Aid Coordination with Neighboring Systems

- A. The County of Monterey EMS Agency will coordinate its trauma care system with those in neighboring systems to ensure that patients are transported to the most accessible trauma facility equipped, staffed, and prepared to administer care appropriate to the needs

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of the patient. Written mutual aid agreements will be executed as necessary to ensure coordination with neighboring systems.

1. The Monterey County EMS Agency will coordinate with neighboring LEMSAs to develop and maintain appropriate trauma services to address the comprehensive needs of trauma patients in the region.
- B. When patients from the County of Monterey are transported to a Trauma Center in another EMS system, the County of Monterey EMS Agency will request injury/outcome data from the LEMSA or destination hospital, as needed for database management and EMS system quality assurance review.
- C. When trauma patients from another EMS system are transported to a County of Monterey EMS receiving hospital, the receiving hospital will provide injury/outcome data to the LEMSA from which the patient originated.
- D. County of Monterey-based EMS providers are expected to cooperate with EMS agencies in other counties in data collection and evaluation efforts regarding patients who are served by the County of Monterey EMS System.

END OF POLICY

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POLICY DEFINITIONS

I. PURPOSE

To establish standard terminology and definitions throughout Monterey County EMS Agency policies.

II. POLICY

A. The following terminology and definitions shall apply to all Monterey County EMS Agency Policies.:

1. "12-Lead ECG" means a 12-Lead electrocardiogram.
2. "Accessible Emergency Department" means a facility with the shortest transport time and is usually the closest emergency department. Factors such as traffic congestion, weather, the ability to get around a traffic collision scene, and other conditions could make a geographically further emergency department the more accessible emergency department.
3. "Accreditation" means a Paramedic, EMT, or Emergency Medical Dispatcher who has been oriented to the local EMS System and has been deemed competent by the Local EMS Agency in the use of those additional skills and/or medications in accordance with California Health and Safety Code.
4. "Accusation" means a written statement of charges set forth in ordinary and concise language the acts or omissions with which the respondent is charged.
5. "Acute Coronary Syndrome (ACS)" means a group of clinical symptoms compatible with acute myocardial ischemia.
6. "Adult" means a person who is at least 18 years of age or an emancipated minor as per California law.
7. "Advance Healthcare Directive" means a document indicating an individual's wishes regarding medical care should that individual become incapacitated.
8. "Advanced Life Support" or "ALS" means special services designed to provide definitive prehospital emergency medical care, including, but not limited to, cardiopulmonary resuscitation, cardiac monitoring, cardiac defibrillation, advanced airway management, intravenous therapy, administration of specified drugs and other medicinal preparations, and other specified techniques and procedures administered by authorized personnel under the direct supervision of a base hospital as part of a local EMS system.
9. "Advanced Life Support (ALS) First Responder Agency" means a First Responder Agency that provides Paramedic personnel and ALS equipment to

respond to medical emergencies with the capabilities to provide immediate Advanced Life Support medical care but does not provide ambulance services or other transport services.

10. “Advanced Life Support (ALS) Rescue Aircraft” means a rescue aircraft whose medical flight crew has at a minimum one attendant certified or licensed in advanced life support.
11. “Advanced Life Support (ALS) Transport Agency means a public or private corporation, institution, or agency that has demonstrated the capability to meet all criteria for approval and has a written agreement signed by the EMS Director to provide ALS ambulance services to a designated geographic area.
12. “Agency” means the Monterey County EMS Agency.
13. “ALS ambulance” means a Monterey County designated emergency vehicle staffed by and equipped to provide ALS at the scene of an emergency and/or during transport of (a) patient(s) experiencing a medical emergency to a hospital.
14. “Applicant” or “Applicant Agency” means the First Responder Agency that has applied to provide ALS services.
15. “Authorized Stock” means the lowest level of supply for each described item that should be found on an EMS unit when it is available for service.
16. “Ambulance Provider” means an entity permitted and contracted to provide BLS or ALS ambulances in Monterey County.
17. “ALS Service Provider” means an organization that has an agreement with the Monterey County EMS Agency to provide paramedic-level services within their service area.
18. “Ambulance” means any vehicle that is specially designed, constructed, modified, or equipped for transporting sick, injured, infirm, or otherwise incapacitated persons, capable of supporting BLS or a higher level of care.
19. “Air Ambulance” means any aircraft specially constructed, modified or equipped, and used for the primary purposes of responding to emergency calls and transporting critically ill or injured patients whose medical flight crew has at a minimum two (2) attendants certified or licensed in advanced life support.
20. “Airway adjuncts” means nasopharyngeal airways (NPA) or oral pharyngeal airways (OPA).
21. “Authorized person” means a person who is legally authorized to give consent for another person. Examples include:
 - a. A parent with legal custody of a minor child.
 - b. The legal guardian of a minor.
 - c. An adult relative, with whom a minor lives, and who presents a signed Caregiver’s Authorization
 - d. Affidavit.

- e. An adult who has been authorized in writing by a custodial parent, legal guardian, or qualified adult relative (as above) to consent to medical treatment for a minor.
 - f. The patient's designated agent under a Power of Attorney for Healthcare Decisions.
 - g. The conservator of a person who has been found by a court to lack capacity.
22. "Automated External Defibrillator (AED)" means an external defibrillator capable of cardiac rhythm analysis that will charge and deliver a defibrillation shock either automatically or by user interaction after electronically detecting and assessing ventricular fibrillation or rapid ventricular tachycardia.
23. "Auxiliary Rescue Aircraft" means a rescue aircraft which does not have a medical flight crew, or whose medical flight crew does not meet the minimum requirements established in Section 100284. "Basic Life Support (BLS) Rescue Aircraft" - A rescue aircraft whose medical flight crew has, at a minimum, one attendant certified as an EMT with at least eight (8) hours of hospital clinical training and whose field/clinical experience specified in Section 100074 of Title 22, California Code of Regulations, is in the aero-medical transport of patients.
24. "Base Hospital" means a hospital designated by the Monterey County EMS Agency to provide online medical control of the prehospital EMS system, to cooperate in the Quality Improvement process, and to provide EMS continuing education opportunities for prehospital personnel.
25. "Base Hospital Physician" means a currently licensed physician in California who is assigned to the emergency department of a base hospital and has been trained to issue advice and instructions to EMS personnel consistent with Monterey County policy and protocol.
26. "Basic Life Support" (BLS) means emergency first aid and cardiopulmonary resuscitation procedures to maintain life without invasive techniques until the patient may be transported or until advanced life support is available.
27. "BEFAST Stroke Scale" means A patient assessment tool to identify stroke patients by assessing for the presence of Balance, Eye vision problems, Facial droop, Arm drift, Speech slurred, and Time.
28. "BLS Airway" means Bag valve mask (BVM) ventilation with or without airway adjuncts.
29. "Cardio-Pulmonary Resuscitation (CPR) training" means a course of instruction in CPR equivalent to the Healthcare Provider level or BLS Provider level as established by the American Heart Association and includes both cognitive and skill testing.
30. "Certificate" means one of the following:
- a. A valid EMT certificate issued pursuant to California Health and Safety Code, Division 2.5.

- b. A valid Paramedic license issued pursuant to Health and Safety Code, Division 2.5.
- 31. “Certificate holder” means one of the following:
 - a. An individual that has been issued a valid EMT certificate pursuant to California Health and Safety Code, Division 2.5.
 - b. An individual that has been issued a valid Paramedic license accreditation issued pursuant to Health and Safety Code, Division 2.5.
- 32. “Certified Flight Paramedic” or FP-C means a paramedic educated and trained in critical care transport and flight medicine, holds a current certification as an FP-C by the Board for Critical Care Transport and Flight Medicine, has a current California paramedic license, has Monterey County paramedic accreditation, and is employed by an EMS Aircraft Provider.
- 33. “Certifying entity” means the medical director of the Monterey County EMS Agency or a public safety provider, if that provider has a training program approved pursuant to California Health and Safety Code, Section 1797.109.
- 34. “Code 2” means a non-emergent response with no lights and siren.
- 35. “Code 3” means an emergent response with lights and siren.
- 36. “Consent” means to give permission for medical treatment or transportation. Consent can be expressed or implied.
 - a. Expressed Consent. Verbal, nonverbal, or written communication by a patient that they wish to receive medical care.
- 37. “Continuous Positive Airway Pressure (CPAP)” means a device that transmits an increased airway pressure to patients during oxygen delivery.
- 38. “Controlled Substance” is a medication declared by federal or state law to be illegal for sale or use but may be dispensed under a physician’s prescription. These laws aim to control the danger of addiction, abuse, physical and mental harm, trafficking by illegal means, and the dangers from the actions of those who have used the substances. These medications are grouped by “schedules” and are tightly regulated by the Drug Enforcement Administration (DEA).
- 39. “Controlled Substances Management Plan” is the plan established by an ALS service provider agency to comply with this policy, and state and federal law and regulations pertaining to these substances.
- 40. “Convalescent Transport” means transport by a Monterey County authorized vehicle which is specially designed, constructed, modified, or equipped for transporting patients who require wheelchairs or who cannot travel in an upright, sitting position, and who in addition do not require medical care, assistance, or monitoring during the transport.
- 41. “Critical Care Transport (CCT)” means any transport of a patient by ambulance when the ambulance staff on board includes a physician a registered nurse, or

other medical personnel authorized to provide a level of care that exceeds the paramedic scope of practice.

42. “CQI” means Continuous Quality Improvement.
43. “Critical Care Paramedic” or CCP-C means a paramedic educated and trained in critical care transport in accordance with California Code of Regulations, Title 22, Division 9, Chapter 4, Section 100155(c).
44. “Date of Service” means Date that the call was received at the County 911 Communications Center.
45. “Determination of Death” means the process by which EMS providers, using predetermined criteria, conclude that resuscitative measures would be futile due to patient condition.
46. “Disciplinary Cause or Reason” means only an action that is substantially related to the qualifications, functions, and duties of a paramedic and is considered evidence of a threat to the public health and safety as identified in the California Health and Safety Code, Section 1798.200 (c).
47. “Department Operations Center (DOC)” means Monterey Health Department Operations Center established by the Health Officer to support an incident.
48. “Do Not Resuscitate Order (DNR)” means an order to withhold resuscitative measures. This may include written documentation such as the EMS Prehospital DNR form, POLST form, a Durable Power of Attorney for Health Care Decisions (DPAHCD) form, a physician order written in the patient’s medical record, or a DNR medallion.
49. “Durable Power of Attorney for Health Care Decisions (DPAHCD)” means this document allows individuals to designate an “Attorney-In-Fact” who is authorized to make decisions regarding health care when the individual is incapacitated.
50. “EMD” means a certified Emergency Medical Dispatcher.
51. “Emergency Department” or “Emergency Room” means the area of a licensed general acute care hospital that customarily receives patients in need of emergency medical evaluation and/or care.
52. “Emergency Medical Responder” (EMR) means a person who has successfully completed a Monterey County EMS Agency-approved First Responder course and is trained in BLS.
53. “Emergency Medical Technician” (EMT) means a person who has successfully completed an EMT course which meets the requirements of California Code of Regulations, Title 22, Division 9, Chapter 2, has passed all required tests, and who has been certified by an EMT certifying entity.
54. Emergency Medical Technician-Expanded Scope- An EMT who has completed additional training specified by the Monterey County EMS Agency and works for an EMS service provider who has been accredited to provide EMT-Expanded Scope level of service.

55. “Emergency Trauma Re-Triage” means the rapid identification and movement of patients meeting specific high-acuity criteria to a trauma center for specialized trauma care. Timeliness of evaluation and intervention at the trauma center is critical.
56. “EMS Agency” means the Monterey County Emergency Medical Services Agency.
57. “EMS Aircraft” means any aircraft utilized for the purpose of pre-hospital emergency patient response and transport. EMS Aircraft include Air Ambulances, ALS Rescue Aircraft, BLS Rescue Aircraft and Auxiliary Rescue Aircraft.
58. “EMS Authority” means California EMS Authority.
59. “EMS Communications Center” means the Monterey County EMS Communications Center as designated by Monterey County EMS Agency.
60. “EMS Communications System” means the EMS Communications System which encompasses all of the components and infrastructure used by EMS providers to communicate with each other. That includes radios, phones – including satellite, ReddiNet, and paging and alerting systems.
61. “EMS Duty Officer” means the EMS Agency representative who is on call 24 hours/day, available through the Monterey County EMS Communications Center.
62. “EMS Service Provider” means an organization employing certified EMT-I, certified EMT-II, or licensed paramedic personnel for the delivery of emergency medical care to the sick and injured at the scene of an emergency, during transport, or during interfacility transfer.
63. “EMS System” or “System” means the Monterey County EMS System.
64. “EMS-System Ambulance” means ambulance operating under Monterey County contract and authorized by Monterey County contract and authorized by Monterey County to respond to 9-1-1 emergency calls.
65. “EMT-P employer” or “paramedic employer” means an entity or organization approved by the Monterey County EMS Agency to provide advanced life support (ALS) services.
66. “Emergency Operations Center (EOC)” means the Monterey County Emergency Operations Center established to provide management and support in a disaster.
67. “ePCR” means electronic Patient Care Report or Patient Care Report.
68. “ETCO₂” means end tidal co₂ or capnography.
69. “Event of Concern” means events that do not necessarily breach any policies or procedures but are felt by the individuals involved to be potentially detrimental, should be reported.
70. “Field Training Officer” or “FTO” is an individual designated by a provider agency to conduct pre-accreditation field evaluations for new paramedics to the system, to conduct evaluations for new basic life support (EMT) personnel in the system, to provide both provider-based and county-based training in the field

environment, and to conduct quality improvement activities as determined by their employer.

71. “Health maintenance organization” (HMO) means an organization authorized under the Knox-Keene Health Care Service Plan Act of 1975.
72. “Hemothorax” means an accumulation of blood in the chest cavity.
73. “IAED” means International Academies of Emergency Dispatch.
74. “Immediately” or “immediately available” means:
 - a. Unencumbered by conflicting duties or responsibilities;
 - b. Responding without delay when notified; and
 - c. Being physically available to the specified area of the hospital within the timeframe designated by the STEMI Receiving Center.
75. “Imminent Death” means a condition wherein illness or injuries are of such severity that in the professional opinion of EMS personnel, death will probably occur before the patient arrives at the receiving hospital. This definition does not include any conscious patient regardless of the severity of illness or injury.
76. “Implied Consent” means consent for treatment that is presumed for a patient who is mentally, physically, or emotionally unable to grant consent.
77. “Incident Commander” means the individual that is jurisdictionally responsible for the command of all functions at the field response level.
78. “In-extremis” means the inability to secure the airway.
79. “Intranasal naloxone” means a commercially prepared device containing naloxone hydrochloride intended to deliver the naloxone hydrochloride intranasally.
80. “Intraosseous Infusion” means establishing vascular access through bone marrow.
81. “Intubation attempt” means any time the endotracheal tube (ETT) is placed past the patient’s teeth.
82. “Landing Zone” means a place at or as near as practical to a medical emergency; a transfer point, or a site at or near a medical facility pre-selected and approved by an officer authorized by a public safety agency.
83. “Law enforcement personnel” means peace officers required to complete the CPR and first aid training described in California Code of Regulations, Title 22, Division 9, Section 100017.
84. “Local EMS Agency” means the agency, department, or office having primary responsibility for administration of emergency medical services in a county and which is designated pursuant to the California Health and Safety Code.
85. “Major Trauma Victim (MTV)” means a patient who meets the criteria for transport to a designated Trauma Center in the Field Triage Decision Algorithm (meets CDC Step 1, 2, or 3 criteria).

86. “Mechanical Circulatory Support (MCS) device” means a collective term denoting both the TAH and VADs.
87. “Medical Care, assistance, or monitoring” means care, assistance, or monitoring that requires any medical knowledge or skill.
88. “Medical Director” means the Monterey County EMS Agency Medical Director.
89. “Medical Health Operational Area Coordinator (MHOAC)” means the person designated by the Board of Supervisors to fill the role of MHOAC in a disaster.
90. “MPDS” means Medical Priority Dispatch Systems, an authorized, systematic caller interrogation system that provides coding, medical triage, and post-dispatch and pre-arrival life support instructions.
91. “M.T.P.” means Major Trauma Patient as defined in the Field Triage Criteria policy.
92. “Non-contract ambulance” means an EMT-I ambulance, an EMT-P ambulance, or a Critical Care Transport unit operating under Monterey County license but not authorized by Monterey County to respond to 9-1-1 emergency calls.
93. “On-call” means agreeing to be available to respond to the trauma center in order to provide a defined service, as defined by Monterey County EMS Agency Policy #5090 (Trauma Team Availability and Activation).
94. “Operational Area (OA)” means the County of Monterey or subdivision of the County of Monterey.
95. “Operational Area Lead Agency” means Agency having primary jurisdictional or statutory responsibility.
96. “Operational Support Agency” means Agency having secondary jurisdictional or statutory responsibility.
97. “Organ Donor” means an individual whose body or part is the subject of an anatomical gift.
98. “P1” means a Priority 1 emergency response requiring emergency lights and siren.
99. “P2” means a Priority 2 emergency response requiring emergency lights and siren.
100. “P3” means a Priority 3 urgent response that does not require lights and siren.
101. “Palliative Measures” means measures intended to make the patient comfortable, or to provide care that is not resuscitative in nature. Examples include: treating airway obstruction, pain management, bleeding control, and other care appropriate to the patient’s condition and in accordance with Monterey County EMS Policy and Protocol.
102. “Paramedic” or “EMT-P” means an individual licensed by the EMS Authority to provide advanced life support in accordance with the standards prescribed by California Code of Regulations, Title 22, Division 9, Chapter 4.

103. “Paramedic Intern” or “Intern” means a paramedic student that has completed didactic and clinical portions of paramedic training and is eligible for, and who is in the process of completing, the field internship portion of paramedic training.
104. “Paramedic Preceptor” or “Preceptor” means a licensed and accredited paramedic that is authorized by the Monterey County EMS Agency to supervise and instruct paramedic students during their field internship.
105. “Paramedic Service Provider” means an organization authorized by the County of Monterey EMS Agency to provide advanced life support services utilizing paramedics.
106. “Partial Spinal Motion Restriction” means the use of a cervical collar only.
107. “Patient” means any person that:
 - a. Has experienced an event that could cause illness or injury; or
 - b. Is in a circumstance or situation that creates a suspicion of illness or injury; or
 - c. Makes a request for assistance, examination, or treatment; or
 - d. Has a chief complaint; or
 - e. Has signs or symptoms of illness or injury; or
 - f. Has spoken of or acted toward suicide; or
 - g. Is dead.
108. “Pediatric Patient” means a patient with an estimated weight < 40 kg or age < 9 years.
109. “Pediatric Trauma Patient” means a person less than 15 years of age who meets the criteria for transport to a designated trauma center in the Field Triage Decision Algorithm (Meets step 1, 2, or 3 criteria).
110. “Physician Orders for Life-Sustaining Treatment (POLST)” means a request regarding resuscitative measures that directs a healthcare provider regarding resuscitative measures. This form takes an individual’s wishes regarding life-sustaining treatment and converts them into a medical order. It addresses a range of life-sustaining interventions as well as the individual’s preferred intensity of treatment for each intervention.
111. “Pneumothorax” means accumulation of air in the pleural space.
112. “POST” means Commission on Peace Officer Standards and Training.
113. “Prehospital Emergency Medical Care Certificate” means any valid certificate to provide prehospital emergency medical care issued pursuant to California Health and Safety Code, Division 2.5.
114. “Promptly” or “Promptly Available” means responding without delay when notified and requested to respond to the hospital; and being physically available to the specified area of the trauma center within a period of time that is medically prudent and in accordance with Monterey County EMS Agency Policy #5090.

115. “Public Access AED” means an AED which is accessible to the public and designed to be operated by individuals who do not have any specific medical training or expertise and intended for the purpose of assessing the need for and delivering, if necessary, a defibrillation in an unconscious person who has no signs of circulation. This definition does not apply to individuals who have been prescribed an AED by a physician for use on a specifically identified individual.
116. “Public Safety AED Service Provider (includes EMT AED Service Providers)” means an EMS Service Provider agency or organization that is responsible for, and is approved to operate, an AED.
117. “PSAP” means Public Safety Answering Point.
118. “Public Safety First Aid (PSFA)” means the recognition of and immediate care for injury or sudden illness, including medical emergencies, by public safety personnel prior to the availability of medical care by licensed or certified health care professionals.
119. “Public Safety Personnel” means a member of a fire department or district, or law enforcement officer whose primary duties are not primarily clerical or administrative.
120. “Qualified Flight Paramedic” is a paramedic with a current California paramedic license and Monterey County accreditation, have at least three (3) years of critical care experience, is employed by a Qualified Flight Program, has completed the Qualified Flight Program’s initial training and orientation, and is either an FP-C or FP in training, or a CCP-C or CC in training with additional education in flight and altitude physiology.
121. “Qualified Flight Program” means an aeromedical transport program that has met the requirements to participate in the Paramedic Scope of Practice-EMS Aircraft Based by meeting CAMTS Emergency Critical Care Accreditation or equivalent and is approved by the Monterey County EMS Agency.
122. “Qualified Specialist” or “Qualified Surgical Specialist” or “Qualified Non-Surgical Specialist” means a physician licensed in California who is board certified in a specialty by the American Board of Medical Specialties, the Advisory Board for Osteopathic Specialties, a Canadian board or other appropriate foreign specialty board as determined by the American Board of Medical Specialties for that specialty. Upon request of the Chief of Trauma of a trauma center, a non-board-certified physician may be recognized as a “qualified specialist” by the Monterey County EMS Agency Medical Director upon substantiation of need by a trauma center if:
 - a. The physician can demonstrate to the appropriate hospital body and the hospital is able to document that he/she has met requirements which are equivalent to those of the Accreditation Council for Graduate Medical Education (ACGME) or the Royal College of Physician and Surgeons of Canada;
 - b. The physician can clearly demonstrate to the appropriate hospital body that he/she has substantial education, training, and experience in treating

and managing trauma patients which shall be tracked by the trauma quality improvement program; and

c. The physician has successfully completed a residency program.

123. “Quality Improvement” means a method of evaluation of services provided, which includes defined standards, evaluation methodologies and utilization of evaluation results for continued system improvement. Such methods may include, but not be limited to, a written plan describing the program objectives, organizations, scope and mechanisms for overseeing the effectiveness of the program objectives, organizations, scope and mechanisms for overseeing the effectiveness of the program, as well as metrics for measuring and identifying areas in need of improvement and recognition of excellence.
124. “Reasonable Search” means a brief attempt by EMS personnel to locate documentation that may identify a patient as a potential organ donor, or one who has refused to make an anatomical gift. This search shall be limited to a wallet or purse that is on or near the individual, or to locate a driver’s license or other ID card with donor status information.
125. “Receiving Hospital” means a hospital designated by the Monterey County EMS Agency, which has a written agreement with Monterey County to receive patients from the EMS System.
126. “Regional Disaster Medical Health Coordinator/Specialist” means the person appointed by the EMS Authority and Department of Health Services and their assistant who are responsible for supporting mutual aid requests from the MHOAC for disaster response within the region and mutual aid support to other areas of the State.
127. “Relevant employer(s)” means those ambulance service providers permitted by the Department of the California Highway Patrol or a public safety agency (i.e. fire department or law enforcement agency) that the certificate holder works for or was working for at the time of the incident under review as an EMT whether as a paid employee or a volunteer.
128. “Respondent” means the EMT or applicant on whom the EMS Agency submits an accusation charging violation(s) of Health and Safety Code section 1798.200 or other good cause for taking action against an EMT certificate or for denial of EMT certification.
129. “Resuscitation” means resuscitative measures to include chest compressions, defibrillation, endotracheal intubation, assisting ventilations, and other treatments such as medication administration.
130. “Reportable Event” means any event that falls outside the norms for patient treatment or system operation, but does not result in death, or serious physical or psychological injury to the patient, or risk thereof but does require investigation for the purpose of quality improvement.
131. “Rescue Aircraft” means an aircraft whose usual function is not pre-hospital emergency patient transport but which may be utilized, in compliance with local

EMS policy, for prehospital emergency patient transport when the use of an air or ground ambulance is inappropriate or unavailable. Rescue Aircraft includes ALS Rescue Aircraft, BLS Rescue Aircraft, and Auxiliary Rescue Aircraft.

132. "Senior Resident" or "Senior Level Resident" means a physician licensed in the State of California who has completed at least three (3) years of the residency or is in their last year of residency training and has the capability of initiating treatment and who is in training as a member of the residency program as defined in §100244 of CCR, at the designated trauma center.
133. "Sentinel Event" means an unexpected occurrence involving death or serious physical or psychological injury, or risk thereof.
134. "Service area" means that geographic area defined by the local EMS agency in its trauma care system plan as the area served by a designated Trauma Center. §100246, CCR, Title 22, Div. 9.
135. "Situation Report" means Medical and Health Situational Report.
136. "Special Event" means any planned and organized event where an EMS service provider utilizes its personnel and resources to *provide* stand-by dedicated EMS services to participants and attendees of the event.
137. "Special weapons and tactics (SWAT) team" means a special group of law enforcement officers trained to deal with unusually dangerous or violent situations.
138. "Spinal Motion Restriction" means the process of providing restriction in movement of the patient's spine.
139. "Stand-by EMS Services" means the utilization of personnel and resources to provide EMS services above regular staffing levels which are dedicated to the event and not expected to be available to respond to incidents unrelated to the event.
140. "STEMI Activation" means direction by the ED physician and/or interventional cardiologist to assemble the STEMI team and prepare the cath lab upon receipt of a STEMI Alert.
141. "STEMI Alert" means the report from prehospital personnel that notifies a STEMI Receiving Hospital that a patient has a 12-Lead EKG indicating a STEMI. This alert should be made as soon as possible to allow the STEMI Receiving Hospital the greatest opportunity to prepare for the patient.
142. STEMI Receiving Center (SRC). A facility designated by the Monterey County EMS Agency to receive STEMI patients by ambulance and capable of providing percutaneous coronary intervention (PCI).
143. "ST Elevation Myocardial Infarction (STEMI)" means a specific finding on the 12-Lead EKG showing ST segment elevation of greater than 1 mm in anatomically contiguous leads, indicating this specific type of myocardial infarction.

- 144. “Stroke Center” means a facility designated by the Monterey County EMS Agency for the purpose of receiving EMS patients who have positive findings on the BEFAST Stroke Scale.
- 145. “Tactical Medicine” means the delivery of medical services for law enforcement special operations.
- 146. “Tension pneumothorax” means an accumulation of air under pressure in the pleural space.
- 147. “Total Artificial Heart (TAH)” means a device that replaces the two ventricles of the heart. It may be used as a bridge to transplant or as a permanent replacement heart for patients dying from end-stage biventricular failure.
- 148. “Trauma Care System” or “Trauma System” means a system that is designed to meet the needs of all injured patients. The system shall be defined by the Local EMS Agency in its trauma system care plan in accordance with Trauma Regulations. §100248.
- 149. "Trauma Center" or "designated Trauma Center" means a licensed hospital, accredited by the Joint Commission, which has been designated as a Level I, II, III, or IV Trauma Center and/or Level I or II pediatric Trauma Center by the local EMS agency, in accordance with Trauma Care System regulations and statutes.
- 150. “Trauma Transfer” means the movement of other less seriously injured patients with traumatic injuries to the trauma center (those not meeting Emergency Re-Triage criteria) whose needs may be addressed in a prompt fashion but are less likely to require immediate intervention.
- 151. “Trauma Evaluation Quality Improvement Committee (TEQIC)” means a multi-disciplinary advisory committee to the EMS Medical Director whose purpose is to review the trauma system on a regular basis. It is comprised of designated representatives from the acute care hospitals within Monterey County. This is a closed committee.
- 152. “Trauma Registry” - a repository of data on the incidence, diagnosis, treatment, and outcome of acute trauma victims treated by emergency service personnel.
- 153. “Trauma Receiving Area” means a designated area within the trauma center that routinely receives and manages the care of trauma patients.
- 154. “Trauma Team” means the multi-disciplinary group of personnel who have been designated to collectively render care for trauma patients at the trauma center.
- 155. “Trauma System Participants” means all hospitals within Monterey County that receive trauma patients, and all prehospital care providers who treat and/or transport trauma patients.
- 156. “Time of Arrival at the Scene” means the time the responding resource arrives fully stopped at the location where the resource was dispatched. Arrival at a staging location will also be considered the time of arrival at the scene.
- 157. “Time of Arrival at the Receiving Facility” means the time that the resource comes to a stop at the destination when transporting a patient. Ambulance only.

- 158. “Time of Call Receipt” means the time that the call is received by the agency dispatch position either electronically or by phone.
- 159. “Time of Dispatch” means the time that a resource was first notified/alerted by dispatch to respond to an incident.
- 160. “Time of Dispatch” as referenced in Policy #4140 (ALS Service Provider Response and Scene Time Reporting) means the time recorded in the CAD system by the dispatcher to denote the time that the responding unit was requested to respond to the request for service.
- 161. “Time of Departure From the Scene” means the time when a resource leaves a scene to transport a patient to another location.
- 162. “Time of Departure from the Scene or Cancellation” as referenced in Policy # 4140 (ALS Service Provider Response and Scene Time Reporting) means the time recorded in the CAD system when the unit notified their dispatch center that they are leaving the scene due to patient transport, becoming available, responding to another call, to pick up a staff member, or cancellation.
- 163. “Time of Transport Complete” means the time that the ambulance comes to a stop at the receiving facility or landing zone when transporting a patient. Ambulance only.
- 164. “Unusual Occurrence” means any event deemed to have an impact or potential impact on patient care, and/ or any practice felt to be outside the norm of acceptable patient care, as defined by MCEMS policies and protocols.
- 165. “Validated” means to determine by preliminary evaluation that an act specified in California Health and Safety Code, Section 1798.200 may have occurred.
- 166. “Ventricular Assist Device (VAD)” means a mechanical pump that is used to support heart function and blood flow in people who have weakened hearts.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 2020

Effective Date:

Review Date:

SCOPE OF PRACTICE FOR EMERGENCY MEDICAL RESPONDERS

I. PURPOSE

To define the scope of practice for an Emergency Medical Responder (EMR) in the County of Monterey.

II. POLICY

- A. EMR personnel shall adhere to the County of Monterey EMR scope of practice while working in a BLS assignment with a County of Monterey EMS service provider. This includes volunteer and paid staff.
- B. The scope of practice of EMR personnel shall not exceed those activities authorized in California Code of Regulations, Title 22, and by County of Monterey EMS System policy.

III. SCOPE OF PRACTICE

- A. The Emergency Medical Responder is authorized to perform any of the following:
 - 1. Render BLS care, rescue, and first aid to patients according to County of Monterey EMS System policies and protocols.
 - 2. Obtain vital signs.
 - 3. Evaluate the ill and injured.
 - 4. Perform CPR.
 - 5. Provide basic treatment for shock.
 - 6. Use manual airway opening methods, including head-tilt-chin-lift and/or jaw thrust to support breathing.
 - 7. Use OPAs.
 - 8. Perform manual methods to remove an airway obstruction in adults, children, and infants, and use hand-powered and mechanical suction devices.
 - 9. Administer oxygen.
 - 10. Use a BVM ventilation device.
 - 11. Place patients into the recovery position.
 - 12. Perform spinal motion restriction.
 - 13. Splint injured extremities.

14. Perform emergency eye irrigation using water or normal saline.
15. Assist with the administration of oral glucose.
16. Assist patients with administration of physician-prescribed epinephrine devices and naloxone.
17. Assist in emergency childbirth.
18. Control hemorrhage using direct pressure, pressure bandages, dressings, pressure points, and tourniquets.
19. Apply chest seals and dressings.
20. Perform simple decontamination techniques and use decontamination equipment.
21. Package and transport amputated body parts
22. Provide basic wound care.
23. Use intranasal naloxone when trained according to County of Monterey EMS System Policy and performing duties for a County of Monterey EMS service provider.
24. Use an AED when performing duties for a County of Monterey EMS service provider.
25. Use various types of stretchers.
26. Extricate entrapped persons.
27. Perform field triage in accordance with County of Monterey protocol.
28. Administer 2-PAM and atropine by auto-injector (Mark 1 kit) to self or public safety (Fire, Law Enforcement, and/or EMS) personnel when trained according to County of Monterey EMS System policy and performing duties for a County of Monterey EMS service provider.
29. Administer epinephrine by auto-injector for suspected anaphylaxis when performing duties for a County of Monterey EMS service provider.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 2040

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PARAMEDIC ACCREDITATION-INITIAL

I. PURPOSE

Accreditation provides a California licensed paramedic with authorization to function as a paramedic in the County of Monterey and to utilize the County of Monterey paramedic scope of practice established by the EMS Medical Director while functioning as part of the County of Monterey EMS system.

II. POLICY

- A. Accreditation is authorization by the County of Monterey EMS Medical Director to practice as a paramedic within the jurisdiction of the employing Paramedic Service Provider.
- B. Employment with an approved Monterey County Paramedic Service Provider is required for accreditation.
- C. Accreditation remains active only when the paramedic continues to meet the qualifications for accreditation.
- D. Accreditation will be effective on the date the application and supporting documentation is processed by the EMS Agency and it is determined that all requirements have been met.
- E. Accreditation will expire on the expiration date of the California paramedic license or upon termination of employment with an approved County of Monterey Paramedic Service Provider.
- F. The County of Monterey Paramedic Service Provider must notify the EMS Agency, in writing, within five (5) business days of the separation in service by an accredited paramedic or when the paramedic is no longer functioning in a paramedic capacity. The notification must include a description of the cause for separation.

III. PROCEDURE

- A. Initial Accreditation - to be eligible for accreditation in Monterey County an individual must:
 - 1. Possess a valid and current California paramedic license.
 - 2. Submit the County of Monterey EMS Paramedic Accreditation application to the County of Monterey EMS Agency along with copies of the following items, all of which must be current:
 - a. Current and valid paramedic license.

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- b. CPR course completion card. The CPR course must meet the standards for course completion of the AHA Healthcare Provider or BLS course.
 - c. ACLS course completion card.
 - d. PALS, PEPP, or equivalent course completion card.
 - e. PHTLS, ITLS, or equivalent course completion card.
 - f. California driver's license or other acceptable government issued photo ID.
3. Pay the established fee.
4. Successfully complete an orientation to the County of Monterey EMS system.
 - a. The orientation shall be provided by the paramedic's employer and include:
 - 1) County of Monterey EMS policies and protocols with a County of Monterey EMS Agency approved test
 - 2) Communications systems
 - 3) County of Monterey geography, highways and roads, hospital locations and preferred routes of travel
 - 4) Equipment and supplies in use by the employer.
5. Successfully complete training in any basic and/or local optional scope of practice for which the paramedic has not been trained and tested.
6. Pay the accreditation fee if applicable.
7. Successfully complete a supervised pre-accreditation field evaluation to consist of no more than ten (10) ALS patient contacts to verify competence in local policies, procedures and expanded scope of practice within 30 days of applying for initial accreditation. Failure to successfully complete the field evaluation will result in denial of accreditation.
 - a. A licensed paramedic who has completed their paramedic internship in Monterey County within the preceding six (6) months or is returning to paramedic employment with a Monterey County EMS Paramedic Service Provider after a break of six (6) months or less is not required to complete the field evaluation process. All other requirements, including orientation, must be met.
 - b. A licensed paramedic with greater than two (2) years of full-time experience as a paramedic providing direct patient care through their County of Monterey employer may request a waiver of this requirement. The paramedic must submit the following to be considered for waiver of this requirement:
 - 1) Documentation from their paramedic employer attesting to the full-time status of the paramedic in a direct patient care capacity.

- 2) Documentation from the employer that the paramedic has had no disciplinary actions or clinical issues resulting in a plan for improvement, and that the paramedic is not currently under an investigation that could potentially result in discipline or a clinical improvement plan.
8. The relevant employer shall provide documentation of training and competency in the Monterey County EMS Agency's policies, protocols, and expanded scope of practice within 30 days of applying for accreditation. This shall include documentation of:
 - a. Successful completion of a supervised field evaluation.
 - b. Completion of training and orientation in the Monterey County EMS policies, protocols, and expanded scope of practice.
 - c. Demonstration of competence in and knowledge and ability to utilize the Monterey County EMS policies, protocols, and optional scope of practice to established standards approved by the Monterey County EMS Agency.
 - d. Appointment to a paramedic position with the approved Paramedic Service Provider.
 - e. The date on which the paramedic completed the requirements for accreditation.

B. Failure to complete the accreditation process

1. Failure to successfully complete the field evaluation process shall be reported to the Monterey County EMS Agency within five (5) business days of completion or termination of the field evaluation process by the employer.
 - a. The employer will provide all relevant documentation to allow the Monterey County EMS Medical Director to evaluate the paramedic.
 - b. The EMS Medical Director may recommend further evaluation or training to ensure the paramedic is competent. If the EMS Medical Director has reason to believe that the paramedic's competency to practice is questionable, then the EMS Medical Director shall notify the California EMS Authority.

C. Termination of accreditation

1. Accreditation to practice under the County of Monterey paramedic scope of practice shall end when the paramedic:
 - a. Ends employment with a County of Monterey paramedic service provider.
 - b. Does not fulfill the requirements for paramedic accreditation in the County of Monterey policies.
 - c. Does not maintain current licensure in the State of California.

END OF POLICY

County of Monterey EMS System Policy



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Effective Date:

Review Date:

PARAMEDIC ACCREDITATION RENEWAL

I. PURPOSE

Accreditation provides a California licensed paramedic with authorization to function as a paramedic in the County of Monterey and to utilize the County of Monterey paramedic scope of practice established by the EMS Medical Director while functioning as part of the County of Monterey EMS system.

II. POLICY

- A. Accreditation is the authorization by the County of Monterey EMS Medical Director to practice as a paramedic within the jurisdiction of the employing Paramedic Service Provider.
- B. Accreditation remains active only while the paramedic continues to meet the qualifications for accreditation and is employed by an approved County of Monterey Paramedic Service Provider.
- C. The paramedic must receive training and demonstrate competency in new and revised policies and protocols through an approved County of Monterey Paramedic Service Provider within the three (3) months prior to the policy and protocol effective date.
- D. The paramedic seeking to maintain accreditation shall submit a renewal application to the EMS Agency verifying that they meet the requirements to renew their accreditation. This application and supporting documents must be submitted to the EMS Agency following renewal of their paramedic license. The application and supporting documents must be received by the EMS Agency prior to the expiration date of their current Monterey County paramedic accreditation.
- E. Lapse in paramedic licensure or in any of the required courses, and/or the skills maintenance requirements outlined in County of Monterey EMS System Policy 6090 (Annual ALS Skills Maintenance Verification and Policy Review) may result in loss of accreditation or accreditation action.
- F. Accreditation renewal will be for the same period as the paramedic license submitted with the application for renewal. Accreditation will end upon termination of employment with an approved County of Monterey Paramedic Service Provider.
- G. The County of Monterey Paramedic Service Provider must notify the EMS Agency, in writing, within five (5) business days of the separation in service by an accredited paramedic or when the paramedic is no longer functioning in a paramedic capacity. The notification must include a description of the cause for separation.

III. PROCEDURE

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- A. Continuous Accreditation - to be eligible for continuous accreditation in Monterey County, an individual must submit the following to the EMS Agency before the expiration date of their current paramedic accreditation:
1. Proof of maintained California State paramedic licensure.
 2. A complete Monterey County EMS Agency accreditation renewal application.
 3. Accreditation fee if applicable.
 4. Proof of continued affiliation with an approved Monterey County EMS Paramedic Service Provider.
 5. Documentation of training and competency in those skills and medications added by the Monterey County EMS Agency since the paramedic's initial accreditation.
 6. Documentation of current successful course completion of ACLS, BCLS, PALS or equivalent pediatric course, and PHTLS or equivalent trauma course.
 7. Verification of paramedic skills maintenance as required by County of Monterey EMS System Policy 6090.
- B. Termination of accreditation
1. Accreditation to practice under the County of Monterey paramedic scope of practice shall end when the paramedic:
 - a. Ends employment with a County of Monterey paramedic service provider.
 - b. Does not fulfill the requirements for paramedic accreditation in the County of Monterey policies.
 - c. Does not maintain current licensure in the State of California.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 2060

Effective Date:

Review Date:

EMT CERTIFICATION (INITIAL)

I. PURPOSE

To specify the requirements and procedure for initial Emergency Medical Technician (EMT) certification in the County of Monterey.

II. POLICY

- A. To initially certify as an EMT, an applicant must satisfy all relevant State regulations and County of Monterey EMS requirements. If this policy should be superseded by changes in State regulations, then the EMS Agency practice will be adjusted accordingly, until this policy can be revised and disseminated.
- B. The application for EMT certification is considered “received” by the EMS Agency once EMS Agency staff have reviewed the application and supporting documents and determined the application packet is complete.
- C. To perform EMT procedures under the basic EMT scope of practice requires the individual to obtain an EMT certificate from a Local EMS Agency (LEMSA) or other approved certifying entity. Completion of the National Registry exam and receiving a National Registry EMT card does not, by itself, provide certification to function as an EMT.
- D. EMT accreditation and affiliation with a County of Monterey EMS service provider organization is required to perform procedures authorized by the County of Monterey EMT Scope of Practice while functioning as an EMT for the County of Monterey EMS service provider organization as part of the County of Monterey EMS system.

III. PROCEDURE

- A. Applicants for EMT certification in Monterey County shall submit all of the following documents to the Monterey County EMS Agency through the method posted on the EMS Agency website:
 - 1. Completed application for EMT certification
 - 2. Government issued photo ID to confirm applicant identity, and that the applicant is 18 years of age or older.
 - 3. Copy of a current CPR/AED card that is equivalent to the American Heart Association (AHA) Healthcare Provider or BLS level of training and includes skills demonstration.
 - 4. Copy of a current National Registry Emergency Medical Technician (NREMT) Certificate Card.
 - 5. Copy of the National Registry certificate with date of issue.

6. Completed Department of Justice Live Scan application.
 - a. All DOJ Live Scan issues need to be adjudicated and resolved prior to certification.
 - b. The application will not be processed until the Live Scan results have been received by the EMS Agency.
 7. Copy of an EMT Basic Course Completion Certificate from an approved EMT Basic Course provider. To apply for EMT certification, the EMT Basic Course Completion Certificate is valid for two (2) years from the course completion date.
 8. Written disclosure of any prior and/or current certification, licensure, or accreditation actions, including actions:
 - a. Against an EMT or Advanced EMT certificate, or any denial of certification by a LEMSA, including active investigations.
 - b. Against a Paramedic license, or any denial of licensure by the State EMS Authority, including any active investigations.
 - c. Against any EMS-related certification or license from another state or other issuing entity, including denials and any active investigations.
 - d. Against any medical or healthcare-related license.
 - e. A written explanation (with appropriate documentation) of any affirmative affidavit response is required and shall accompany the application.
 9. Written disclosure of any pending or current criminal investigation, pending criminal charges, or any prior felony and/or misdemeanor convictions.
 10. Written disclosure of each certifying entity or LEMSA to which the applicant has applied for certification in the previous twelve (12) months.
 11. Payment of the established fee.
 12. A current and valid out-of-state EMT certificate and documentation of passing the cognitive and psychomotor exam within the previous two (2) years as required in Title 22 if the EMT wishes to transfer from another state.
- B. All of the necessary documents must be received by the EMS Agency within the relevant time limits specified above. An application will not be processed until all of the necessary documents are received and accepted. Illegible documents will not be accepted.
- C. The EMS Agency will perform a medical license or certification check through the National Practitioner Data Bank on all applicants. EMT certification will not be provided until this records check has been completed and any actions reported through the NPDB have been reviewed and addressed.
- D. The EMS Agency will electronically issue a forty-five (45) day temporary wallet-sized certificate to eligible applicants. The certificate will contain the following information:
1. The name of the individual certified.
 2. The assigned EMT California Central Registry certification number.

3. The date the certification is effective.
 4. The date certification expires.
 5. The status of the EMT certification (i.e., active, probation, denied, suspended, expire, etc.).
 6. The name and location of the EMT certifying authority (County of Monterey EMS Agency).
 7. The name and signature of the individual (EMS Medical Director) authorized to certify, or facsimile of same.
 8. A statement that the individual named on the card has fulfilled the requirements for certification as an EMT in California.
- E. The EMS Agency will print the regular EMT certificate (card), which will be sent to the applicant's mailing address by USPS.
- F. The effective date of EMT certification shall be the date the EMS Agency verifies that the applicant has satisfactorily completed all certification requirements.
- H. Certification as an EMT shall be no more than two (2) years from the effective date of issuance. The certification expiration date will be the last day of the month two (2) years from the effective date of initial certification.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 2070

Effective Date:

Review Date:

EMT CERTIFICATION (RENEWAL)

I. PURPOSE

To specify the requirements and procedure for renewing an Emergency Medical Technician (EMT) certification in the County of Monterey.

II. POLICY

- A. To maintain EMT certification, an applicant must satisfy all relevant state regulations and County of Monterey EMS policy requirements. If this policy should be superseded by changes in state regulations, then the EMS Agency practice will be adjusted accordingly, until a revised policy can be developed and disseminated.
- B. The application for EMT certification is considered “received” by the EMS Agency once EMS Agency staff have reviewed the application and supporting documents and determined the application packet is complete.
- C. EMT accreditation and affiliation with a County of Monterey EMS service provider organization is required to perform procedures authorized under the County of Monterey EMT Scope of Practice while functioning as an EMT for a County of Monterey EMS service provider organization as part of the County of Monterey EMS system.

III. PROCEDURE

- A. Applicants for EMT certification renewal shall submit the following documents to the County of Monterey EMS Agency through the method posted on the EMS Agency website:
 - 1. County of Monterey EMS Application for EMT (completed, signed, and dated). A written explanation (with appropriate documentation) of any affirmative affidavit response is required and needs to accompany the application for any affirmative response not previously disclosed to the EMS Agency
 - 2. Course-completion records documenting successful completion of either:
 - a. At least twenty-four (24) hours of continuing education (CE) hours from an approved continuing education provider in accordance with Title 22 CCR Division 9, Chapter 3.5 (*EMS Continuing Education*); or
 - b. A twenty-four (24) hour refresher course from an approved EMT training program.
 - c. CE hours may be used to renew both a paramedic license and an EMT certificate. The CE hours must be earned during the certification cycle being renewed.

3. Copy of applicant's California EMT certification card.
 4. Copy of a current CPR card that is equivalent to the American Heart Association (AHA) Healthcare Provider or BLS card and must include skills evaluation.
 5. Government issued photo ID to confirm applicant identity, and that the applicant is 18 years of age or older.
 6. Payment of the established fee.
 7. A completed skills competency verification form. Skills competency shall be verified by direct observation of an actual or simulated patient contact. Skills competency shall be verified by an individual who is currently certified or licensed as an EMT, AEMT, Paramedic, RN, PA, or physician and who shall be designated by an EMS-approved training program (EMT training program, paramedic training program, or continuing-education provider) or an EMS service provider. EMS service providers include, but are not limited to, public safety agencies, private ambulance providers, and other EMS providers.
 8. Completed Department of Justice (DOJ) and Federal Bureau of Investigation (FBI) Live Scan Background Investigation application.
 - a. Results of the Live Scan report must be received before the application will be processed.
 - b. Live Scan is only required to be submitted when changing certifying entities (*i.e.*, for an applicant who already has an EMT certificate and is coming to Monterey County to renew). See below for the reinstatement procedure if an EMT certification has been expired for greater than one (1) year.
 9. An EMT renewing their certification for the first time after July 1, 2019, shall submit documentation of successful completion of the following training by an approved EMT training program or approved EMS CE provider:
 - a. The use and administration of naloxone that meets the standards and requirements of California Code of Regulations, Title 22, Division 9, Chapter 3.2.
 - b. The use and administration of epinephrine by auto-injector that meets the standards and requirements of California Code of Regulations, Title 22, Division 9, Chapter 3.2.
 - c. The use of a glucometer that meets the standards and requirements of California Code of Regulations, Title 22, Division 9, Chapter 3.2.
 - d. The requirement for training in the use of naloxone, epinephrine, and glucometer does not apply to a person who has an active California-issued paramedic license.
- B. The application and supporting documentation must be received by the EMS Agency and with sufficient time to allow for review and processing by the EMS Agency prior to the

expiration date. An application will not be processed until all necessary documents are received and accepted. Illegible documents will not be accepted.

- C. The EMS Agency will perform a medical license or certification check through the National Practitioner Data Bank on all applicants. EMT certification will not be provided until this records check has been completed and any actions reported through the NPDB have been reviewed and addressed.
- D. The EMS Agency will electronically issue a forty-five (45) day temporary wallet-sized certificate to eligible applicants. The certificate will contain the following information:
 - 1. The name of the individual certified.
 - 2. The assigned EMT California Central Registry certification number.
 - 3. The date the certification is effective
 - 4. The date certification expires.
 - 5. The status of the EMT certification (i.e., active, probation, denied, suspended, expire, etc.).
 - 6. The name and location of the EMT certifying authority (County of Monterey EMS Agency).
 - 7. The name and signature of the individual (EMS Medical Director) authorized to certify, or facsimile of same.
 - 8. A statement that the individual named on the card has fulfilled the requirements for certification as an EMT in California.
- E. The EMS Agency will issue the regular EMT certificate (card), which will be sent to the applicant's mailing address by USPS.
- F. The effective date of EMT certification shall be the date following expiration of the current EMT certificate if the completed application for renewal is received before the expiration date.
- G. The EMT renewal certification period is for two (2) years from the effective date and expires on the last day of the month. When the completed application is received greater than six (6) months prior to the expiration date of the current EMT certificate, the expiration date will be the last day of the month two (2) years from the month the completed application was received.

IV. CONTINUING EDUCATION (CE) REQUIREMENTS

- A. CE certificates submitted for EMT certification renewal shall be for courses completed during the current period of certification.
- B. CE certificates submitted for recertification of a lapsed EMT certificate shall only be accepted for courses completed within the two (2) years prior to the date of application for recertification.
- C. The EMT shall maintain their CE course records as specified in California Code of Regulations, Title 22, Division 9, Chapter 3.5.

- D. An individual may receive credit for taking the same CE course, class, or activity no more than two (2) times during a single certification cycle. The second CE course will not be accepted for EMT certification renewal if it is taken within six (6) months of the first course.

V. REINSTATEMENT OF A LAPSED EMT CERTIFICATE

- A. Upon lapse of an EMT certificate, the individual is not authorized to perform EMT level services until the EMT certificate has been reinstated.
- B. For a lapse of less than six (6) months, the individual shall comply with the requirements above for EMT recertification. The effective date for reinstatement will be the date when the application is received and determined to be complete.
- C. For a lapse of six (6) months or more but less than twelve (12) months, the individual shall comply with the requirements above for EMT renewal and complete an additional twelve (12) hours of continuing education for a total of thirty-six (36) hours of training.
- D. For a lapse of twelve months or more, the individual shall comply with the requirements above for EMT renewal, complete an additional twenty-four (24) hours of continuing education for a total of forty-eight (48) hours of training, pass the written and skills NREMT exam or have a current and valid NREMT registration, and complete (resubmit) a Department of Justice (DOJ) and Federal Bureau of Investigation (FBI) Live Scan Background Investigation.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 2110

Effective Date:

Review Date:

DENIAL OR REVOCATION OF AN EMT CERTIFICATE

I. PURPOSE

- A. To specify causes for the denial or revocation of a prehospital emergency medical care certificate.

II. POLICY

- A. The EMS Medical Director may deny or revoke an EMT certificate for disciplinary cause that has been investigated and verified pursuant to the requirements of California Health and Safety Code, Division 2.5.
- B. The EMS Medical Director shall deny or revoke any EMT certificate if any of the following apply to the applicant or certificate holder:
 - 1. Has committed any sexually related offense specified under Section 290 of the Penal Code.
 - 2. Has been convicted of murder, or attempted murder, or murder for hire.
 - 3. Has been convicted of two (2) or more felonies.
 - 4. Is on parole or probation for any felony.
 - 5. Has been convicted and released from incarceration for said offense during the preceding fifteen (15) years for the crime of manslaughter or involuntary manslaughter.
 - 6. Has been convicted and released from incarceration for said offense during the preceding ten (10) years for any offense punishable as a felony.
 - 7. Has been convicted of two (2) or more misdemeanors within the preceding five (5) years for any offense relating to the use, sale, possession, or transportation of narcotics or addictive or dangerous drugs.
 - 8. Has been convicted of two (2) or more misdemeanors within the preceding five (5) years for any offence relating to force, or threat, or violence, or intimidation.
 - 9. Has been convicted within the preceding five (5) years of any theft related misdemeanor.
- C. The EMS Medical Director may deny or revoke any prehospital certificate if any of the following apply to the applicant:
 - 1. Has committed any act involving fraud or intentional dishonesty for personal gain within the preceding seven (7) years.

2. Is required to register pursuant to the California Health and Safety Code, Section 11590.
- D. This policy shall not apply to convictions that have been pardoned by the Governor and shall only apply to convictions where the applicant/certificate holder was prosecuted as an adult. Equivalent convictions from other states shall apply to the type of offenses listed in B or C. As used in this policy, “felony” or “offense punishable as a felony” refers to an offense for which the law prescribes imprisonment in the state prison as either an alternative or the sole penalty, regardless of the sentence the particular defendant received.
- E. This policy shall not apply to an EMT who obtained an EMT certificate prior to July 1, 2010; unless:
1. The certificate holder is convicted of any misdemeanor or felony after July 1, 2010.
 2. The certificate holder committed any sexually related offense specified under California Penal Code, Section 290.
 3. The certificate holder failed to disclose any prior convictions when completing his/her application for initial EMT certification or recertification.
- F. Nothing in this policy shall negate an individual’s right to appeal a denial or revocation of a prehospital certificate.
- G. Negative certification action by the EMS Medical Director shall be valid statewide and honored by all certifying entities for a period of at least twelve (12) months from the effective date of the certification action. An EMT whose application was denied or an EMT whose certification was revoked by the EMS Medical Director shall not be eligible for certification by any other certifying entity for a period of at least twelve (12) months from the effective date of the certification action.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 2230

Effective Date:

Review Date:

EMERGENCY MEDICAL DISPATCHER EDUCATION AND QUALITY IMPROVEMENT (QI) PROGRAM

I. PURPOSE

To establish the minimum requirements and standards for initial orientation, Continuing Dispatch Education (CDE), and the Quality Improvement (QI) Program of a designated EMS Communications Center. Such a program will be designed to promote and ensure the high- quality performance of the Medical Priority Dispatch System (MPDS) and promote a culture of excellence in performance.

II. POLICY

A designated EMS Communications Center shall:

- A. Implement and maintain an active initial orientation, Continuing Dispatch Education (CDE), and a Quality Improvement (QI) Program.
- B. Actively participate in County of Monterey EMS system-wide QI activities and drills.
- C. Have all initial orientation, CDE, and QI Programs approved by the EMS Agency.

III. PROCEDURE

- A. A designated EMS Communications Center shall submit a written initial orientation, CDE, and QI Plan including course curriculum and final exams to the EMS Agency for approval.
- B. Each initial orientation, CDE, and QI Plan shall be consistent with International Academies of Emergency Dispatch (IAED) recommendations and standards and shall contain the following:
 - 1. Name and credentials of the CDE and QI Program Coordinator, who shall be qualified by education and experience in methods, materials, and evaluation of instruction. The Program Coordinator shall be responsible for the following:
 - a. Administration of all training and education including CDE and QI Programs.
 - b. Signing all training and course completion records.
 - c. Ensuring that all training, CDE, and QI activities are compliant with EMS Agency policies.
 - 2. CDE and QI program overview, goals, and objectives to include the structure and function of the following:
 - a. Dispatch Review Committee (DRC).

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- b. Dispatch Steering Committee (DSC).
 - c. Quality Improvement Unit (QIU).
- 3. Description of how QI is integrated into the day-to-day operations.
- 4. Description of the collection, reporting, and analysis of data and feedback processes.
- 5. Description of internal QI committee participation, responsibilities, and structure.
- 6. Description of Medical Director responsibilities and involvement.
- C. EMD/MPDS QI shall also be represented and integrated into County Prehospital QI committees.
- D. The EMS Agency shall review all plans submitted and notify the EMS Communications Center within 14 (fourteen) days of receipt of any required missing information.
- E. The EMS Agency shall notify the EMS Communications Center in writing within thirty (30) days of receipt of a complete plan of an approval or rejection decision.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 2260
Effective Date: 7/1/2026
Review Date: 6/30/2029

EMERGENCY MEDICAL DISPATCHER (EMD) CERTIFICATION

I. PURPOSE

To establish criteria for certification of Emergency Medical Dispatchers in the County of Monterey.

II. POLICY

Any individual providing Medical Priority Dispatch System (MPDS) services to the public as a part of the County of Monterey 9-1-1 system shall be certified as an Emergency Medical Dispatcher (EMD) as per standards maintained by the International Academies of Emergency Dispatch (IAED) and all applicable state regulations and guidance as referenced below.

Individuals shall comply with all applicable standards for testing, initial certification, recertification, continuing education, and continuous cardiopulmonary resuscitation (CPR) certification at either the American Heart Association Healthcare Provider or American Red Cross CPR for the Professional Rescuer level. An equivalent CPR certification may be substituted with EMS Agency approval.

III. PROCEDURE

- A. Dispatch Centers that provide EMD services are responsible for maintaining up to date documentation of all personnel records pertaining to Emergency Medical Dispatcher testing, certification, recertification, continuing education, and CPR certification.
- B. Documentation will be made available to the EMS Agency for auditing upon request.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 3000

Effective Date:

Review Date:

EMS COMMUNICATIONS SYSTEM

I. PURPOSE

To prescribe the methods participants in the County of Monterey EMS System use to communicate with each other during routine operations and medical disaster situations and to define required testing schedules for every component of the EMS Communications system.

II. POLICY

- A. All vehicles utilized in the County of Monterey EMS system shall be equipped with the appropriate radio and telecommunications equipment necessary for dispatch and other related communications
- B. To ensure that communication systems used by EMS System participants are operational during a disaster, those systems shall be regularly tested, as described in this policy.
- C. To ensure personnel are always capable of using EMS communication systems, participants must practice using infrequently used methods of communication to ensure reasonable familiarity with those systems during times of disaster.
- D. EMS system participants are responsible for acquiring and maintaining their EMS communications systems to ensure they are in good working order.
- E. Any significant problems or outages of any component of the EMS communications system should result in the immediate notification of all appropriate personnel – including the EMS Duty Officer.

III. COMMUNICATION SYSTEMS AND USE

- A. EMS System Participants shall use the EMS Communications systems identified below for routine and disaster communications. In the event a primary communication method fails, EMS providers shall immediately use a backup or redundant system.
 - 1. 800 MHz Disaster Medical Radio System
 - 2. Field to Hospital Mednet Frequencies (Med 2, 6 and 7)
 - 3. EMS Communications to Field Mednet Frequencies (Med 1, 5, 9 and 10)
 - **Med 1** – Used on the Peninsula, covering from the Carmel Highlands south to the County Line.
 - **Med 5** – Used in the Valley, from Greenfield south to the County Line.

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- **Med 9** – Used in the Valley, spanning from the North County Line south to just south of Greenfield.
- **Med 10** – Used on the Peninsula, covering the area from Marina to the Carmel Highlands.

4. ReddiNet

5. EMS Paging and Alerting System

B. ReddiNet Use

1. Hospitals should update their HAvBED and overall bed status in ReddiNet at least every 24 hours during routine operations.
2. Hospitals are responsible for updating change in their facility's status (e.g., specialty care diversion (Stroke, STEMI, Trauma), internal disasters, Incident Command System (ICS) activation).
3. The EMS Communications Center is responsible to enter actual MCI and disaster information in ReddiNet immediately upon declared MCI situations or other disasters.

IV. COMMUNICATION SYSTEM TESTING

A. EMS System participants shall participate in EMS System communication tests. The performance standard is a 95% response rate in each communication system.

B. Implementation of Communication System Testing

1. The EMS Communications Center will lead and coordinate the testing of each of the communication methods within the EMS Communication System.
2. EMS communication tests will occur daily for the following systems:
 - a. 800 MHz. Disaster Medical Radio System
 - b. ReddiNet
3. The EMS Communications Center will record and provide the EMS Agency with the results of EMS System communication tests.
4. If a facility does not respond to one or both daily communication tests for three (3) consecutive days, EMS Communications Center staff will follow up with the facility via phone to verify operational status and ensure that the communication system is functioning properly. If the facility is unable to rectify the issue or continues to fail to respond, the EMS Communications Center will notify the EMS Duty Officer for further evaluation and coordination.

C. Each organization's responsiveness will be tracked, trended, and reported by the EMS Agency.

D. Training needs will be addressed as identified by system stakeholders and the EMS Agency.

V. COMMUNICATION SYSTEM TRAINING, EXERCISING, AND REVIEW

- A. Employing organizations shall train each employee to competently use each component of the EMS System communication equipment applicable to their position. Recurring training will be done on those systems used less frequently.
- B. Following any critical incident or medical disaster, any post-incident after action review shall include a section on EMS communications, including reporting on components that worked well and components that either failed or did not perform as expected.
- C. The EMS Communications Center will work collaboratively with other EMS system stakeholders in quality improvement, policy and procedure development, and clinical and operational changes to the system and will provide the data needed for monitoring and evaluation of the EMS system.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 3010

Effective Date:

Review Date:

EMS/EMD COMMUNICATION CENTER DESIGNATION AND REQUIREMENTS

I. PURPOSE

To establish criteria and minimum requirements for designation of an Emergency Medical Services (EMS) /Emergency Medical Dispatch (EMD) Communications Center in the County of Monterey EMS System to ensure that highly coordinated EMS/EMD services are delivered to the people in the County of Monterey.

II. POLICY

- A. The EMS/EMD Communications Center(s) shall be designated by the EMS Agency to provide contracted ambulance provider dispatch and/or Emergency Medical Dispatch (EMD) services for any portion of the County of Monterey EMS System.
- B. Designated EMS/EMD Communications Center(s) shall meet the standards outlined in American Society for Testing and Materials (ASTM) F-1258 (Standard Practice for Emergency Medical Dispatch), F-1552 (Standard Practice for Training Instructor Qualification and Certification Eligibility of Emergency Medical Dispatchers), and ASTM F-1560 (Standard Practice for Emergency Medical Dispatch Management).
- C. Designated EMS/EMD Communications Center(s) shall have a written agreement with the EMS Agency for such services. (3010 ATTACHMENT A)

III. EMS AGENCY DESIGNATED EMS/EMD COMMUNICATIONS REQUIREMENTS

- A. Designated EMS/EMD Communications Center(s) shall meet the following minimum requirements:
 - 1. Submit an application to the EMS Agency for EMS/EMD designation and meet all criteria herein (3010 ATTACHMENT B). The EMS Agency shall return the application for further information, approve the application, or deny the application request in writing within sixty (60) days of receipt.
 - 2. Indicate the full scope of EMS/EMD related services provided.
 - 3. Comply with all applicable State and Federal statutes, codes and regulations, and EMS System Policies and Procedures.
 - 4. Utilize the current version (within 1 year of release) of the Medical Priority Dispatch System™ (MPDS) on all possible medical calls within the County of Monterey EMS System pursuant to EMS System Policies and Procedures.
 - 5. Provide EMD/MPDS services for all appropriate medical calls within its jurisdiction, consistent with EMS System Policy 3030 “EMS Call Routing &

Processing”.

6. Submit for approval and maintain internal written EMD policies and procedures approved by the EMS Agency, including, but not limited to:
 - a. Emergency call surge (multiple callers for single event) and “Urgent Disconnect”.
 - b. EMD orientation, Continued Dispatch Education (CDE) program, and EMD Quality Improvement (QI) program.
 - c. Third party caller interrogation.
 - d. Language translation.
 - e. Continuity of Operations Plan (COOP).
7. Obtain and maintain continuous EMD certifications, issued by the International Academies of Emergency Dispatch (IAED), for all employees providing EMD/MPDS or supervising EMS/EMD related activities, and shall maintain all records related to EMD certification, re- certification, training, testing, performance improvement plans, and CDE for a period of no less than four (4) years.
8. Have on site direct dispatch supervision by at least an EMS/EMD Communications Manager, EMS/EMD Communications Supervisor, or EMS/EMD Shift Lead Dispatcher, or equivalent, 24 hours a day.
9. At the request of the EMS Agency, shall serve as the Operational Area Patient Distribution Center (OAPDC) and/or local Disaster Control Facility (DCF), manage ReddiNet, and operate on behalf of the Medical Health Operational Area Coordinator (MHOAC) for the County of Monterey pursuant to EMS Agency policy.
10. Designate a representative(s) to attend, participate in any relevant EMS Agency or other committee meetings, such as Dedicated Fire Dispatch, EMS System QI, or other appropriate technical advisory committees and/or working groups.
11. Actively participate in medical advisory oversight related to MPDS determinate response planning, QI, and protocols related to MPDS.
12. Provide accurate data and reports to the EMS Agency in a timely manner as requested.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 3030
Effective Date: 7/1/2026
Review Date: 6/30/2029

EMS CALL ROUTING AND PROCESSING

I. PURPOSE

To define the requirements and procedures for Public Safety Answering Points (PSAPs) and Public Safety Agencies to identify and route all appropriate Medical Calls to a designated EMS Communications Center in the County of Monterey, so that certified Emergency Medical Dispatchers can provide Medical Priority Dispatch System (MPDS) services. These procedures include both concurrence or non-concurrence of Public Safety Agencies with jurisdiction to continue delegation of EMS call processing or to maintain EMS call processing within a Public Safety jurisdiction.

II. POLICY

- A. Concurrence: All appropriate Medical Calls in the County of Monterey, once identified, shall be immediately routed to a designated County of Monterey EMS Communications Center for proper MPDS assessment, Post-Dispatch Instructions, and Pre-Arrival Instructions. This includes all first, second, and appropriate third-party callers.
- B. Non-Concurrence: All appropriate Medical Calls in the County of Monterey, once identified, shall immediately receive proper MPDS assessment, Post-Dispatch Instructions, and Pre-Arrival Instructions by Public Safety communications personnel in compliance with EMS System Policies.

III. PROCEDURE

- A. Concurrence Public Safety/PSAP Medical Call Routing:
 - 1. All Medical Call phone transfers shall ensure ANI/ALI information, if available, is sent with the caller to the designated EMS Communications Center.
 - 2. The designated EMS Communications Center shall work with local PSAPs to develop and implement policies that address special call handling procedures, including but not limited to: multiple callers for a single incident, language barrier calls, and rescue events.
 - 3. All County of Monterey PSAPs shall ensure that call intake procedures only obtain essential information on Medical Calls prior to transfer of the caller to the designated EMS Communications Center and shall not conduct any medical screening or other redundant questioning contained within a MPDS interrogation.
 - a. An exception to the routing of the caller in this policy is when a PSAP determines it is not safe to transfer the caller to the designated EMS Communications Center. In those cases, such as Law Enforcement calls, scene safety concerns, and some specialized rescue, the PSAP shall ensure

the address, chief compliant, and safety instructions are immediately included in the comments of the call notes and shall not delay EMS call creation or response to the address of the emergency. For the Exclusive Operating Area (EOA) contracted ambulance provider, all such calls that cannot be immediately processed through MPDS shall be assigned a Priority 2 response unless otherwise requested by first responders on scene, law enforcement, or subsequent MPDS determinant.

4. Once a PSAP has completed their interrogation, if appropriate, they should transfer the caller to a designated EMS Communications Center for MPDS processing.
5. All initial, subsequent, or modified Public Safety/PSAP policies and procedures related to this EMS System policy shall be made available to the EMS Agency before implementation.

B. Non-Concurrence Public Safety/PSAP Medical Call Routing:

1. Public Safety/PSAPs shall develop and implement policies that ensure all Medical Calls are properly identified and processed through MPDS procedures consistent with EMS System policies.
2. Public Safety/PSAPs shall ensure all appropriate responders (Police, Fire, EMS) receive the call simultaneously for proper assignment by their respective dispatchers.
3. All initial, subsequent, or modified Public Safety/PSAP policies and procedures related to this EMS System policy shall be made available to the EMS Agency before implementation.

C. Concurrence and Non-Concurrence Public Safety/PSAPs and/or designated EMS Communications Center Medical Call entry and MPDS:

1. The Public Safety/PSAPs and/or designated EMS Communications Center shall ensure that all Medical Calls include a problem nature protocol description selected in CAD that corresponds exactly with the MPDS protocol numbers 1-33 descriptions.
2. The Public Safety/PSAPs and/or designated EMS Communications Center shall ensure that MPDS is used on all Medical Calls, including requests from medical facilities to transport a patient to the emergency department (ED) (Card 33).
3. The Public Safety/PSAPs and/or designated EMS Communications Center shall ensure all available comments; including scene safety, and patient condition, are documented in the call by the call taker or Emergency Medical Dispatcher and are available, real time, for responders to view.
4. The Emergency Medical Dispatcher shall remain on the phone on all ECHO level, and appropriate DELTA level calls, to provide post-dispatch instructions, pre-arrival instructions, or to maintain and monitor, until first responders are with the patient.

5. For MPDS lower-acuity determinants as approved by the EMS Agency, the Emergency Medical Dispatcher may transfer callers to the 9-1-1 Nurse Navigation Program for further assessment and potential alternative disposition, consistent with EMS Agency policy. The Emergency Medical Dispatcher shall ensure all required MPDS questioning is completed prior to initiating a Nurse Navigation transfer.

D. Concurrence Public Safety/PSAP and/or designated EMS Communications Center Medical Call Dispatch Process:

1. Once Public Safety/PSAPs and/or designated EMS Communications Center identifies a call as a Medical Call, and an accurate location is identified, Public Safety/PSAPs and/or designated EMS Communications Center shall ensure the call is created/generated in the CAD assignment queue simultaneously for Fire first response and ambulance response.
2. All Medical Calls shall initially be created as a Priority 2 response for the EOA contracted ambulance provider.
3. Once the Emergency Medical Dispatcher has assigned a MPDS determinant, the EOA contracted ambulance provider response priority shall be modified by the appropriate dispatcher, if necessary, based on the criteria in EMS System Policy and Procedure 3050: MPDS Response Priorities and Assignments to EMS Calls.
4. Once an Emergency Medical Dispatcher has assigned a MPDS determinant, the Emergency Medical Dispatcher shall immediately provide the updated determinant to the first responder.
5. Any reduction or increase of an ambulance response priority to a Medical Call, for any reason, shall be recorded so that upon request it is available to the EMS Agency for accurate data analysis and response time calculations.

County of Monterey EMS System Policy



Policy Number: 3050
Effective Date: 7/1/2026
Review Date: 6/30/2029

MPDS RESPONSE PRIORITIES AND RESOURCE ASSIGNMENTS TO EMS CALLS

I. PURPOSE

To establish clinically-indicated response priorities and response planning for all emergency medical calls using the Medical Priority Dispatch System™ (MPDS). The MPDS determinants will drive response plans and priorities to reduce the unnecessary use of lights and siren and will provide crucial data for EMS system review and Quality Improvement.

II. BACKGROUND

- A. Response determinant methodology is defined by the International Academy of Emergency Dispatch (IAED) in non-linear response levels based on Capability (BLS vs ALS), single vs multiple resources, and Response Priorities based on the general acuity of the patient and resources needed.
- B. Response Levels (ECHO, DELTA, CHARLIE, BRAVO, ALPHA, and OMEGA) are designated by the IAED and cannot be modified. However, for the EOA-contracted ambulance provider, Capability, Response Priority, and Response Resources shall be assigned by the EMS Medical Director or the EMS Agency.
- C. Response Priorities and Resource Assignments are detailed below (see Table 1). The EMS Medical Director may modify Response Priority and Response Resources to any level or determinant as needed.

III. POLICY

- A. Only the designated EMS Dispatch Center(s) shall dispatch the EOA-contracted ambulance provider, as specified in this policy.
- B. The designated EMS Dispatch Center shall ensure that all medical calls will be dispatched using a standard dispatch script.
- C. The EOA-contracted ambulance provider response is subject to reassignment to a higher priority call only if the following conditions exist, as determined by the Emergency Medical Dispatcher (EMD):
 - a. The reassignment presents a clear and obvious time savings to the higher priority call; OR
 - b. The diversion from the lower priority call will not subject unnecessary risk to the lower priority patient.

- D. Any EOA-contracted ambulance provider response is subject to upgrades or downgrades of Priority as additional information is obtained or the patient's condition changes, as determined by MPDS or by first responders on scene. Upgrade of a Priority to lights and siren is prohibited for the sole purpose of expediting a perceived extended response by a transport provider, unless the first responder on scene has determined the patient requires immediate transport or ALS intervention.
- E. EOA-contracted ambulance provider Response Resources and Response Priorities must be approved by Local Medical Control (EMS Medical Director) and are subject to internal EMS Agency review at any time. The EMS Medical Director may approve modifications of Response Resources and/or Response Priorities based on other sources including, but not limited to: QI processes, medical best practices, case studies, internal review, and other sources.
- F. Each fire department or fire district may determine their own Response Priorities to each type of call. If a fire department or fire district chooses to use response plans and/or Response Priorities that differ from EMS System policies, that fire department or fire district shall submit its response plans to the EMS Agency.
- G. The designated EMS Dispatch Center(s) shall be responsible for implementing internal Dispatch Review Committee (DRC) and/or Dispatch Steering Committee (DSC) processes.

IV. RESPONSE ASSIGNMENTS

- A. EOA-contracted ambulance provider Response Resources and Response Priorities shall be built into the CAD in accordance with this policy and EMS Medical Director approval.
- B. Creation of the Medical Call Incident and routing to the dispatch queue for first response and ambulance dispatch shall occur simultaneously as defined in EMS System Policy 3030.
- C. County of Monterey EMS Agency-approved MPDS Response Resources and Response Priorities for chief complaint protocols are specified in Table 1.
 - a. CHARLIE and BRAVO response levels may indicate a single resource or multiple resources based on recommended resource ETA.
 - b. BRAVO and ALPHA response levels may have BLS or ALS response assignment based on recommended resource ETA.
- D. Nurse Navigation Alternative Disposition:
 - a. For MPDS-approved low-acuity determinants (e.g., selected OMEGA and ALPHA), the EMD may transfer the caller to the EMS Agency-approved 9-1-1 Nurse Navigation Program for secondary triage and referral to alternative care resources.

- b. Use of Nurse Navigation must align with EMS Medical Director–approved determinants.
- c. Assignment of an ambulance response is not required when a caller is transferred to Nurse Navigation, unless directed by the Nurse Navigator or based on additional information obtained.
- d. All Nurse Navigation dispositions shall be documented in CAD and included in EMS Agency quality improvement review.

Table 1.

Response Level	Capability (Level of Care Required)	Clinically-Indicated Response Resource(s) and Priorities		Rationale
		First Responder	Ambulance	
ECHO	ALS	Red Lights & Siren	Red Lights & Siren	- Information exists that death is imminent - Dispatch closest First Response Unit and ALS Ambulance - Immediate lights and siren response required - Multiple resources are likely necessary - Rapid ALS transport almost certain
DELTA	ALS	Red Lights & Siren	Red Lights & Siren	- Life-threatening conditions exist - Dispatch closest First Response Unit and ALS Ambulance - Immediate lights and siren response required - Multiple resources are likely necessary - ALS transport likely
CHARLIE Urban	ALS	Optional Response - No Red Lights & Siren	Red Lights & Siren	- Condition is potentially life-threatening - Dispatch ALS Ambulance - Dispatch of First Response Unit recommended for extended ETAs - Multiple resources may not be necessary - ALS or BLS transport likely
CHARLIE Rural	ALS	Red Lights & Siren	Red Lights & Siren	
BRAVO Urban	ALS	Optional Response - No Red Lights & Siren	Red Lights & Siren	- Condition is unlikely to be life-threatening - Dispatch ALS Ambulance - Dispatch of First Response Unit recommended for extended ETAs - Ambulance transport less likely
BRAVO Rural	ALS	Red Lights & Siren	Red Lights & Siren	
ALPHA	Closest BLS, ALS, or referral to 9-1-1 Nurse Navigation	Optional Response- No Red Lights & Siren	No Red Lights & Siren	- Condition is likely not life-threatening - Dispatch BLS or ALS Ambulance or refer patient to 9-1-1 Nurse Navigation as appropriate - First Response Unit not recommended - Multiple resources likely unnecessary

County of Monterey EMS System Policy 3050

	(for select determinants)			Ambulance transport unlikely
OMEGA	BLS or Referral to 9-1-1 Nurse Navigation (for select determinants)	Not indicated	No Red Lights & Siren	- Dispatch BLS ambulance or refer patient to 9-1-1 Nurse Navigation as appropriate - Ambulance transport unlikely

County of Monterey EMS System Policy



Policy Number: 4000

Effective Date:

Review Date:

EMERGENCY MEDICAL SCENE MANAGEMENT

I. PURPOSE

To establish guidelines for the management of an emergency medical scene.

II. POLICY

A. Authority for Scene Management

1. California Health and Safety Code Section 1798.6, “Authority for the management of the scene of an emergency shall be vested in the appropriate public safety agency that has investigative authority. The scene of an emergency shall be managed in a manner to minimize the risk of death or health impairment to the patient and to other persons who may be exposed to the risks as a result of the emergency condition, and priority shall be placed upon the interests of those persons exposed to the more serious and immediate risks of life and health. Public Safety officials shall consult emergency medical services personnel or other authoritative health care professionals at the scene in the determination of relevant risks.”

B. Authority for Patient Health Care Management

1. California Health and Safety Code Section 1798.6, “Authority for patient health care management in an emergency shall be vested in that licensed or certified health care professional, that may include any paramedic or other prehospital emergency personnel, at the scene of the emergency who is most medically qualified specific to the rendering of emergency medical care. If no licensed or certified health care professional is available. The authority shall be vested in the most appropriate medically qualified representative of public safety agencies who may have responded to the scene of the emergency.”

III. PROCEDURE

A. Scene management or incident organization should be based on the Incident Command System (ICS) and established ICS procedures.

1. The first arriving unit on-scene shall assume the role of Incident Commander (IC).
 - a. The IC shall immediately assess the scene and situation. The IC shall inform County Communications of the following:
 - 1) Traffic and/or other hazards
 - 2) Approximate number of victims
 - 3) Necessity of additional resources
 - 4) Any extrication difficulties

- 5) Known/suspected presence of hazardous materials
 - b. All requests for additional resources shall be directed to the County of Monterey Communications Center.
2. Disagreements pertaining to triage or patient care arising at the scene of an emergency shall be resolved in one of two manners:
 - During an MCI, triage disputes shall be addressed by the Treatment Group Supervisor.
 - During normal medical incidents, treatment disputes shall be resolved by base consultation. These discussions should not be conducted in the presence of either the patient or the general public.
3. Rescue and extrication will be performed by appropriate agencies, as necessary. Trapped patients shall be treated to the extent possible to prevent a deterioration in the patient's condition while extrication is being performed.
4. For incidents involving multiple patients, medical care and scene management shall be handled in accordance with guidelines established in the Monterey County Multi-Casualty Incident (MCI) Plan when applicable.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 4012

Effective Date:

Review Date:

CONTROLLED SUBSTANCES

I. PURPOSE

To establish minimum requirements for compliance with the Controlled Substances Act and California statutes and regulations by County of Monterey authorized Paramedic Service Provider agencies.

II. POLICY

A. The following medications are considered “controlled substances” and are approved for use in the County of Monterey EMS system:

1. Schedule II - Fentanyl; Morphine Sulfate
2. Schedule III - Ketamine; Buprenorphine
3. Schedule IV - Midazolam

B. All Paramedic Service Provider organizations will have a formal agreement with a DEA registrant who is accountable for the agency or service’s compliance with the Controlled Substances Act.

1. The DEA registrant will maintain a separate DEA registration number for each agency or service that they affiliate with; separate from the DEA registrant’s own practice and separate from any other legal entity.
2. The DEA registrant will establish policies and procedures, which are compliant with this policy, for each agency or service they serve.

C. The Paramedic Service Provider organization will designate personnel authorized to manage controlled substances for the organization.

III. PROCEDURE

A. Security Mechanisms and Procedures

1. Ordering and Order Tracking

- a. Each agency or service will order controlled substances from a DEA registered distributor or pharmacy.
- b. Schedule II controlled substances require use of the DEA Form 222 or the Controlled Substance Ordering System (CSOS). These orders may only be delivered to the agency or service’s facility found at the single physical location and address noted on the DEA license.
- c. Each order must be tracked in a manner that documents the parties requesting, ordering, and receiving controlled substances.

2. Receipt and Accountability

- a. Controlled substances must be received at the agency or service's facility found at the single physical location and address noted on the DEA license.
- b. Personnel receiving controlled substances must be authorized by the DEA registrant and included on the agency or service's roster of personnel authorized to manage controlled substances.
- c. A witness, also included on the roster of personnel authorized to manage controlled substances, must participate in the receipt and its documentation.
- d. The receipt of controlled substances will be documented in the master supply log(s) including: the date and time, the name of the medication, the concentration, the quantity of vials, the expiration date, the manufacturer, the lot number, and the receiving party and the witness, including their signatures.

3. Master Supply Storage, Security and Documentation

- a. The master supply storage of controlled substances will be at the agency or service's facility found at the single physical location and address noted on the DEA license.
- b. Controlled substances must be stored in a "securely locked, substantially constructed cabinet" as required under the Controlled Substances Act.
- c. Follow the manufacturer's guidelines regarding storage of each controlled substance:
 - 1) Store within the required temperature range
 - 2) Protect from light as required
- d. Master supply security measures will include:
 - 1) Tamper evident containers
 - 2) Witnessed counting; no less than once each month
 - 3) Tracking of access to the master supply by individual for each instance of access to the master supply.
- e. Personnel handling and/or counting controlled substances at the master supply must be authorized by the DEA registrant and included on the agency or service's roster of personnel authorized to manage controlled substances. A witness, included on the roster of personnel authorized to count controlled substances, must also participate in each count and its documentation.
- f. Master supply documentation will include:
 - 1) The agency or service's roster(s) naming personnel authorized to:

- a) Manage controlled substances
 - b) Count controlled substances
 - c) Administer controlled substances
 - d) Audit controlled substances
- 2) The original of each DEA Form 222, including voided forms; purchase records; a log(s) of all controlled substances ordered, received, stored, damaged during storage, placed into service, damaged while in service, administered, wasted, restocked, returned to master supply, reverse distributed; and an electronic patient care record (ePCR) or other appropriate report corresponding to each administration, waste, damage, or expiration
- 3) These records will be:
 - a) Maintained at and/or electronically accessible from the master supply location
 - b) Available for inspection within 24 hours by the EMS Agency
 - c) Maintained for a period of no less than two years (older records should be shredded)
- 4. Controlled Substance Labeling and Tracking
 - a. Controlled substances must remain in the original manufacturer's containers, Food and Drug Administration (FDA) compliant labels remaining intact and unaltered, until the time of administration.
 - b. Tracking of Controlled substances will include documentation in the log(s) as described throughout this policy; including: the date and time of each transaction, the name of the medication, the concentration, the quantity of vials, the expiration date, the manufacturer, the lot number, and the party(ies) involved, including signature(s). Additional methods of tracking are encouraged.
- 5. Vehicle Storage and Security
 - a. Make every reasonable attempt to follow the manufacturer's guidelines regarding vehicle storage of each controlled substance while in service:
 - 1) Avoid exposure to temperature extremes
 - 2) Protect from light as required
 - b. Vehicle storage security measures will include:
 - 1) Tamper evident containers
 - 2) Witnessed counting with each change in personnel or change of shift; no less than once each day (24 hours)
- 6. In-Service Record Keeping

- a. Personnel handling and/or counting controlled substances while in service must be authorized by the DEA registrant and included on the agency or service's roster of personnel authorized to administer controlled substances.
- b. A witness, preferably included on that same roster, but, at minimum, included on the roster of personnel authorized to count controlled substances, must also participate in each transaction and its documentation.
- c. Documentation while in service will include:
 - 1) A log(s) of all controlled substances accepted into service, counted, damaged while in service, received as re-stock, and/or returned to master supply
 - 2) These records will be:
 - a) Maintained with the medications until submitted to and/or electronically accessible from the master supply location
 - b) Available for inspection within 24 hours by the EMS Agency
 - c) Submitted to master supply at least monthly
 - d) Maintained as master supply documentation for a period of no less than two years

7. Usage Procedures and Documentation

- a. Controlled substances will be administered by ALS personnel only as authorized per EMS policy and protocols currently in effect at the time of use. Personnel administering controlled substances must be authorized by the DEA registrant and included on the agency or service's roster of personnel so authorized.
- b. Usage documentation will include:
 - 1) A log(s) of all controlled substances administered
 - 2) An ePCR corresponding to each administration
 - 3) These records will be:
 - a) Maintained with the medications until submitted to and/or electronically accessible from the master supply location
 - b) Available for inspection within 24 hours by the EMS Agency
 - c) The corresponding PCR must be accessible for matching with these records
 - d) Maintained as master supply documentation for a period of no less than two years

8. Reverse Distribution

- a. Each agency or service will send expired and/or damaged controlled substances to an authorized reverse distributor.
 - 1) Schedule II controlled substances must be transferred using the DEA's Form 222
 - 2) Schedule III – V controlled substances may be transferred by invoice.
 - 3) These reverse distributions will be sent to the reverse distributor's facility found at the single physical location and address noted on the reverse distributor's DEA registration.
 - 4) Each reverse distribution must be tracked in a manner that documents the parties sending and receiving the expired and/or unwanted controlled substances.
- b. Personnel sending controlled substances for reverse distribution must be authorized by the DEA registrant and included on the agency or service's roster of personnel authorized to manage controlled substances.
 - 1) A witness, also included on the roster of personnel authorized to manage controlled substances, must participate in the shipment and its documentation.
- c. All reverse distribution will be documented in the master supply log(s) including: the date and time, the name of the medication, the concentration, the quantity of vials, the expiration date, the manufacturer, the lot number, and the sending party and the witness, including their signatures.

9. Disposal

- a. Disposal of expired and/or unwanted controlled substances will be performed as described above under "Reverse Distribution."
- b. Disposal of controlled substances residual to patient administration, wasting, will be performed following the agency or service's internal policy.
 - 1) This policy must include a method of wasting which renders the remaining medication non-retrievable as defined by the DEA.
 - 2) Disposal in a sharps container and sewerage of controlled substances are both methods which do not meet the DEA requirement.
 - 3) Process for disposition of the empty, or partially empty, narcotics container shall be described in the policy.

- c. Personnel wasting controlled substances must be authorized by the DEA registrant and included on the agency or service's roster of personnel authorized to administer controlled substances.
 - 1) A witness, preferably included on that same roster, but, at minimum, included on the roster of personnel authorized to count controlled substances, must also participate in each waste and its documentation.
 - 2) The witness shall observe the wasting of the substance from its original container.
- d. Wasting documentation will include:
 - 1) A log(s) of all controlled substances wasted
 - 2) An ePCR corresponding to each waste
 - 3) These records will be:
 - a) Maintained with the medications until submitted to and/or electronically accessible from the master supply location
 - b) Available for inspection within 24 hours by the EMS Agency
 - c) Submitted to master supply at least monthly and maintained for a period of no less than two years

10. Re-Stocking Procedures

- a. Re-stocking of controlled substances will be performed following the agency or service's internal policy that will include at minimum verification of administration, waste, damage, and/or expiration.
- b. If an agency or service chooses to require the retention and transport of used and/or damaged containers and/or sharps for restock purposes, internal policies will include the use of appropriate sharps containers if a used sharp is retained following administration.
- c. Personnel providing re-stock of controlled substances must be authorized by the DEA registrant and included on the agency/service's roster of personnel authorized to manage controlled substances.
- d. Personnel receiving re-stocked controlled substances must be authorized by the DEA registrant and included on the agency/service's roster of personnel authorized to administer controlled substances.
 - 1) Both parties must participate in and document the re-stocking.
- e. Re-stocking documentation will include:
 - 1) A log(s) of all controlled substances restocked
 - 2) An ePCR or other appropriate report corresponding to each administration, waste, damage, or expiration

3) These records will be:

- a) Maintained at and/or electronically accessible from the master supply location
- b) Available for inspection within 24 hours by the EMS Agency
- c) Maintained for a period of no less than two years

11. Transfer or Exchange Between Agencies and/or Services

- a. The transfer or exchange of controlled substances between agencies and/or services is highly discouraged. There are additional federal requirements and documentation necessary for this process to take place; the process has proven problematic when done in other areas. If a provider experiences or expects to experience a shortage of a controlled substance, that provider should contact the EMS Agency as per County of Monterey EMS System Policy # 4011 (Medication/Solution Shortages).

B. Investigation and Mitigation of Suspected Tampering or Diversion

- 1. Drug inventories and all related records are subject to inspection by the EMS Agency, the California EMS Authority (EMSA), the California State Board of Pharmacy, the Federal Drug Enforcement Administration, and the Justice Department's Bureau of Narcotic Enforcement.

2. Controlled Substance Testing

- a. Testing personnel for controlled substances may be performed following the agency or service's internal policy.
- b. Such policies may provide for controlled substance testing that is random, routine, or in response to suspected tampering and/or diversion.
- c. Any such policy should be developed in consultation with the DEA Registrant and legal counsel.

3. Discrepancy Reporting

- a. Each agency or service will follow its internal policy for reporting discrepancies including tampering, theft, loss, or diversion of controlled substances.
- b. This policy will be established by the DEA Registrant and must include immediate verbal reporting followed by written reports and investigation.
- c. The DEA Registrant must notify the DEA of the discrepancy within one business day of discovery using DEA Form 106, "Report of Theft or Loss of Controlled Substances."
- d. In addition, the EMS Agency must be notified within eight (8) hours utilizing the Unusual Occurrence reporting system as per County of Monterey EMS System Policy # 6020 (Unusual Occurrence Reporting).

4. Tampering, Theft and Diversion Prevention and Detection

- a. Each agency or service's internal policy regarding controlled substances will comply with this policy; with the intent to prevent and detect the tampering, theft, loss, and/or diversion of controlled substances.
- b. Areas to be addressed include: ordering and order tracking, receipt and accountability; master supply storage, security, and documentation; labeling and tracking, vehicle storage and security, usage procedures and documentation, reverse distribution, disposal, re-stocking procedures, transfer or exchange of controlled substances between agencies and/or services, controlled substance testing, discrepancy reporting; tampering, theft and diversion prevention and detection; and usage audits.
- c. Reporting the suspected tampering, theft, and/or diversion of controlled substances to local law enforcement is required.
 - 1) If the tampering, theft, and/or diversion of controlled substances is substantiated, written reports must be made within eight (8) hours to the EMS Agency and EMSA for action against the responsible party's certification, license, or accreditation.

5. Usage Audits

- a. Each agency or service will follow its internal policy for usage audits. These audits will:
 - 1) Be conducted by the DEA registrant/designee
 - a) Any such designee must be authorized by the DEA registrant and included on the agency or service's roster of personnel authorized to audit controlled substances
 - 2) Account for the current disposition of all controlled substances
 - a) Include review of forms, purchase records, logs, and ePCRs
 - b) Identify and report discrepancies as required
 - 3) Identify and investigate unusually high rates of administration
 - a) Establish a baseline for the rate of controlled substance administration among all individuals authorized to administer controlled substances during the time-period being audited
 - b) Identify high outliers, individuals with high rates of controlled substance administration
 - c) Review each administration of controlled substances performed by these high outliers for accountability and clinical appropriateness
 - 4) Be performed at least quarterly
- b. Records of these audits will be:

- 1) Maintained at and/or electronically accessible from the agency or service's quality assurance location
- 2) Available for inspection within 24 hours by the EMS Agency
- 3) Maintained for a period of no less than two years

6. Biennial On-Hand Inventory

- a. In addition to daily and other routine running inventories, the DEA requires a separate Biennial On-Hand Inventory. This inventory includes all controlled substances, including those in a central facility and those which have been deployed. It also includes those in the reverse distribution process. It is a "snapshot" count in time. These counts should be scheduled by every provider in such a way that they are not forgotten.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 4060

Effective Date:

Review Date:

PHYSICIAN ON SCENE

I. PURPOSE

This policy provides direction for prehospital personnel when, at the scene of an injury or illness, a bystander identifies themselves as a physician and attempts to be involved in the care of the patient.

II. PROCEDURE FOR PHYSICIAN INVOLVEMENT

- A. If a bystander identifies themselves as a physician and wants to take over care of the patient, prehospital personnel should show the physician the card issued by the State of California entitled “**Note to Physician on Involvement with Prehospital Personnel**” (attached) and ask the physician to select one of the three options provided.
 1. If the physician on scene selects **OPTION 1**, medical control remains with the protocols established by the Monterey County EMS Agency Medical Director and, as needed, the Base Hospital physician.
 2. If the physician on scene selects **OPTION 2** and requests to talk to the base station physician, prehospital personnel shall:
 - a. Ask for the physician’s full name and California medical license number and document this information on the PCR.
 - b. Immediately contact the Base Hospital and allow the physician on scene to speak with the Base Hospital physician. If there is disagreement between the physician on scene and the Base Hospital physician regarding the medical care required, the Base Hospital physician will maintain authority.
 3. If the physician on scene selects **OPTION 3**, or if the physician on scene performs a procedure or continues to engage in direct patient care after the arrival of EMS, prehospital personnel shall:
 - a. Ask for the physician’s full name and California medical license number and document this information on the PCR.
 - b. Make equipment and supplies available to the physician and assist the physician as needed.
 - c. Ensure that the physician accompanies the patient in the ambulance to the receiving hospital.
 - d. Ensure that the physician signs the EMS Patient Care Report for all instructions and medical care given.

- B. Once EMS personnel have arrived at the scene and assumed care of the patient, only California-licensed physicians may participate in the medical care of the patient. Physician Assistants (PAs), Nurse Practitioners (NPs), Registered Nurses (RNs), and other non-physician medical providers are not authorized to perform procedures, direct medical care, or assume responsibility for the care of a patient being attended by EMS personnel.



STATE OF CALIFORNIA

California Medical Association

NOTE TO PHYSICIAN ON INVOLVEMENT WITH PREHOSPITAL PERSONNEL

A life support team (PARAMEDIC) operates under standard policies and procedures developed by the local EMS agency and approved by their Medical Director under the Authority of Division 2.5 of the California Health and Safety Code. The drugs they carry and procedures they can do are restricted by law and local policy.

If you want to assist, this can only be done through one of the alternatives listed on the back of this card. CMA, State EMS Authority, CCLHO, and BMQA have endorsed these alternatives.

Assistance rendered in the endorsed fashion, without compensation, is covered by the protection of the "Good Samaritan Code" (see Business and Professions Code, Sections 2144, 2395-2398 and Health and Safety Code, Section 1799.104).

(over)

ENDORSED ALTERNATIVES FOR PHYSICIAN INVOLVEMENT

After identifying yourself by name as a physician licensed in the State of California, and, if requested, showing proof of identity, you may choose to do one of the following:

1. Offer your assistance with another pair of eyes, hands, or suggestions, but let the life support team remain under base hospital control; or,
2. Request to talk to the base station physician and directly offer your medical advice and assistance; or,
3. Take total responsibility for the care given by the life support team and physically accompany the patient until the patient arrives at a hospital and the receiving physician assumes responsibility. In addition, you must sign for all instructions given in accordance with local policy and procedures. (Whenever possible, remain in contact with the base station physician).

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 4061

Effective Date:

Review Date:

INTERFACILITY TRANSFER-PHYSICIAN ORDERS

I. PURPOSE

To establish guidelines for patient management during an interfacility transport, and how to carry out physician orders.

II. POLICY

- A. All paramedics and EMTs function under the direction of the EMS Medical Director through established policies, protocols, and procedures, and through medical direction provided by the Base Hospital physician.
- B. Paramedics and EMTs shall carry out written orders from a California-licensed physician for patient care during an interfacility transfer, with the following exceptions:
 - 1. Orders that exceed the County of Monterey scope of practice for the paramedic or EMT.
 - 2. The patient's condition had changed, and continuation of the orders risks harming the patient.
- C. Physician orders and/or patient care needs that exceed the scope of practice for the EMS providers shall be provided by a healthcare provider who is trained, authorized, and credentialed to provide such care during the interfacility transfer.
- D. If needed, medical direction from a Base hospital physician shall be obtained.

III. PROCEDURE

- A. Upon arrival at the sending facility, EMS personnel shall consult with the medical staff and shall review any physician orders pertaining to the interfacility transfer.
- B. If a patient has written orders for care that exceed their scope of practice, EMS personnel shall promptly notify medical staff at the sending facility of the issue. The transfer may proceed if the orders are changed to be within the EMS provider's scope of practice, or if a clinical provider (nurse or physician) authorized to carry out the orders accompanies the patient during transport. If neither of these accommodations are feasible, the appropriate hospital staff shall discuss the situation with the EMS Supervisor, who will work toward a solution. If needed, the EMS Agency Duty Officer may also be contacted.
- C. If the treatment ordered is within the County of Monterey scope of practice but requires Base Hospital physician authorization, EMS personnel shall contact the Base Hospital physician for authorization prior to initiation of the interfacility transfer.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 4070
Effective Date: 7/1/2026
Review Date: 6/30/2028

EMS AIRCRAFT OPERATIONS

I. PURPOSE

- A. To establish guidelines and standards of operation for the utilization of EMS Aircraft within the County of Monterey EMS System.
- B. To establish minimum standards for the integration of EMS Aircraft and personnel into the local EMS prehospital patient transport system as a specialized resource for the transport and care of emergency medical patients.

II. POLICY

- A. EMS Aircraft service providers and personnel shall follow the standards established by California Code of Regulations Title 22, along with the County of Monterey EMS policies regarding patient transport and destination.
- B. EMS aircraft must be authorized by Monterey County EMS Agency to provide prehospital patient transport within Monterey County. Authorized EMS aircraft service providers shall comply with this and other Monterey County EMS Policies related to provision of air transport for emergency patients. The classification of EMS Aircraft by the EMS Agency shall be included as part of the EMS Plan.
- C. Patients shall be transported to the hospital via ground ambulance unless such transport is unavailable or if ground transport is significantly longer than air transport (and this difference in time may negatively impact the patient's condition).
- D. The request for activation of an EMS aircraft should consider:
 - 1. Whether the patient will receive a time to arrival at the destination benefit over ground ambulance transport.
 - 2. Patient needs, such as advanced airway management, that may be more promptly delivered by EMS aircraft personnel than if the patient were transported by ground and the patient's condition requires the intervention for survival.
- E. The request for an EMS aircraft shall be from one of the following:
 - 1. A request from the scene of an incident shall be made through Incident Command.
 - 2. Responding fire, EMS, and/or law enforcement may request an EMS aircraft response prior to arrival of any responders.

3. Emergency Communications Personnel who determine that the nature of the call or the condition of the patient meets auto-launch criteria established by the EMS Agency may request an EMS aircraft response.
- F. The medical flight crew of an EMS aircraft shall have training in aeromedical transportation as specified and approved by the authorizing EMS agency.
- G. EMS Aircraft service providers shall orient their pilots and medical flight crews to the local EMS system.
- H. An EMS Aircraft shall not be used as a replacement for a ground ambulance. A ground ambulance shall always be dispatched for scene calls. The ground ambulance shall not be cancelled until the air ambulance is off the ground and en-route to the destination hospital.
- I. In accordance with County of Monterey EMS Agency policies, all EMS aircraft shall have the capability of communicating with:
 1. Designated dispatch center(s)
 2. EMS ground units
 3. Designated base hospitals
 4. Receiving hospitals
 5. Other facilities or agencies that the County of Monterey EMS Agency deems appropriate

III. PATIENT DESTINATION DECISIONS, ADULT AND PEDIATRIC TRAUMA

- A. Adult patients who meet criteria for classification as a Major Trauma Patient (MTP) under the Field Trauma Triage Criteria policy are to be transported to Natividad or the closest most appropriate Level I or Level II trauma center.
- B. Air transport should be considered when the anticipated transport time by ground exceeds 45 minutes. Anticipated transport times must be determined on a case-by-case basis. The decision must consider travel times, weather, road conditions, traffic volume, availability and location of air ambulance resources, ongoing extrication or patient access, responding ambulance ETA, and other pertinent factors.
- C. Pediatric MTPs shall be transported to a pediatric trauma center or to the destination ordered by Natividad.
- D. When possible, the landing zone (LZ) should be established in a location that is closer to Natividad rather than transporting the patient away from Natividad should air transport become unavailable.
- E. Patients requiring emergency airway management that cannot be successfully performed by EMS or EMS Aircraft providers shall be transported to the closest, most accessible emergency department.
- F. Base hospital contact with Natividad is mandatory if initial rendezvous is aborted or redirected.

IV. PATIENT DESTINATION DECISIONS, MEDICAL PATIENTS

- A. For patients whose emergency medical condition requires emergency (Code 3) transport to a hospital, air transport should be considered if:
 - 1. The destination hospital has an on-site helicopter landing pad and the anticipated transport time by ground exceeds 45 minutes, or
 - 2. The destination hospital does not have an on-site helicopter landing pad and the anticipated transport time by ground exceeds 60 minutes.
- B. The decision about whether air transport is appropriate must be determined on a case-by-case basis. Considerations should include travel times, weather, road conditions, traffic volume, availability and location of air ambulance resources, responding ambulance ETA, and other pertinent factors.
- C. When possible, the LZ should be established in a location that is closer to a local hospital rather than transporting the patient away from a local hospital should air transport become unavailable.
- D. Patients in extremis or with an uncontrolled airway shall be transported to the closest, most accessible emergency department.

V. PROCEDURES

- A. Activation of an EMS aircraft response shall follow the requirements found in this policy.
- B. All requests for EMS Aircraft shall be made through the County of Monterey Emergency Communications Department (ECD) or by the CALFIRE Dispatch Center through the ECD.
- C. The following information shall be given to the ECD when requesting EMS Aircraft:
 - 1. Agency name and dispatch frequency of agency requesting helicopter;
 - 2. Location and the number of patient(s);
 - 3. Type and extent of injuries, if known;
 - 4. Request for a hoist or other specialized capabilities, if needed;
 - 5. Designated Landing Zone;
 - 6. General location of nearest landing site (Thomas Bros. Page and GPS coordinates and/or significant landmarks identifiable from the air); and,
 - 7. General weather/visibility conditions.
- D. The ECD shall request an estimated time of arrival (ETA) when notified that an EMS aircraft will accept the mission. This ETA shall be provided to the Incident Commander.
- E. EMS Aircraft should not accept a mission unless there is the reasonable expectation that they will be able to complete the transport to the destination hospital. Factors including weather, fuel, or other considerations shall be taken into account.
 - 1. Any delay in response of the EMS aircraft shall be reported by the flight crew to the Incident Commander. Notification shall include the reason for delay, ETA,

and any pertinent information to assist the IC in determining whether another EMS aircraft should be requested or alternate means of patient transport should be utilized.

F. EMS Aircraft auto-launch shall be initiated by ECD in instances that meet the following criteria and are within the EMS Agency's designated auto-launch area:

1. Penetrating trauma to the head or trunk
2. Burns to the head or trunk
3. Fall greater than 20 feet
4. Explosion with suspected injuries to people present
5. Electrocution
6. MCI
7. Industrial/Agricultural accident with loss of consciousness or major injuries suspected
8. Vehicle versus Pedestrian/Bicycle/ATV/Motorcycle with loss of consciousness or major injuries suspected
9. MVC with loss of consciousness or major injuries suspected
10. Watercraft collision with loss of consciousness, near drowning, or major injuries suspected

G. Communications:

1. The ECD shall request the closest EMS Aircraft, according to the matrix established by the EMS Agency, be dispatched unless the Incident Commander determines the use of a Rescue Aircraft is appropriate.
2. The ECD shall advise all responding ground and air units of EMS Aircraft dispatch, estimated time of arrival, radio frequency, and/or cancellation.
3. CALCORD is the preferred channel for ground-to-helicopter communication. The Incident Commander or designee, in consultation with the ECD, can identify other radio frequencies to be used.
4. The EMS Aircraft shall contact the ECD when enroute to the scene to ascertain the radio frequency and contact person to be used for air-to-ground communication.
5. The EMS Aircraft shall notify the receiving facility of their impending arrival as soon as possible prior to arrival. This notification is to include patient information and an ETA.
6. EMS Aircraft providers shall notify the ECD if a ground ambulance is needed from the terminal airport or LZ to the receiving hospital.
7. Ambulance crews may talk directly with fire crews for coordination of EMS Aircraft requests. The ECD shall advise the EMS Aircraft Dispatch Center(s) if multiple aircraft are providing support to the incident.

H. Cancellation of EMS Aircraft

1. Only the Incident Commander may cancel the EMS Aircraft from the scene of an emergency. This decision should be made after consultation with the ground ALS provider and when it is determined that transport is no longer needed or ground transport is appropriate.
2. The Incident Commander's decision to cancel the requested EMS Aircraft should be based upon the total pre-hospital time from dispatch to arrival at the hospital, patient's medical condition, severity of injury, and/or safety and environmental conditions.
3. The EMS Aircraft crew may transfer responsibility for patient care to a ground ambulance crew if no interventions beyond the County of Monterey Paramedic Scope of Practice have been utilized, and if both parties are in agreement.
4. EMS aircraft personnel may cancel their response when the crew has determined that safety of the aircraft, personnel, and/or the patient may be compromised by continuing the mission.

I. Air and Ground Ambulance Rendezvous:

1. In the event that an EMS Aircraft is unable to land at the scene, consideration shall be given by the Incident Commander or designee to arrange for a rendezvous at a pre-designated LZ. The ground ALS transport provider shall be consulted.
2. Following Initiation of Ambulance Transport:
 - a. If patient transport is initiated by the ground ambulance but the original ground ETA becomes extended, rendezvous at an approved LZ may be initiated by ambulance personnel when the patient's condition deteriorates, traffic conditions are prohibitive, or the unit develops mechanical failure. ECD shall be notified.
 - b. If circumstances preclude EMS Aircraft from landing at an approved LZ, the ground ALS transport provider should request a public safety agency response to assist in securing and preparing the selected LZ.
3. Only personnel trained in EMS Aircraft landing procedures shall be utilized to determine the LZ and assist in landing the EMS Aircraft.

J. Medical Control

1. Each EMS Aircraft provider shall have a designated Flight Medical Director.

K. Aircrafts that do not have a medical flight crew shall not transport patients unless accompanied by medically-qualified ground EMS personnel. Ground ambulance personnel shall obtain approval from the on-duty supervisor prior to transport.

L. Helicopter Safety:

1. All first responder and ambulance provider personnel shall be trained in helicopter safety and landing procedures.

2. Scene security is of prime importance. The agency coordinating the landing shall ensure the LZ is secure and safe.
3. The EMS Aircraft pilot shall have final authority as to the safe operation of the air transport. If the pilot deems that patient transport by EMS Aircraft would be unsafe, the patient will be transported by ground ambulance.
4. Ground personnel shall not approach the EMS Aircraft unless directed to do so by EMS Aircraft crew. When approaching or departing the helicopter, ground personnel shall not lift anything higher than their head nor shall they at any time approach or depart a helicopter from the rear or from the uphill side when the aircraft is landed on a slope.
5. Hats, helmets, turnouts, or any loose items are to be secured or removed prior to approaching the helicopter. Any removable objects shall be stripped from the gurney when approaching the helicopter.

VI. AUTHORIZATION OF AN AIR AMBULANCE

- A. All EMS Aircraft and Air Rescue service providers—including companies, lessees, agencies (excluding federal agencies), owners, operators, and hospitals that base, house, or station EMS aircraft either permanently or temporarily—must comply with all applicable federal, state, and local laws, regulations, policies, and procedures governing EMS aircraft operations. This includes requirements related to flight crew qualifications and aircraft maintenance.
- B. EMS Aircraft service providers that want to operate within the County of Monterey shall submit a written request to the EMS Agency. The request shall also include the classification and location of aircraft to be available to Monterey County.
- C. Within five (5) business days following transport, EMS Aircraft service providers shall provide the County of Monterey EMS Agency with copies of all ePCRs for patients transported within the County of Monterey. EMS Aircraft ePCR's will be reviewed by EMS Agency staff on a regular basis.

VII. FORMAL COMPLAINTS

- A. Complaints regarding the integration of EMS Aircraft into the EMS system shall be handled by the County of Monterey EMS Agency.
- B. EMS Aircraft service providers shall follow County of Monterey EMS System Policy #6020 (Unusual Occurrence Reporting) for reporting of all complaints.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 4080
Effective Date: 7/1/2026
Review Date: 6/30/2029

EMT PATIENT CARE

I. PURPOSE

To authorize and provide a procedure for an EMT to be the primary patient care provider when working with a paramedic on an ALS ambulance.

II. POLICY

- A. Transfer of patient care from a paramedic to an EMT may only occur after the paramedic has performed an initial assessment and deemed the patient to be safe and appropriate for BLS care.
- B. The patient must not have any cardiac, respiratory, traumatic, or neurological complaints that may warrant an ALS intervention(s) on scene or during transport. No patient will be turned over from an ALS provider to a BLS provider once ALS interventions have been initiated.
- C. At the scene of a declared MCI, BLS ambulances may be utilized when the patient is not expected to need care which exceeds the scope of practice of the EMT.

III. UTILIZATION GUIDELINES

- A. EMTs may provide primary patient care when reasonable care has been taken to ensure that no ALS interventions or assessments are anticipated during patient transport.
- B. Paramedics shall be responsible for the initial patient assessment and care provided to the patient until the transfer of care occurs to the EMT. If it is discovered during the initial assessment there were signs that indicated a need for ALS care, that paramedic shall be held responsible.
- C. EMTs may monitor the following treatments/medical devices:
 - 1. IV lines and the IV infusion of either glucose solutions or isotonic balanced salt solutions, including Ringer's Lactate. The EMT may only monitor, maintain, and adjust if necessary to maintain a preset rate of flow or turn off the flow of IV fluid.
 - 2. Nasogastric (NG) tubes or gastrostomy tubes.
 - 3. Foley catheters.
 - 4. Tracheostomy tubes.

IV. AMBULANCE SERVICE PROVIDER RESPONSIBILITIES

- A. The ambulance service provider shall ensure that the EMT functioning under this policy is familiar with and has a working knowledge of this policy.
- B. The ambulance service provider shall notify the EMS Agency immediately if a patient being transported under this policy clinically worsens, if patient care responsibilities are taken over by the paramedic, and/or if the patient is diverted to a specialty center.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 4090
Effective Date: 7/1/2026
Review Date: 6/30/2029

TRANSFER OF PATIENT CARE

I. PURPOSE

This policy establishes guidelines for transferring patient care in the prehospital setting.

II. POLICY

- A. EMS personnel shall seek to ensure continuity of patient care through effective communication of patient history, physical exam and patient assessment findings, diagnostic signs, treatment provided, patient response, and any other relevant information to the healthcare providers taking over patient care responsibilities.

III. PROCEDURE

- A. When initial care is under an Emergency Medical Responder (EMR) or EMT:
1. The first ALS-level provider arriving at the scene shall be responsible for contacting the EMR/EMT and ensuring the coordination of care. The EMR/EMT shall give the ALS provider a hand-off report describing the patient's condition and care rendered prior to the arrival of the ALS provider. If the patient requires ALS-level care, or if the ALS provider will be transporting the patient, the ALS provider shall assume responsibility for patient care.
- B. When initial care is under a paramedic:
1. The initial paramedic provider may either transfer patient care to the transport paramedic or maintain primary patient care. The decision to transfer care shall be agreed upon by both providers and communicated verbally. If the initial paramedic provider does not communicate the decision, the transport paramedic shall confirm which paramedic will have primary patient care responsibility.
 2. The transport paramedic is to assist with patient care should the initial paramedic provider maintain primary care.
 3. If the transporting paramedic wants to transfer care to a transporting EMT, a full assessment shall be completed. For this transfer to occur, the following criteria must be met:
 - The patient must not have any cardiac, respiratory, traumatic, or neurological complaints that may warrant an ALS intervention(s) on scene or during transport. No patient will be turned over once ALS interventions have been initiated.
 - The EMT must agree to take over patient care.
 - The patient must be clinically and hemodynamically stable.

County of Monterey EMS System Policy 4090

4. If the patient is to be transported by air ambulance, the initial paramedic shall transfer care to the ALS flight crew.
 5. If a conflict regarding patient care occurs that cannot be resolved amongst the personnel, contact the base hospital physician for direction.
- C. EMS System first responders shall locate, assess, and treat patients prior to the arrival of EMS transport providers. To maximize patient safety and to ensure adequate assistance for EMS transport providers, first responders shall remain on scene to assist EMS transport providers until verbally cleared by EMS transport providers to return to service. When responding to healthcare facilities such as physician offices and skilled nursing facilities, first responders shall remain at the scene to provide assistance until cleared by EMS transport providers, even if facility staff advise first responders that their services are not required.
- D. Whenever one EMS provider assumes care from another, both individuals must complete an ePCR as outlined in EMS Policy #6180. The time of transfer of care should be documented on the ePCR.
- E. Any paramedic unit taking over patient care from another paramedic unit should, whenever possible, resupply the first paramedic unit according to the ambulance provider's resupply offer.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 4110

Effective Date:

Review Date:

ORGAN DONOR

I. PURPOSE

To establish guidelines for Emergency Medical Services (EMS) personnel to meet the requirements to search for organ donor information on patients for whom death appears imminent.

II. POLICY

- A. Section 7150.55 of the Health and Safety Code, Division 7, Chapter 3.5 requires emergency medical personnel to make a reasonable search for a document of anatomical gift, or other information identifying the patient as a donor or an individual who has refused to make an anatomical gift. Sections 7150.10 and 7150.15 also apply to this policy.
- B. Emergency medical treatment and transport of the patient is the highest priority and a search for organ donor authorization/refusal shall not interfere with patient care activities.
- C. EMS personnel shall attempt, when practical, to locate documentation that indicates whether an individual desires to be an organ or tissue donor.

III. PROCEDURE

- A. All patients require immediate medical evaluation. The search for organ donor information shall not interfere with patient care activities.
- B. When EMS personnel encounter an unconscious adult patient for whom it appears death is imminent, they shall attempt a reasonable search of the patient's belongings to determine if the individual carries information indicating the patient's status as an organ donor. This search must be done in the presence of a witness, preferably a law enforcement officer.
- C. If documentation is found to indicate that the patient desires to be an organ donor, this documentation shall be brought with the patient to the receiving hospital. If a law enforcement officer on the scene requests to take possession of the documentation, the documentation shall be provided to the officer and verbal notice made to the receiving hospital staff. If the patient is not transported to the hospital, the documentation shall remain with the patient.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 4140

Effective Date:

Review Date:

ALS SERVICE PROVIDER RESPONSE AND SCENE TIME REPORTING

I. PURPOSE

To meet the requirements established in the statutes referenced below for EMS system planning, implementation, and evaluation.

II. POLICY

- A. ALS service providers are required to provide reports to the EMS Agency containing specified data elements for every patient response as requested by EMS Agency staff.
- B. This report is to be provided to the designated EMS Agency staff person within 15 days of request.
- C. The following data elements may be requested:
 - 1. Date of service
 - 2. Time of call receipt by the dispatch provider
 - 3. Time of dispatch to the scene
 - 4. Time of arrival at the scene
 - 5. Time of departure from the scene or cancellation
 - 6. Time of Transport Complete if the ALS service provider transports the patient
 - 7. Unit identifier
 - 8. Incident location by address, intersection, or nearest location identifier entered into the CAD
- C. The report shall be submitted to the EMS Agency in Excel document format.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 4150

Effective Date:

Review Date:

ALS FIRST RESPONDER AUTHORIZATION AND APPROVAL PROCESS

I. PURPOSE

- A. To establish standards and procedures for the EMS Agency to authorize First Responder Applicant Agencies to provide Advanced Life Support (ALS) within the County of Monterey EMS System.

II. POLICY

- A. This policy defines the process by which a First Responder Agency that desires to provide ALS First Response Services, but not transport services, applies to provide ALS First Response Services. This policy also details the evaluation, processing, and adjudication of that application by the County of Monterey EMS Agency.
- B. The County of Monterey EMS Agency will evaluate each potential ALS First Responder Agency's application primarily for: (1) clinical benefit, clinical detriment, and clinical effect on patients within the County of Monterey EMS System; and (2) financial and operational benefit, financial and operational detriment, and financial and operational effect on the County of Monterey EMS System.
- C. The County of Monterey EMS Agency uses a four-phase process to integrate new ALS First Responder Agencies into the County of Monterey EMS System. Those phases are:
 - 1. Phase 1: Application Phase
 - 2. Phase 2: Application Review and Approval Phase
 - 3. Phase 3: Paramedic Service Provider Agreement Phase
 - 4. Phase 4: Final inspection, approval, and authorization to implement ALS Phase
- D. Potential ALS First Responder Agencies may not implement ALS service before all phases are complete.
- E. ALS service areas will be granted for and defined by specific geographic areas.
- F. ALS First Responder agencies shall ensure that personnel participate in a Quality Improvement process. Each provider shall facilitate call review and provide Quality Improvement reports to the EMS Agency, consistent with County of Monterey EMS System policies and procedures.
- G. The EMS Agency shall review and collaboratively determine whether to renew, revise, cancel or otherwise modify its agreement with each ALS First Responder Agency at least every five (5) years.
- H. The EMS Agency may deny, suspend, or revoke the approval of an ALS First Responder Agency for failure to comply with applicable policies, procedures, and regulations.

- I. Successful completion of this process allows an organization to provide ALS First Response Service. It does not provide authorization for ALS or BLS transport.
- J. Successful completion of this process does not confer exclusivity to provide BLS or ALS First Response Services within the authorized zone.

III. PROCEDURE

- A. A First Responder Agency wishing to provide ALS services must complete the following application process:
- B. Phase 1: Application Phase
 - 1. Submit a proposal to the EMS Agency that documents:
 - a. Justification for ALS services
 - 1) Document the medical/clinical need and justification for ALS service
 - b. Geographic Service Area
 - 1) Provide a narrative description and a map identifying the Applicant Agency's proposed ALS service area. Identify whether the Applicant Agency's proposed ALS service area includes the entire response zone or if part of the Applicant Agency's response zone will continue to receive BLS service.
 - 2) Identify the proposed specific locations of the Applicant Agency's emergency response units that will be staffed and equipped at the ALS level.
 - 3) Identify whether the Applicant Agency's proposed ALS response area overlays the jurisdiction of another local EMS agency.
 - c. Quality Improvement Program
 - 1) Applicant Agency must have a current, approved QI plan on file with the EMS Agency.
 - 2) Submit documents that demonstrate a QI Program consistent with the QI Plan for at least six months (*e.g.*, annual updates, data reports).
 - 3) Document the number of hours per month that the Applicant Agency will allocate to quality improvement activities.
 - 4) Designate an agency member to serve as the provider liaison, training officer, and as the main contact person for Applicant Agency-based quality improvement and training issues.

d. Staffing

- 1) Document the number of Applicant Agency's proposed paramedic positions and the number of paramedics that will fill those positions.
- 2) Document the Applicant Agency's proposed shift schedule, the number of hours per shift, and the amount of time off between shifts.
- 3) Document that the Applicant Agency's response to emergency medical requests will be staffed with a minimum of one County of Monterey-accredited Paramedic and one California-licensed EMT.

e. Hours of Operation

- 1) Document that the Applicant Agency shall provide ALS services on a continuous 24 hours/day basis.

f. Response Time Performance

- 1) Provide dispatch and response time data or authorize access to the Applicant Agency's data for the most recent 12-month calendar year, in an editable excel spreadsheet, using the most current version of the National EMS Information System (NEMSIS) and California EMS Information System (CEMSIS) Standards.
- 2) Provide dispatch and response time data or authorize access to the Applicant Agency's data on an ongoing basis.

g. EMS Call Handling/Dispatch of ALS Resources

- 1) Identify the flow of the Applicant Agency's emergency calls from the primary PSAP through each intermediary point to a County of Monterey-designated EMS Communication Center to ensure callers have access to emergency medical-based dispatch, including MPDS-based caller interrogation and, if indicated, pre-arrival instructions.
- 2) Describe the Applicant Agency's dispatch process of ALS resources within the proposed ALS service area..
- 3) The Applicant Agency's call taking and dispatch process must assure that the Applicant Agency is dispatched in a manner consistent with EMS Agency-approved MPDS protocols.

h. EMS System Participation/Coordination with Ambulance Providers

- 1) Document the Applicant Agency's commitment to support

the EMS System by:

- a) Participating in EMS Agency-recognized MCI and disaster exercises.
- b) Allowing EMT and paramedic students to ride as interns.
- c) Otherwise participating in the training and mentorship of EMT and paramedic students and interns, ED personnel, and communications center personnel.

i. Patient Care Record System

- 1) Document the electronic patient care record system used.
- 2) If not currently utilizing the County of Monterey EMS Data System, document that the Applicant Agency will either switch to the County of Monterey EMS Data System or incur the costs and accept the liability to connect and continuously maintain bi-directional interoperability with the County of Monterey EMS Data System.

j. Compliance with Federal, State, and Local Law

- 1) State that the Applicant Agency will comply with all federal and state laws, local ordinances, and EMS System Policies, Procedures, and Protocols.

k. Equipment and Supplies

- 1) Document the Applicant Agency's plan to secure, maintain and replenish controlled substances, other drugs, solutions, and equipment.

l. ALS Financing

- 1) Describe the method that the Applicant Agency will use to finance the ALS service, including any proposed fees and method of fee collection.
- 2) The EMS Agency will reject any application that utilizes a funding mechanism that may detrimentally affect the financial viability of the County of Monterey EMS System.

- 2. The application shall be signed by the individual authorized by the Applicant Agency to enter into agreements.

C. Phase 2: Application Review and Approval Phase

The EMS Agency and EMS Medical Director will review and determine whether to approve the Applicant Agency's application.

- 1. The EMS Agency shall date and time stamp the Applicant Agency's application

at the time of receipt, and shall acknowledge receipt of the application in writing to the applicant.

2. EMS Agency personnel shall review the application and determine if it is complete and meets applicable federal, state, and local ordinances and EMS System Policies, Procedures, and Protocols.
3. If the application is incomplete or does not meet applicable laws and EMS System Policies, Procedures, and Protocols, the EMS Agency will decline to process the application, and will notify the Applicant Agency in writing of the application's deficiencies. The Applicant Agency may resubmit the application after correcting any deficiencies.
4. If the application is complete and meets applicable laws and EMS System Policies, Procedures, and Protocols, the EMS Agency will further evaluate the application. The evaluation will consider, but will not be limited to, the information presented in the application and the clinical, operational, and financial impacts of granting the applicant's request for that organization, those that it serves, and the County of Monterey EMS System.
5. The EMS agency shall meet and discuss the application with representatives of the Applicant Agency before issuing a tentative determination.
6. The EMS Agency will issue a tentative determination to either authorize or not authorize the First Responder Agency to provide ALS services. This determination will identify the EMS Agency's rationale for its decision. If the authorization includes conditions, those conditions will be explained in this tentative determination.
7. Upon EMS Agency verification of receipt of a completed application from the Applicant Agency, the EMS Agency will schedule a discussion of the application on the agenda of the next meeting of the Clinical Care Committee (CCC) to seek the CCC's recommendation regarding the application. At the CCC meeting, the applicant may present its rationale for its request to provide ALS first response service. The EMS Agency may present its rationale for supporting or denying the applicant's application. The CCC may endorse the EMS Agency's recommendation or may issue its own recommendation.
8. The EMS Director and EMS Medical Director will make a final determination.
 - a. A final determination to approve or deny the Applicant Agency to provide ALS services will be made within 90 days of receipt of a completed application.
 - b. If the EMS Director and EMS Medical Director's determination is to authorize the Applicant Agency to provide ALS services, the Applicant Agency advances to Phase 3: Paramedic Service Provider Agreement Phase.
 - c. If the EMS Director and EMS Medical Director's determination is to not

authorize the Applicant Agency to provide ALS services, the application process ends.

D. Phase 3: Paramedic Service Provider Agreement Phase

The EMS Agency and the Approved Applicant Agency wishing to provide ALS services must enter into the ALS Service Provider Agreement, identified as Attachment A, as required by California Code of Regulations, Title 22, Section 100096.01. The Paramedic Service Provider Agreement shall minimally address the following items:

1. Term of agreement
2. Scope of services (to include primary service/geographic area to be served)
3. Medical control and Quality Improvement Program/Plan
4. EMS data management and reporting
5. Telecommunications
6. MPDS, communication, and dispatch standards
7. Equipment, supplies, and medications
8. MCI and EMS System Preparedness
9. Ride along by EMS Agency staff
10. Personnel standards
11. Shift schedules/fatigue
12. Performance reporting
13. Record inspection
14. Patient privacy
15. Prohibitions on sub-contracting or transferring ALS Authorization
16. Insurance and indemnification
17. Pricing, Fees, Billing, and Compensation (if applicable)
18. Compliance with statutes, regulations, ordinances, policies, and procedures
19. Suspension, Revocation and Termination of Agreement
20. Waiver
21. No Third-Party Rights
22. Relationship of the parties
23. Notices

E. Phase 4: Final inspection, approval, and authorization to implement ALS Phase

1. Following the execution of a paramedic service provider agreement by all parties, the EMS Agency will inspect the Applicant Agency's ALS Program. This

inspection will verify the presence of the personnel, equipment, and supplies, and will verify the Applicant Agency's compliance with EMS System Policies, Procedures, and Protocols.

2. Following a successful inspection, the EMS Agency shall authorize the Applicant Agency to start providing ALS service.
3. The EMS Agency will notify Base Hospitals, Receiving Hospitals, the County of Monterey Emergency Communications Department, and the EMS Communications Center that the Applicant Agency is an authorized ALS First Responder Agency.

NOTE:

In instances where a First Responder Agency has an active paramedic service provider agreement in place at the time of this policy's implementation or subsequent revisions, that agreement shall remain valid and enforceable through its original expiration date. The agency shall not be required to renew or modify the existing agreement to comply with the updated policy until the current agreement term has expired or is otherwise terminated.

Attachment A

NEMSIS 3.4 Time Point Standards

Provide the following Dispatch and Response Time data points, consistent with the following definitions:

1. PSAP Call Date/Time: The date/time the phone rights (9111 call to public safety answering point or other designated entity requesting EMS services. (NEMSIS eTimes.01)
2. Dispatch Notified Date/Time: The date/time dispatch was notified by the 911 call taker (if a separate entity) (NEMSIS eTimes.02)
3. Unit Notified by Dispatch Date/Time: The date/time the responding unit was notified by dispatch. (NEMSIS eTimes.03)
4. Dispatch Acknowledged Date/Time: The date/time the dispatch was acknowledged by the EMS Unit. (NEMSIS eTimes.04)
5. Unit En Route Date/Time: The date/time the unit responded; that is, the time the vehicle started moving. (NEMSIS eTimes.05)
6. Unit Arrived on Scene Date/Time: The date/time the responding unit arrived on the scene; that is, the time the vehicle stopped moving at the scene. (NEMSIS eTimes.06)
7. Arrived at Patient Date/Time: The date/time the responding unit arrived at the patient's side. (NEMSIS eTimes.07)
8. Transfer of EMS Patient Care Date/Time: The date/time the patient was transferred from this EMS agency to another EMS agency for care. (NEMSIS eTimes.08)
9. Unit Left Scene Date/Time: The date/time the responding unit left the scene with a patient (started moving). (NEMSIS eTimes.09)
10. Arrival at Destination Landing Area Date/Time: The date/time the Air Medical vehicle arrived at the destination landing area. (NEMSIS.10)
11. Patient Arrived at Destination Date/Time: The date/time the responding unit arrived with the patient at the destination or transfer point. (NEMSIS eTimes.11)
12. Destination Patient Transfer of Care Date/Time: The date/time that patient care was transferred to the destination healthcare facilities staff. (NEMSIS eTimes.12)
13. Unit Back in Service Date/Time: The date/time the unit back was back in service and available for response (finished with call, but not necessarily back in home location). (NEMSIS eTimes.13)
14. Unit Canceled Date/Time: The date/time the unit was canceled. (NEMSIS eTimes.14)
15. Unit Back at Home Location Date/Time: The date/time the responding unit was back in their service area. With agencies who utilized Agency Status Management, home location means the service area as assigned through the agency status management protocol. (NEMSIS eTimes.15)
16. EMS Call Completed Date/Time: The date/time the responding unit completed all tasks associated with the event including transfer of the patient, and such things as cleaning and restocking. (NEMSIS eTimes.16)

County of Monterey EMS System Policy



Policy Number: 4160
Effective Date:
Review Date:

ADVANCED LIFE SUPPORT (ALS) TRANSPORT AGENCY

I. PURPOSE

To establish requirements for the designation of a County of Monterey Advanced Life Support (ALS) Transport Agency.

II. REQUIREMENTS

A. A designated ALS Transport Agency shall:

1. Have a written agreement, as specified in **EMS System Policy 4150-ALS First Responder Authorization and Approval Process**, with the local EMS Agency to participate in the County of Monterey EMS System, and shall comply with all applicable State regulations, local policies, procedures, protocols, and ordinances.
2. Provide ALS emergency medical service response on a continuous twenty-four (24) hours per day basis.
3. Have and agree to utilize and maintain communications as specified by the EMS Agency.
4. Have and agree to maintain a drug and solution inventory, and BLS and ALS medical equipment and supplies as specified by the EMS Agency.
5. Designate a Quality Improvement Manager who shall be at minimum an EMT-P and the liaison to the County, ALS Base Hospital(s), and ALS Receiving Hospital(s).
6. Participate in the County of Monterey-approved Quality Improvement Program.
7. Establish a mechanism to ensure that all paramedics employed by the agency are currently licensed and hold all certificates (*e.g.*, CPR, ACLS, PALS or equivalent, PHTLS or equivalent) required by the EMS Agency.
8. Agree to permit the EMS Agency to conduct ride alongs on ambulances.
9. Agree to permit base hospital coordinators and emergency department physicians to observe on ambulances.
10. Agree to provide paramedic internship opportunities to qualified applicants in accordance with County policy.
11. Agree to utilize the assigned Base Hospital(s) for on-line medical direction.
12. Agree to ensure that paramedic personnel have visible identification at all times when those personnel are on duty.
13. Agree to notify the EMS Agency of significant operational problems or changes and unusual occurrences in a timely manner and as per policy, including changes

County of Monterey EMS System Policy 4160

in the number of units, permanent unit location(s), and major agency personnel changes.

14. Ensure liability, malpractice, and other insurances necessary for ALS participation in the County of Monterey EMS System.
 15. All ALS ambulances, both public and private, shall be staffed at minimum with one (1) State of California-licensed and County of Monterey-accredited Paramedic. It is the responsibility of the ALS Transport Agency to ensure the availability of well-trained and rested personnel necessary to provide optimal patient care.
 16. Any person who drives an ambulance will have in his/her possession a valid California Driver's License and a current, valid Ambulance Driver's Certificate. Ambulance may be driven by individuals who do not possess an Ambulance Driver's Certificate if they are salaried full-time police officers, deputy sheriffs or fire fighters, as per Title 13, Section 1100.2 California Code of Regulations.
- B. All ALS ambulances will be stationed in such a manner as to permit arrival at the scene of an emergency within the requirements set forth for each Agency as per contract.
- C. No Agency shall advertise itself or a responding unit as providing ALS services unless it does, in fact, routinely provide ALS services on a continuous twenty-four (24) hour per day basis and is currently designated by the EMS Director per contract to provide such services.
- D. The EMS Agency may deny, suspend or revoke the contract with an ALS Transport Agency, as per the contract, for failure to comply with applicable policies, procedures, protocols, ordinances, and regulations.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 4501
Effective Date:
Review Date:

AED GUIDELINES

I. PURPOSE

To establish the procedures for accredited personnel to utilize an Automatic External Defibrillator (AED) in the pre-hospital management of a patient in cardiac arrest and to establish the process for AED Service Provider approval.

II. POLICY

- A. A California-licensed Paramedic or currently-certified EMT or Emergency Medical Responder may utilize an AED as part of their scope of practice when employed by a Public Safety AED Service Provider.
- B. A Public Safety AED Service Provider shall meet the requirements of this policy and all California State regulations regarding the use of an AED by their personnel.
- C. Paramedics, EMTs, and other EMS responders may utilize a Public Access AED.
- D. Public Safety AED Service Provider personnel shall follow the guidelines established by the American Heart Association for CPR and for use of an AED, as specified in County of Monterey EMS System policies and protocols.

III. AED SERVICE PROVIDER APPROVAL:

- A. Public Safety AED Service Providers desiring to operate within Monterey County shall submit an application for approval to the County of Monterey EMS Agency, or in the case of state or federal agencies, to the EMS Authority, prior to beginning service. The EMS Agency shall review the application and render a decision within sixty (60) days of receiving a complete application packet. In order to receive and maintain AED service provider approval, a Public Safety AED Service Provider shall ensure compliance with the requirements of this policy and California regulations.
- B. Public Safety AED Service Provider approval may be revoked or suspended for failure to maintain the requirements under California regulations and/or County of Monterey EMS System Policies.
- C. Conditions of approval for a Public Safety AED Service Provider to operate within Monterey County include the following:
 - 1. Submission of a completed application packet to the EMS Agency that includes identification of the Public Safety AED Service Provider's AED instructors;
 - 2. Written policies and procedures for orientation, training, and continued competency of AED-authorized personnel pertaining to the safe and proper use of the AED;

3. Written policies and procedures to ensure appropriate AED equipment maintenance in accordance to manufacturer recommendations;
4. Written policies and procedures for collection and reporting of data from each clinical encounter involving AED utilization. Data shall be submitted via the County of Monterey AED Utilization Reporting Form.
5. Maintenance of a current list of the Public Safety AED Service Provider's authorized personnel and AED instructors, to be provided to the EMS Agency and/or EMS Authority upon request.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 4505

Effective Date:

Review Date:

INTRAOSSEOUS INFUSION

I. PURPOSE

Intraosseous infusion is to provide an alternate means of vascular access when speed in vascular access is critical, the IV route is not available, or IV access attempts were unsuccessful and the patient would benefit from the timely administration of drugs or fluids.

II. POLICY

A. Intraosseous infusion may be performed by paramedics who have successfully completed Monterey County EMS Agency approved training within the previous 12 months. Documentation of this training shall be maintained by the paramedic's employer and provided to the Monterey County EMS Agency.

B. Intraosseous infusion is approved for adult and pediatric patients.

C. Approved sites for IO insertion:

1. Adult (9 years of age or older, or 40 kg or greater)
 - a. Proximal tibia or proximal humerus
2. Pediatric (8 years of age and younger, or less than 40 kg)
 - a. Proximal tibia only

D. Indications:

1. Cardiac arrest or impending cardiac arrest.
2. Decompensated shock when IV access cannot be rapidly achieved.
3. Shock state or patient condition indicates the need for rapid parenteral (routes other than oral or rectal) fluid or medication infusion and IV access is not available (e.g.: significant burns) or multiple IV attempts have failed.

E. Contraindications:

1. Recent fracture of the involved bone.
2. Infection at the site selected for insertion.
3. Inability to locate anatomical landmarks for insertion.
4. Those patients who have a patent IV or in whom an IV may be established in a timely manner.
5. Second attempt on the same bone.
6. Patients who are medically stable and whose condition does not require rapid parenteral fluid or medication administration.

III. PROCEDURE

A. Assure that indications for use have been met.

1. Determine patient age and weight to select the appropriate IO insertion device.
 - a. For a patient who weighs 3 kg or less, use a manual IO device.
 - b. For a patient who weighs more than 3 kg but is either less than 40 kg or less than 8 years of age, use the Pediatric EZ-IO or manual device.
 - c. For a patient who weighs 40 kg or greater or is 8 years of age or older, use the Adult EZ-IO.
2. Locate approved insertion site
 - a. Adult (8 years of age or older, or 40 kg or greater)
 - 1) Proximal tibia (see Figure 1)
 - 2) Proximal humerus (see Figure 2)
 - b. Pediatric (less than 8 years of age, or less than 40 kg)
 - 1) Proximal tibia only (see Figure 1)

B. Process for insertion.

1. Use appropriate personal protective equipment (gloves, mask, eye protection).
2. Obtain age/weight appropriate supplies.
3. Rule out contraindications.
4. Locate appropriate insertion site.
5. Prepare insertion site using aseptic technique.
6. Prepare the intraosseous device.
7. Stabilize the site and insert the needle at a 90-degree angle to the bone (**see Figure 1**). For proximal humerus insertion (adults only), insert the needle at a 45-degree downward angle to the lateral plane of the shoulder (**see Figure 2**).
8. Remove the stylet from the catheter.
9. Confirm placement of the catheter by flushing the catheter with 10 cc normal saline.
10. Consider the administration of Lidocaine (2% solution) 20 mg IO for the adult or 0.5 mg/kg (up to 20 mg) IO for the pediatric patient who is conscious and complains of pain at the insertion site.
11. Attach IV tubing to the IO device.
12. Flush catheter with 10 cc normal saline. Check for signs of extravasation (soft tissue swelling or difficulty infusing fluids through the catheter). Begin normal saline infusion or medication administration. Consider use of pressure device to provide rapid infusion of fluids, if needed.

13. Dress insertion site, stabilize and secure the catheter.
14. Closely monitor insertion site for signs of extravasation. If there is progressive soft tissue swelling at the insertion site, or if there is significant resistance to infusion of fluids or administration of medication through the IO catheter, leave the IO catheter in place but discontinue use of the IO device.
15. Advise hospital staff of any difficulty administering fluids or medication through the IO catheter, any signs of extravasation, or any other complications associated with IO placement.

Figure 1: Proximal Tibia Insertion Site (Adult and Pediatric Patients)



(Insert needle at a 90-degree angle to the plane of the bone.)

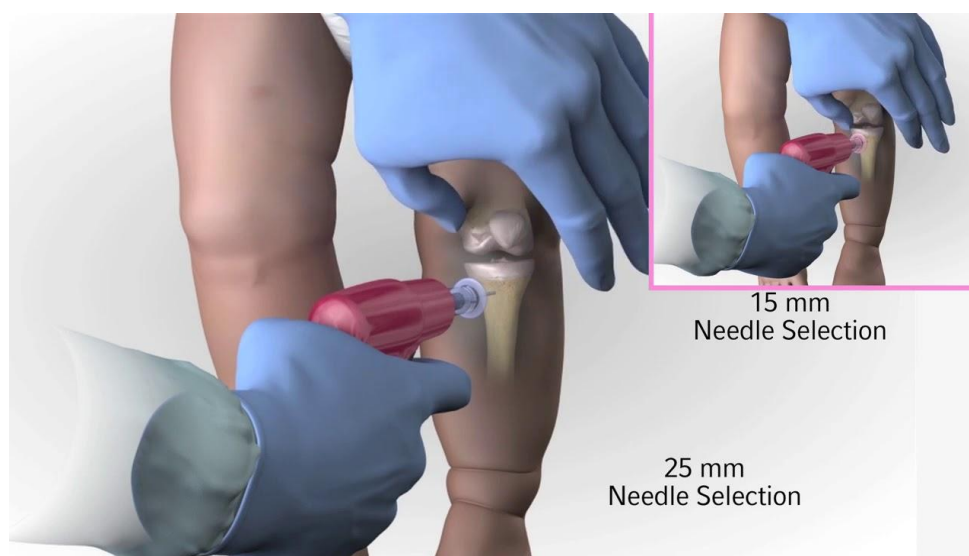
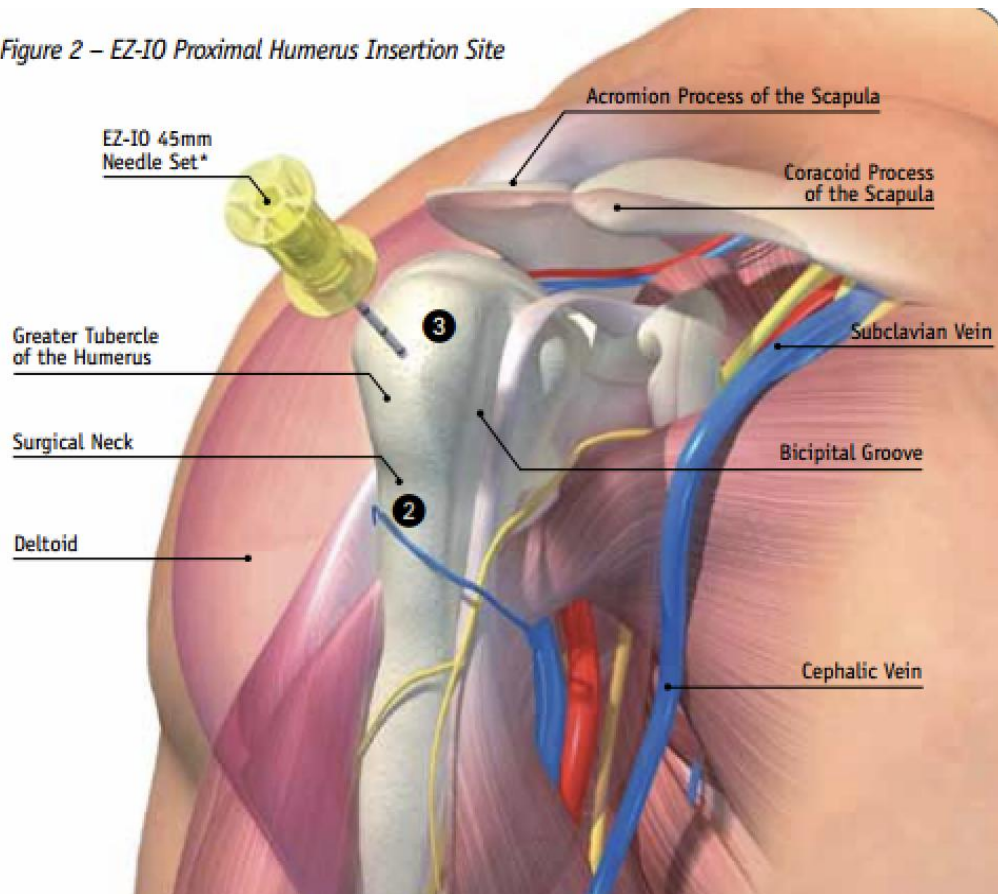
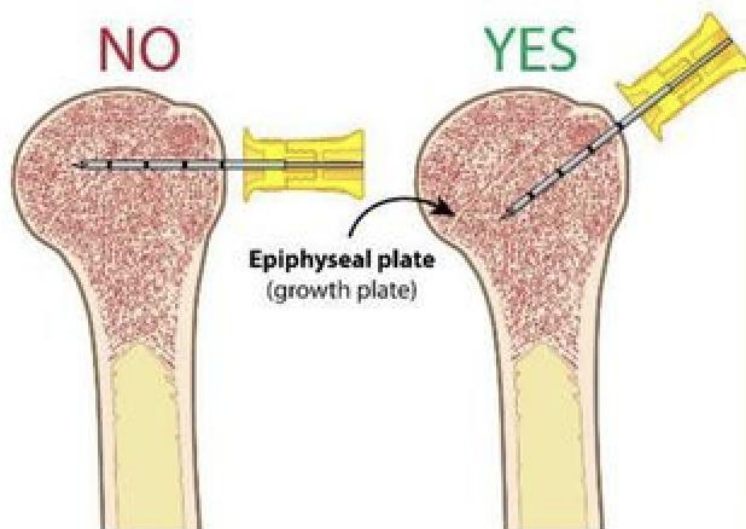


Figure 2: Proximal Humerus Insertion Site (Adult Patients Only)

Figure 2 – EZ-IO Proximal Humerus Insertion Site



(To avoid the epiphyseal growth plate, insert the needle at a 45-degree downward angle to the lateral plane of the shoulder.)



END OF POLICY

County of Monterey EMS System Policy



Policy Number: 4508

Effective Date:

Review Date:

USE OF RESTRAINTS

I. PURPOSE:

To establish standards for the use of restraints.

II. POLICY:

The following principles shall be followed when utilizing patient restraints:

- A. Restraints shall be humanely and professionally applied to protect the patient's safety and dignity.
- B. The method of restraint should be the least restrictive necessary for the safety of the patient and others.
- C. The use of restraints shall be thoroughly documented. Documentation shall include the reason(s) for use of restraints, type of restraints used, what limbs are placed in restraints, the use of sedation (if applicable), and regular assessments of the restrained patient.
- D. Assessments of the restrained patient shall be conducted and documented at least every 15 minutes. Documentation shall include the patient's level of consciousness, respirations, and distal perfusion to any restrained extremities.
- E. **Patients SHALL NOT be restrained in a prone position.**

III. PROCEDURE:

A. Physical restraints

- 1. Patients who meet one or more of the following criteria shall be restrained to the gurney utilizing a minimum of two-point restraints.
 - Patients who have been placed on an involuntary psychiatric hold (referred to herein as a "5150 hold").
 - Patients who present a potential danger to themselves or others as determined by EMS personnel, law enforcement, emergency department staff, or other prehospital personnel.
 - Patients who require medical attention and are in law enforcement custody.
- 2. Unless contraindicated by an upper extremity amputation or injury, acceptable two-point restraints include application of a restraint to both wrists. If an amputation or injury makes it not feasible to place both wrists in restraints, then at least one wrist and one ankle shall be placed in restraints.

B. When transporting a patient in restraints, EMS personnel shall ensure the following:

1. **NEVER RESTRAIN A PATIENT IN A PRONE POSITION.** Restrained patients shall be placed in a supine, semi-Fowler's, or Fowler's position. Patients shall never be transported in a prone or "hog-tied" position.
2. Only restrain patients in a manner that allows for rapid access to the airway, rapid initiation of CPR, and full expansion of the patient's chest during respirations.
3. In addition to restraints, all patients shall be secured to the gurney utilizing all appropriate standard safety belts.
4. The patient shall be securely restrained to the gurney PRIOR to placing the patient in the ambulance.
5. Patient safety buckles and restraints shall be clearly visible and continually monitored by the attending EMS provider at all times during the course of transport.
6. Patients shall remain in restraints for the duration of the transport.
7. Except for brief removal of restraints to enable a patient to utilize the bathroom during a long-distance transport, restraints shall not be removed until the patient is safely inside the receiving facility and patient care has been transferred to the receiving facility staff.
8. The use of chemical sedation DOES NOT satisfy the need for application of physical restraints. Patients who have been chemically sedated due to dangerous, aggressive, or unpredictable behavior shall also be placed in restraints for the duration of the transport.
9. Patient restraints shall never be imposed as a means of coercion, discipline, convenience, or retaliation.

C. Spit hoods

1. A spit hood shall not be applied to the patient if the patient is having any affect from pepper spray or other contaminants.
2. The use of pulse oximetry is required.
 - a. If unable to obtain accurate O2 saturation readings, the spit hood should not be used.
 - b. The pulse oximetry should be monitored every 5 minutes and documented on the patient care record (PCR).
3. The patient must not be placed prone or restrained in the prone position.
4. The spit hood shall be removed if any of the following are present:

- a. Patient complaint of shortness of breath.
- b. The patient shows any signs of increased anxiety and attempts at calming the patient are ineffective.
- c. Signs of hypoxia such as:
 - 1) Drowsiness
 - 2) Increasing agitation
 - 3) Decreased responsiveness
 - 4) O2 saturation less than 94%

IV. PATIENTS IN LAW ENFORCEMENT CUSTODY

- A. Patients in law enforcement custody who require EMS transport shall either be transported in restraints or handcuffed by a law enforcement officer.
 - 1. Patients who are handcuffed require a law enforcement officer to accompany the patient in the back of the ambulance during transport to the receiving facility. EMS personnel are not authorized to use handcuffs or manage the restraint of a patient placed in handcuffs.
- B. Patients who are restrained with EMS restraints (not handcuffs) do not require the presence of a law enforcement officer in the back of the ambulance during transport to the receiving facility.

V. PATIENT ESCAPE:

Should a patient escape from restraints and exit the ambulance, the following steps should be initiated:

- A. Do everything possible to minimize injury to EMS personnel and the patient.
- B. Contact EMS Communications **immediately** and advise them of the situation, location, and status of the patient.
- C. EMS Communications shall notify law enforcement and request their presence at the scene of the incident. EMS Communications shall also notify the Monterey County EMS Duty Officer.
- D. EMS personnel shall attempt to maintain visual contact with the patient as long as possible and shall update EMS Communications as needed.
- E. EMS shall remain at the scene until cleared by law enforcement or the On-Duty Supervisor.
- F. In addition to the ePCR, each crew member of the EMS team transporting the patient at the time of the patient's escape shall complete a detailed incident report.

VI. DOCUMENTATION:

When a patient is transported in restraints, the following information must be documented:

- A. Reason(s) for use of restraints (danger to self/others, risk/history of violence, etc.).
- B. Type of restraints used and what limbs are in restraints.
- C. Assessment of patient condition shall be conducted and documented at least every 15 minutes. Documentation shall include the patient's level of consciousness, respirations, and distal perfusion to any restrained extremities.

VII. NOTES

Please refer to Monterey County EMS Agency **Policy #4050- Psychiatric Evaluation- 5150 Transports** and **Protocol M-5 Agitated/Combative Patient** for further guidance.

END OF POLICY

County of Monterey EMS System



Policy 4512-A

LAW ENFORCEMENT ADMINISTRATION OF INTRANASAL NALOXONE (NARCAN) REPORT

Reporting Law Agency	Date
Officer Name	Time
Call Number	Responding Fire Dept.
Call Address	Responding Ambulance
Patient Name	Patient Date of Birth
Initial Assessment	
Awake? ☐ Yes ☐ No	Speech clear? ☐ Yes ☐ No
Approximate Breaths per Minute	
Treatment Provided	
Reposition Airway ☐ Yes ☐ No	Rescue Breathing ☐ Yes ☐ No
CPR ☐ Yes ☐ No	Administer Naloxone ☐ Yes ☐ No
Other	
Assessment After Treatment	
Awake? ☐ Yes ☐ No	Speech clear? ☐ Yes ☐ No
Approximate Breaths per Minute	

After completing, email to County of Monterey EMS Agency within 48 hours

Email: EMSDutyOfficer@countyofmonterey.gov



County of Monterey EMS Agency

Policy 4512 B

Law Enforcement Intranasal Naloxone Program Application

Applicant Information

Agency: _____ Date: _____

Address: _____
Street Address

City State ZIP Code

Phone: _____ Email: _____

Law Enforcement Agency Point of Contact

Name: _____ Title: _____

Email: _____ Phone: _____

Attachments to Submit

Please attach your agency's training course outline as outlined in County of **MontereyEMS System Policy #4512**.

Email application and attachments to:

emsadmin@countyofmonterey.gov

County of Monterey EMS System Policy



Policy Number: 4512
Effective Date:
Review Date:

LAW ENFORCEMENT ADMINISTRATION OF INTRANASAL NALOXONE (NARCAN)

I. PURPOSE

To establish the process for law enforcement agencies and personnel to obtain approval to administer intranasal naloxone.

To delineate the procedure for the use of intranasal naloxone by law enforcement agencies.

II. POLICY

- A. Law enforcement personnel working for agencies in Monterey County that have received approval by the County of Monterey EMS Agency may administer 4 mg. intranasal naloxone following the procedure outlined in this policy.
- B. To obtain EMS Agency approval, law enforcement agencies must complete the following before using intranasal naloxone.
 1. Submit an application to the EMS Agency requesting approval of the intranasal naloxone program. **(The application is attached as Policy 4512 B.)**
 2. Submit the training course outline to the EMS Agency as outlined below.
 3. Designate a point of contact for the program and provide contact information for the individual to the EMS Agency.
- C. Law enforcement personnel must have successfully completed CPR and First Aid training or retraining to the standards set in California Code of Regulations, Title 22, Chapter 2.3
- D. The course of instruction in the administration of intranasal Naloxone must, at a minimum, include the topics and skills required by California Code of Regulations, Title 22, Division 9, Chapter 2.3. The course may not be held until the EMS Agency has granted course approval. Course approval may be granted for up to four (4) years.
- E. Retraining in the use of intranasal Naloxone will be provided to the law enforcement personnel at least every two years. Retraining will include demonstration of the procedure for administering intranasal naloxone.
- F. Law enforcement personnel shall follow the guidelines established by the American Heart Association for CPR.
- G. The EMS Agency will review each use of intranasal naloxone.

III. PROCEDURE TO ADMINISTER INTRANASAL NALOXONE

- A. Identify the victim of a possible opioid overdose.

- B. Ensure EMS has been activated.
- C. Maintain standard blood and body fluid precautions and use personal protective equipment.
- D. Stimulate the patient. If unresponsive, use a sternal rub technique.
- E. Ensure an open airway using Basic Life Support techniques.
- F. Perform rescue breathing, if indicated, using bag-valve-mask or protective face shield.
- G. If the patient is in cardiac arrest as demonstrated by the absence of breathing and a pulse, begin CPR.
- H. Administer 4 mg intranasal naloxone, following the procedure learned in training.
- I. Continue CPR, rescue breathing, or provide other first aid as indicated.
- J. Prepare for possible narcotic reversal behavior or withdrawal symptoms such as vomiting, agitation, aggression, irritability, etc.
- K. Notify responding EMS personnel of administration of naloxone.
- L. Report the use of naloxone to the EMS Agency on the designated report form.

IV. PROCEDURE FOR REPORTING ADMINISTRATION OF INTRANASAL NALOXONE

- A. Following the use of naloxone, complete Law Enforcement Administration of Intranasal Naloxone (Narcan) Report. **(The form is attached as Policy 4512 A.)**
- B. Forward Report through law enforcement agency's chain of command to the County of Monterey EMS Agency within 48 hours of naloxone administration.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 5140

Effective Date:

Review Date:

EMERGENCY DEPARTMENT RE-TRIAGE AND RAPID TRANSFER OF TRAUMA PATIENTS TO TRAUMA CENTER

I. PURPOSE

To allow for the expedited transport and care of the Major Trauma Patient who arrives at a non-Trauma Center Emergency Department.

II. POLICY

- A. Under the Field Trauma Triage Criteria (Policy #4040), Major Trauma Patients are to be triaged by EMS personnel directly to a Trauma Center. Trauma patients who present at non-Trauma Center Emergency Departments via EMS or another arrival mode should be considered for re-triage and transfer to a trauma center for definitive care. If patients are seriously injured, the re-triage and transfer process should be done as quickly as possible.
- B. Transferring facilities should use the attached algorithm to assist with identification of those trauma patients who would benefit from care at a Trauma Center.
- C. Transferring facilities should use the process outlined in the attached algorithm to facilitate transfer to the Trauma Center.
- D. The re-triage and transfer of trauma patients will be monitored at Trauma Evaluation Quality Improvement Committee (TEQIC) meetings.
- E. Local Base and Receiving Hospitals shall have:
 - 1. Written transfer agreements (for both adult and pediatric patients) with an appropriate designated Level I or Level II Trauma Center.
 - 2. Guidelines for identification of patients who should be considered for transfer to a Trauma Center that are consistent with the County of Monterey EMS Agency policies and protocols.
 - 3. A procedure for arranging the transfer of appropriate patients (adults and pediatrics), including but not limited to:
 - a. Notification of the receiving Trauma Center physician.
 - b. Arranging for transport by either ground or air.
- F. The Trauma Center shall have:
 - 1. Written transfer agreements with:
 - a. The nearest designated Level I Trauma Center.
 - b. An appropriate specialty center providing tertiary-level care for burn injuries.

County of Monterey EMS System Policy 5140

- c. An appropriate facility for patients with spinal cord injuries.
- d. The nearest designated Pediatric Trauma Center.
- 2. A procedure for arranging the transfer of appropriate patients (adult and pediatric), including but not limited to:
 - a. Notification of the receiving center physician.
 - b. Arranging for transport by either ground or air

Monterey County Emergency Trauma Re-Triage Procedure – Adult (age 15 and older)

Step 1	Determine if patient meets Emergency Trauma Re-Triage Criteria	See Criteria below – Adult patients are age 15 and older.	
Step 2	Contact Adult Trauma Center	Natividad – contact the Natividad Transfer Center. Tell the transfer center you have a “Red Box Trauma Re-Triage”. Transfer center will arrange report and transport. Phone 855-445-7872; Fax: 916-646-7100 Santa Clara Valley Medical Center Trauma Report: 408-947-4087 Burn Line: 408-885-6666 Regional Medical Center Trauma Line: 408-729-2841 Stanford Medical Center Trauma Line: 800-800-1551	
Step 3	If not transporting to Natividad: Determine appropriate level of transport and arrange transport. (Can be done simultaneously while contacting the Trauma Center)	If level of transport is within paramedic scope of practice and timely transfer is needed, contact the EMS Communications Center to request a Code- 3 Emergency Interfacility Transfer . Transport should generally arrive within 10-15 minutes. Phone 831-796-6444 If level of transport exceeds paramedic scope of practice, contact EMS Communications Center for Critical Care Transport (CCT), or arrange transport by Air Ambulance, or arrange for nursing staff to accompany an ALS ambulance.	
Step 4	Prepare patient, diagnostic imaging disc(s), and paperwork for immediate transport.	Fax additional paperwork that is not ready at time of transport departure. Do not delay transport.	
Level of Transportation	ALS AMBULANCE	CCT-RN AMBULANCE	AIR AMBULANCE
Provider(s)	Paramedic and EMT	Critical Care Transport – RN & EMT	RN and Paramedic
Scope of Care/Practice	Standard paramedic scope. No paralyzing agents or blood products. Can sedate intubated patients with midazolam. Can monitor chest tubes not requiring suction.	Mechanical ventilation, most medications including paralyzing agents, blood products.	Mechanical ventilation, most medications including paralyzing agents, blood products.
Contact Number	AMR – 831-796-6444	AMR – 831-796-6444 8	CALSTAR/ REACH 800-252-5050 Mercy Air 800-222-3456

EMERGENCY TRAUMA RE-TRIAGE CRITERIA – ADULT

****The following criteria are intended to serve as guidelines, based on County of Monterey Policy 4040 (Field Trauma Triage Decision Algorithm). The decision of whether to activate an emergency trauma re-triage is at the discretion of the emergency department physician.****

Mental Status & Vital Signs:

- ❖ Unable to follow commands (Motor GCS < 6)
- ❖ Respiratory rate < 10 or > 29 breaths/min, or respiratory distress, or need for respiratory support
- ❖ Age 15-64 years: Systolic Blood Pressure (SBP) < 90 mmHg, or HR > SBP
- ❖ Age > 64 years: SBP < 110 mmHg, or HR > SBP

Injury Pattern:

- ❖ Penetrating injuries to head, neck, torso, and extremities proximal to elbow and knee
- ❖ Chest wall instability, deformity, or suspected flail chest
- ❖ Fracture of two or more proximal long bones
- ❖ Crushed, degloved, mangled, or pulseless extremity
- ❖ Amputation proximal to wrist or ankle
- ❖ Pelvic fractures
- ❖ Skull deformity or suspected skull fracture
- ❖ Suspected spinal injury with new motor and/or sensory loss
- ❖ Active bleeding requiring a tourniquet or wound packing with continuous pressure

Mechanism of Injury:

- ❖ Falls from height > 10 feet
- ❖ High-risk auto crash
 - Ejection (partial or complete) from automobile
 - Need for extrication for entrapped/pinned patient with significant pain or injuries
 - MVC at > 45 mph with significant pain or injury
 - Death in same passenger compartment
- ❖ Rider separated from transport vehicle with significant impact (e.g., motorcycle, ATV, horse, etc.)
- ❖ Auto vs pedestrian/bicyclist thrown, run over, or with significant impact

Provider Judgement: Patients who, in the judgment of the evaluating emergency physician, are anticipated to have a high likelihood of requiring emergency surgery or other emergency intervention (within 2 hours of arrival at the Trauma Center) requiring the specialized services of a Trauma Center.

County of Monterey Emergency Re-Triage Procedure – Pediatric (age 14 and younger)

Step 1	Determine if patient meets Emergency Trauma Re-Triage Criteria	See criteria below – Pediatric patients are age 14 and younger .	
Step 2	Contact Pediatric Trauma Center	Santa Clara Valley Medical Center Trauma Line: 408-885-6666 Burn Line: 408-885-6666 Stanford Medical Center: 800-800-1551	
Step 3	Determine appropriate level of transport and arrange transport. (Can be done simultaneously while contacting the Trauma Center.)	If level of transport is within paramedic scope of practice and timely transfer is needed, contact the EMS Communications Center to request a Code-3 Emergency Interfacility Transfer . Transport should generally arrive within 10-15 minutes. If level of transport exceeds paramedic scope of practice, contact the EMS Communications Center for Critical Care Transport (CCT), or arrange transport by Air Ambulance, or arrange for nursing staff to accompany an ALS ambulance.	
Step 4	Prepare patient, diagnostic imaging disc(s), and paperwork for immediate transport.	Fax additional paperwork that is not ready at time of transport departure. Do not delay transport.	
Level of Transportation	ALS AMBULANCE	CCT-RN AMBULANCE	AIR AMBULANCE
Provider(s)	Paramedic and EMT	Critical Care Transport – RN & EMT	RN and Paramedic
Scope of Care/Practice	Standard paramedic scope. No paralyzing agents or blood products. Can sedate intubated patients with midazolam. Can monitor chest tubes not requiring suction.	Mechanical ventilation, most medications including paralyzing agents, blood products.	Mechanical ventilation, most medications including paralyzing agents, blood products.
Contact Number	AMR – 831-796-6444	AMR – 831-796-6444 8	CALSTAR/ REACH 800-252-5050 Mercy Air 800-222-3456

EMERGENCY TRAUMA RE-TRIAGE CRITERIA – PEDIATRIC

****The following criteria are intended to serve as guidelines, based on County of Monterey Policy 4040 (Field Trauma Triage Decision Algorithm). The decision of whether to activate an emergency trauma re-triage is at the discretion of the emergency department physician.****

Mental Status & Vital Signs:

- ❖ Unable to follow commands (Motor GCS < 6)
- ❖ Respiratory rate < 10 or > 29 breaths/min, or respiratory distress, or need for respiratory support
- ❖ Age 0-9 years: Systolic Blood Pressure (SBP) < 70 mmHg + (2 x age in years)
- ❖ Age 10-14 years: SBP < 90 mmHg

Injury Pattern:

- ❖ Penetrating injuries to head, neck, torso, and extremities proximal to elbow and knee
- ❖ Chest wall instability, deformity, or suspected flail chest
- ❖ Fracture of two or more proximal long bones
- ❖ Crushed, degloved, mangled, or pulseless extremity
- ❖ Amputation proximal to wrist or ankle
- ❖ Pelvic fractures
- ❖ Skull deformity or suspected skull fracture
- ❖ Suspected spinal injury with new motor and/or sensory loss
- ❖ Active bleeding requiring a tourniquet or wound packing with continuous pressure

County of Monterey EMS System Policy 5140

Mechanism of Injury:

- ❖ Falls from height > 10 feet
- ❖ High-risk auto crash
 - Ejection (partial or complete) from automobile
 - Need for extrication for entrapped/pinned patient with significant pain or injuries
 - MVC at > 45 mph with significant pain or injury
 - Death in same passenger compartment
 - Child age 0-9 years unrestrained or in unsecured child safety seat
- ❖ Rider separated from transport vehicle with significant impact (e.g., motorcycle, ATV, horse, etc.)
- ❖ Auto vs pedestrian/bicyclist thrown, run over, or with significant impact

Provider Judgement: Patients who, in the judgment of the evaluating emergency physician, are anticipated to have a high likelihood of requiring emergency surgery or other emergency intervention (within 2 hours of arrival at the Pediatric Trauma Center) requiring the specialized services of a Pediatric Trauma Center.

Pediatric Clinical Signs of Poor Perfusion		Pediatric Glasgow Coma Scale – Verbal Scale < 2 years of age		
Cool, mottled, pale or cyanotic skin		5	Coos and Babbles	
Low urine output		4	Irritable	
Lethargic		3	Only cries to pain	
Prolonged capillary refill		2	Only moans to pain	
		1	None	
Normal Vitals (Broselow)				
AGE	WEIGHT	HEART RATE	SYSTOLIC BP	BROSELOW COLOR
Newborn	3-5 kg	80 – 190	65-104	Grey – Pink
1 Year	10 Kg	80-160	70-112	Purple
3 Years	15 Kg	80-140	75-116	White
5 Years	20 Kg	75-130	80-112	Blue
8 Years	25 Kg	70-120	80-112	Orange
10 Years	30 Kg	65-115	85-126	Green
Important Pediatric Emergency Trauma Re-Triage Exceptions:				
1. Pregnant patients of any age should be transferred to an adult trauma center. 2. Patients with trauma and major burns should be preferentially transferred to a Pediatric Trauma Center with a Burn Center. 3. Contact hospital first for major extremity injuries with vascular compromise.				

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 6030

Effective Date:

Review Date:

EMERGENCY MEDICAL DISPATCH PROVIDER QUALITY IMPROVEMENT

I. PURPOSE

To establish and define the Quality Improvement (QI) aspect of Emergency Medical Dispatch (EMD).

II. POLICY

A. A County of Monterey authorized EMD provider shall have a QI plan that at a minimum includes:

1. System Monitoring.
2. Specific Call Review.
3. Call Review Documentation.
4. Call Audit Procedure.
5. Integration into the EMS system QI program.

B. System Monitoring

1. All calls received for EMD will be recorded and maintained for a minimum of 100 days, as required by California Government Code Section 34090.6.
2. All EMD reports submitted to the EMS Agency shall include at a minimum the following for each call:
 - a. Time of call to include all primary and secondary public safety answering points (PSAPs) time of call entry and acknowledgement of call by primary and secondary PSAP.
 - b. Time of ambulance dispatch
 - c. Time of ambulance en route to call
 - d. Time of ambulance on scene of incident
 - e. Time treatment instructions initiated, if applicable
 - f. Time treatment instructions completed, if applicable

C. Specific Call Review

1. The EMD provider will submit the following items in a monthly report to the County of Monterey EMS Agency:
 - a. The designated Emergency Medical Dispatch Quality Assurance Coordinator (EMD-Q) will review 100% of all choking, CPR, and childbirth calls received in the designated dispatch center for EMD.

- b. The designated EMD-Q will review 100 calls or 3% of all other calls, whichever is greater, received in the designated dispatch center for EMD. Calls reviewed will be randomly selected to include calls from all shifts and all dispatchers.
- c. Periodic specific subject audits as determined by the EMS Agency and/or the designated EMD-Q such as, but not limited to:
 - 1) Review requested by an EMS or EMD provider, or the County of Monterey EMS Agency.
 - 2) Medical Priority Dispatch (MPDS) instructions given to caller
 - 3) Code 2 dispatch that returns Code 3 to the hospital
 - 4) Level of dispatch upgraded after initial dispatch, e.g., Code2 to Code3
 - 5) Hazmat or disaster plans utilized
- D. Call Review Documentation: A list of situations, actions, or deviations from MPDS protocol that the designated EMD-Q questioned, along with the response from the dispatcher and the action taken are to be documented and reported to the EMS Agency.
- E. Call Audit Procedure: The EMD-Q shall utilize the International Academies of Emergency Dispatch (IAED) Performance Standards to review and audit calls.
- F. Confidentiality: All dispatch documentation, recordings, and QI evaluations are subject to review by the County of Monterey EMS Agency. All proceedings, documents, and discussions on EMD QI are confidential and covered under Sections 1040, 1157, 1157.5, and 1157.7 of the California Evidence Code.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 6040
Effective Date:
Review Date:

TRAUMA QUALITY IMPROVEMENT AND SYSTEM EVALUATION

I. PURPOSE

To define standards, evaluate methodologies, and utilize the evaluation results for continuous trauma system quality improvement in performance and patient care, and to establish requirements for data collection and management by trauma system participants in the County of Monterey.

II. POLICY

- A. Trauma system participants within the County of Monterey EMS System will maintain a comprehensive internal Quality Improvement (QI) program, as outlined in County of Monterey EMS System Policy 6000 (Emergency Medical Services Quality Improvement).
- B. Trauma system participants will participate fully in and cooperate with the EMS Agency's QI programs.
- C. All Trauma Centers shall complete a trauma registry entry for all patients who meet the criteria identified in the National Trauma Data Standard (NTDS) Inclusion Criteria and by the California Emergency Medical Services Information System (CEMSIS) trauma requirements.

III. REQUIREMENTS

- A. Trauma Center (Internal) Quality Improvement Requirements:
 - 1. Trauma QI Program – A Trauma Center must have a formal and fully functional quality improvement program for its trauma service. Each Trauma Center shall have a written QI Plan which shall include:
 - a. Trauma Medical Director (Chief of Trauma), who shall be responsible for the hospital trauma care, compliance with the EMS Agency trauma plan/standards, and for participation in the Trauma QI program.
 - b. Trauma Program Manager, who shall be answerable to the Chief Nursing Officer or the Chief Medical Officer. This position shall have at least 1 FTE dedicated to this role. The Trauma Program Manager shall oversee the trauma registrar and will perform the following functions:
 - 1) Perform case reviews of all trauma incidents;
 - 2) Identify trauma cases that meet County of Monterey Minimum Audit Criteria for External QI Review;
 - 3) Analyze trends;

- 4) Analyze all trauma patient calls to the trauma base hospital diverted to non-trauma centers;
 - 5) Audit all trauma deaths, major complications, transfers, unexpected outcomes (positive or negative), and unusual occurrences; and
 - 6) Provide loop closure for identified opportunities for improvement.
- c. Trauma Registrar, who shall maintain the efficient operation of the trauma registry, ensure consistency and quality in the data collection system, enter information into the trauma database, and retrieve data for quality improvement purposes.
 - d. Internal multi-disciplinary trauma committee that includes members of emergency medicine, general surgery, and other departments that are responsible for care of the trauma patient. The audit process will include a log of follow-up problems and periodic multi-disciplinary trauma conferences to critique selected trauma cases. This committee will follow the applicable provisions of Evidence Code Section 1157.7 to ensure confidentiality.
 - e. Provision of a system for patients and others as defined in Title 22, Division 9, Chapter 7, Section 100265(e), to provide input and feedback to hospital staff regarding the care provided.
 - f. The County of Monterey EMS Agency designated Trauma Center's Trauma Medical Director and Trauma Program Manager shall each attend at least 50% of Santa Clara County's Trauma Care System Quality Improvement Committee (TCSQIC) and Trauma Executive Committee meetings.
 - g. Generation and submission of required trauma reports to the EMS Agency within the specified time.

B. Trauma System (External) Quality Improvement Plan

1. Written confidentiality agreements shall be made with system participants regarding participation in the EMS Agency's QI Program.
2. The EMS Agency will conduct audits for compliance with statutory, regulatory, California Emergency Medical Services Authority (EMSA), and contractual compliance every three (3) years, or more frequently as determined by the EMS Agency.
3. Trauma Care System Quality Improvement Committee (TCSQIC)
 - a. The County of Monterey EMS Medical Director and the EMS Agency Trauma Coordinator shall each attend at least 50% of Santa Clara County's TCSQIC and Trauma Executive Committee meetings.
4. The Trauma Evaluation and Quality Improvement Committee (TEQIC) is a multi-disciplinary medical advisory committee to the County of Monterey EMS

Agency, comprised of representatives from prehospital agencies, non-Trauma Center facilities, and the Trauma Center. This is a confidential committee.

- a. The EMS Agency shall conduct TEQIC meetings as deemed necessary, but no less than two (2) times per year.
- b. Oath of Confidentiality: The proceedings and records of this committee are confidential and are protected under Sections 1040 and 1157.7 of the Evidence Code of the State of California. Members shall not divulge or discuss information that would have been obtained solely through TEQIC membership. After review, all paperwork shall be disposed of in an appropriate confidential manner. Members and invited guests of the TEQIC shall sign a Confidentiality Agreement as a condition of attendance. The EMS Agency shall maintain signed copies of the agreements on file.
- c. The Trauma Evaluation and Quality Improvement Committee (TEQIC) shall:
 - 1) Establish audit filters;
 - 2) Monitor the process and outcome of trauma patient care and present opportunities for analysis of data and information of scientific value for studies and strategic planning of the trauma system;
 - 3) Serve in an advisory capacity to the EMS Agency on trauma care systems issues and policies, which include the appropriateness and effectiveness of the Trauma Triage policy; and
 - 4) Provide educational forums for trauma care when trends are identified.
- d. Membership: The membership of the EMS Agency TEQIC shall include the positions included in Section VII of County of Monterey EMS System Policy #1020 (EMS Advisory Committees). Other attendees may be included by special invitation of the County of Monterey EMS Agency.
- e. TEQIC Chairperson: The Chairperson for TEQIC shall be the County of County EMS Agency Trauma Coordinator. The Chairperson shall preside over the committee and make recommendations to the EMS Agency Medical Director as directed by the membership of the committee.
- f. TEQIC Process:
 - 1) TEQIC shall meet a minimum of two (2) times per year for chart review, and jointly for formal education and/or trauma system evaluation according to the needs of the committee.
 - 2) Scope of Review: The review conducted by the committee shall include trauma patient care in Monterey County and transfer of trauma patients to other hospitals or designated Trauma Centers. The committee review shall include and be limited to prehospital

trauma care activities and trauma patient care from time of injury through rehabilitation.

- 3) Preparation of cases for TEQIC review: The Trauma Center shall prepare appropriate materials for its cases to be presented to the TEQIC to include:
 - a) Audit reports as requested by the EMS Agency and/or TEQIC. A formal chart review may be performed by the EMS Agency Medical Director and Trauma Coordinator prior to a TEQIC meeting. A letter will be sent out approximately one month prior to the review of the charts, outlining the scheduling, the procedure, and the trauma charts needing to be pulled for review.
 - b) The field representative shall provide the prehospital provider component for presentation when pertinent to the care of the trauma patient.
- 4) The EMS Agency shall provide:
 - a) Staff support for documentation (minutes) of TEQIC meetings, to include any memorandum(s) issued by the EMS Agency in response to Committee recommendations;
 - b) Distribution of meeting announcements;
 - c) Preparation of TEQIC agenda; and
 - d) Maintenance of records of proceedings.
- g. Conclusion of TEQIC case review: Feedback to prehospital providers, the Trauma Center, and other receiving hospitals is critical to the audit process. Action items will be discussed and decided at the conclusion of each system review. The committee shall discuss each system issue and arrive at a conclusion for action that may include one or more of the following:
 - 1) No further review or action required.
 - 2) Request for additional information and a follow-up report from the involved institution or prehospital care provider.
 - 3) Formal recommendations.
- h. Member removal from the TEQIC: The following shall be cause for removal of a member from the committee:
 - 1) Breach of confidentiality;
 - 2) Excessive absence, defined as failure to attend at least one (1) TEQIC meeting over the course of a calendar year; or
 - 3) Disruptive or rude behavior.
- i. Minimum Audit Criteria for external QI review:

- 1) Absence of a patient care report for a patient transported by prehospital personnel within 24 hours of the transport.
- 2) Transport EMS personnel scene time exceeding 20 minutes of patient care prior to departure to the hospital.
- 3) All trauma patients who are diverted or transferred during the acute phase of hospitalization to another trauma center, acute care hospital, or specialty hospital (*e.g.*, burn center, replantation center, or pediatric trauma center).
- 4) All outgoing trauma transfers performed within 24 hours of hospital arrival.
- 5) Any case the EMS Agency feels would benefit from a TEQIC review.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 6050

Effective Date:

Review Date:

TRAUMA CARE DATA COLLECTION AND MANAGEMENT

I. PURPOSE

To establish requirements for data collection and management by trauma system participants.

II. POLICY

- A. Prehospital providers shall record the American College of Surgeons (ACS) National Guidelines for the Field Triage of Injured Patients findings for all trauma patients, as defined in County of Monterey EMS Policy #4040 (Field Trauma Triage Criteria).
- B. The Trauma Center shall complete a trauma registry entry for all patients who meet the following trauma registry inclusion criteria:
 - 1. ICD-10 codes identified in the Trauma Center's current trauma registry AND
 - 2. Physically evaluated by a trauma surgeon in the emergency department or resuscitation area; OR
 - 3. Death in the emergency department due to a traumatic injury(ies); OR
 - 4. Transfer for trauma services (note: may include interfacility and intrafacility).
 - 5. Exclusion: Isolated burn without penetrating or blunt mechanism of injury.
- C. The registry shall include, but not be limited to, the data elements in compliance with the National Trauma Data Base and the California Emergency Medical Services Information System (CEMSIS) trauma requirements.
- D. The Trauma Receiving facility shall submit data to the EMS Agency on all patients who meet the National Trauma Data Standard Inclusion Criteria. The EMS Agency will define the specific data elements in collaboration with the Trauma Evaluation and Quality Improvement Committee (TEQIC).
- E. EMS base and receiving hospitals that are not designated as a Trauma Center shall submit data to the EMS Agency on all patients who meet the criteria outlined in County of Monterey EMS System Policy #5140 (Emergency Department Re-triage and Rapid Transfer of Trauma Patients to Trauma Center) as well as on other trauma patients as requested by the EMS Agency. The EMS Agency will define the specific data elements in collaboration with the TEQIC.
- F. Cooperation with other counties and LEMSAs:

County of Monterey EMS System Policy 6050

1. When patients from the County of Monterey EMS system are transported to a Trauma Center outside the system or county, Natividad will acquire injury/outcome data.
2. If clinically appropriate, Natividad will attempt to repatriate patients whose condition has stabilized to Natividad.
3. Hospitals and ambulance providers within the County of Monterey shall cooperate with requests from the EMS Agency and other EMS agencies in data collection and evaluation efforts.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 6090

Effective Date:

Review Date:

ANNUAL ALS SKILLS MAINTENANCE VERIFICATION AND POLICY REVIEW

I. PURPOSE

To ensure competency and to mitigate the risks associated with high risk/low frequency skills.

To standardize Monterey County paramedic skill maintenance verification.

To require review of Monterey County EMS System policies, procedures, and protocols by Monterey County accredited paramedics through a mandatory annual policy review.

II. POLICY

- A. All paramedics accredited in Monterey County shall meet the standards for skills maintenance verification and policy review as defined in this policy. The EMS Agency will not provide paramedic accreditation renewal without successful completion of the required skills maintenance and policy review requirement.
- B. The paramedic shall complete the skills maintenance requirements and policy review each calendar year or as specified for the specific procedure.
 - 1. The skills maintenance requirement is waived by the EMS Agency for the year in which initial paramedic accreditation is received when accreditation begins in the last calendar quarter.
 - 2. The policy review requirement is not waived by the EMS Agency but may be met as part of the employer orientation.
 - 3. Each skill must be performed in each calendar year except as described in Item 1 of Subsection II(B) above and for Needle Cricothyrotomy, which must be performed once during the first half of the calendar year and once during the last half of the calendar year. The training videos for needle cricothyrotomy are to be viewed and documented every three months (four times each calendar year).
- C. The paramedic is responsible to ensure that they meet the standards specified in this policy.
- D. Annual Skills and Policy Review Form 6091 shall be submitted by each paramedic reaccrediting in Monterey County for each year of the reaccreditation cycle.
 - 1. ALS Skills Verification Forms 6091A through 6091H shall be maintained by the paramedic for a period of four years from the date of skills verification or until paramedic licensure expires, whichever come first. These forms shall be available for review by the EMS Agency upon request.

III. PROCEDURE

A. ALS skill maintenance:

1. The paramedic shall perform each of the skills listed in the Annual Skills and Policy Review Form 6091 to maintain paramedic accreditation.
2. The paramedic's employer shall submit a comprehensive plan of correction to the EMS Agency for approval by the EMS Agency when the paramedic is unable to perform any of the skills. This plan of correction shall be submitted to the EMS Agency within five (5) business days. The EMS Agency shall respond to the paramedic's employer with plan approval or revision within five (5) business days. Training under the plan of correction shall be completed within fifteen (15) calendar days of the EMS Agency response to the employer. This training shall result in the successful completion of the skills demonstration and policy review process.

B. Policy review:

Annually, the employer shall verify that the paramedic understands Monterey County EMS System policies, procedures, and protocols. Emphasis shall be given to EMS policies, procedures, and protocols that have changed or are new.

C. Skills maintenance and policy review reporting:

1. The paramedic service provider shall verify that the paramedic has met the standards in this policy and Policy 6091 Annual Skills and Policy Review Form 6091. Skills verification will be documented on Form 6091 supported by Forms 6091A through 6091H. Completion of the requirements for the policy review will be documented on Form 6091.
2. Paramedic service provider shall retain these documents for a period of four years.

The paramedic service provider shall provide the paramedic with copies of forms 6091A through 6091H.

D. EMS Agency review of records:

1. The EMS Agency may review all records to ensure compliance with this policy.

END OF POLICY



ANNUAL SKILLS AND POLICY REVIEW FORM-6091



1a. Name as shown on Paramedic License (Last Name, First Name):		1b. License Number	
1c. Signature of person demonstrating competency:		1d. Employer	
Skill	Verification of Competency		
1. Orotracheal Intubation (Adult)	Verifier Affiliation:	Date:	
Signature of person verifying competency	Print Name:	License Number:	
2. Supraglottic Airway (Adult)	Verifier Affiliation:	Date:	
Signature of person verifying competency	Print Name:	License Number:	
3. Supraglottic Airway (Pediatric)	Verifier Affiliation:	Date:	
Signature of person verifying competency	Print Name:	License Number:	
4. Pleural Decompression	Verifier Affiliation:	Date:	
Signature of person verifying competency	Print Name:	License Number:	
5. Transcutaneous Cardiac Pacing	Verifier Affiliation:	Date:	
Signature of person verifying competency	Print Name:	License Number:	
6. Synchronized Cardioversion	Verifier Affiliation:	Date:	
Signature of person verifying competency	Print Name:	License Number:	
7. EZ IO (Adult/Pediatric)	Verifier Affiliation:	Date:	
Signature of person verifying competency	Print Name:	License Number:	
8. Epinephrine for Hypotension (Push Dose)	Print Name:	Date:	
Signature of person verifying competency	Verifier Affiliation:	License Number:	
9. Needle Cricothyrotomy-Adult	Print Name:		Date:
	Videos		
Signature of person verifying competency	Verifier Affiliation:	License Number:	
10. Needle Cricothyrotomy-Pediatric	Print Name:		Date:
	Videos		
Signature of person verifying competency	Verifier Affiliation	License Number:	
11. Annual Protocol/Policy Review	Verifier Affiliation:	Date:	
Signature of person verifying competency:	Print Name:	License Number:	

County of Monterey EMS System Form 6091

A completed Annual Skills and Policy Review Form is required to accompany the paramedic reaccreditation application of every individual who is either maintaining paramedic accreditation without a lapse or renewing paramedic accreditation with a lapse of less than six months.

1a. Name of Paramedic License Holder

Provide the complete name, last name first, of the paramedic demonstrating skills competency.

1b. Paramedic License Number

Provide the paramedic license number of the paramedic demonstrating skills competency.

1c. Signature

Signature of the paramedic demonstrating skills competency. By signing this section, the paramedic is verifying that the information contained on this form is accurate and that the paramedic has demonstrated competency in the skills listed in the skills sheet.

1d. Employer

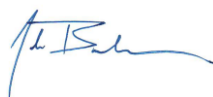
Provide the name of the current paramedic employer(s).

Verification of Competency

1. Verifier Affiliation: Provide the name of the EMS service provider or base hospital that the qualified individual verifying competency is affiliated with.
2. Once competency has been demonstrated by direct observation of an actual or simulated patient contact, i.e. skills station, the individual verifying competency shall sign the Annual Skills and Policy Review Form for that skill.
3. Qualified individuals who verify skills competency shall be currently licensed as a paramedic, registered nurse, physician assistant, or physician and shall be either a qualified instructor designated by an EMS approved training program (paramedic training program or continuing education training program) or a qualified individual designated by an EMS service provider. EMS service providers include, but are not limited to, public safety agencies, private ambulance providers, and other EMS providers.
4. License Number: Provide the certification or license number for the individual verifying competency.
5. Date: Enter the date that the individual demonstrated competency in the skill.
6. Print name – Print the name of the individual verifying competency in the skill.

A completed Annual Skills and Policy Review Form shall be provided to the County of Monterey EMS Agency for every year of the paramedic reaccreditation cycle in compliance with County of Monterey EMS System Policies 2050 6090. A copy of this form shall be retained by the paramedic license holder and the EMS provider agency for a minimum of 4 years as outlined in County of Monterey EMS System Policy 6090.



Orotracheal Intubation		6091-A
Effective: 7/1/2026		Expires: 6/30/2029
Low Frequency/High Risk: Orotracheal Intubation	Approval: Medical Director John Beuerle, MD	Signed: 
Applies To: Paramedics	Approval: EMS Director Debra Hopgood	Signed: 

Terminal Performance Objective

Secure placement of an endotracheal tube (ETT) in the trachea to ensure a patent airway for positive pressure ventilation.

Before performing orotracheal intubation, paramedics must:

1. Determine BLS airway adjuncts are inadequate for effective positive pressure ventilation (PPV) and confirm the need for Endotracheal Intubation (ETI)^{1 2}.
 - a. **NOTE: Orotracheal intubation is approved only for adult patients**
2. Recognize signs of a difficult airway, if present, and select, prepare and employ the appropriate alternative tools and techniques to secure the airway (e.g. ET Introducing Stylet, Rescue Airway).
3. Correctly assemble all equipment required for ETI within 60 seconds.
4. Provide optimal ventilation and oxygenation (minute volume) to the patient while ETI equipment is prepared.
5. Test the cuff of the ET Tube by inflating it with air and ensuring that there is not a leak in the cuff.

While performing orotracheal intubation, paramedics must:

1. Position the patient to best facilitate the intubation.
2. Visualize anatomical structures including the glottic opening (vocal cords) during direct laryngoscopy.
 - a. Use manual percutaneous laryngeal manipulation to assist with visualization of the glottic opening as needed.
3. Minimize oral trauma during laryngoscopy by utilizing correct technique.
4. Place the clinically indicated size ETT securely in the trachea at the correct depth within 30 seconds.
5. Inflate the cuff with enough air to seal the trachea, estimating inflation pressure by palpation of the pilot balloon.
6. Confirm placement of the ETT in the trachea by:
 - a. Direct visualization of the tube passing through the cords.
 - b. IMMEDIATELY attach Waveform capnography and commence ventilation while confirming proper airway placement.
 - 1) Confirm appropriate waveform is present **AND** capnography number is present and consistent with clinical condition or in the event of mechanical failure
 - c. Auscultate lung fields and epigastrium, visualize adequate chest rise and fall.
 - d. Print strip of capnogram and retain for documentation.
 - e. If only right lung sounds heard, carefully adjust ETT as necessary by slowly withdrawing ETT and listening for onset of left lung sounds. Once bilateral lung sounds occur, secure the tube.
 - f. Remove ETT immediately if esophageal placement suspected.
7. Immediately re-establish PPV at the clinically required rate and tidal volume (minute volume) and oxygen at 10-15 LPM following ETT placement.

¹ 2015 AHA Guidelines for CPR and ECC, Part 8 Adult Advanced Cardiovascular Life Support, pp S730-S735

² PHTLS, Ninth Edition

8. Resume BLS PPV within 10 seconds following unsuccessful ETI attempts.
 - a. Employ supraglottic airway after two (2) failed ETI attempts, if BLS airway adjuncts are inadequate for effective PPV.
 - b. Passing the laryngoscope past the teeth with the intent of placing an ETT is considered an intubation attempt.
 - c. A maximum of two (2) attempts per patient is permitted.
9. Secure the ETT in the trachea at the correct depth with tape or a commercial device.
10. Stabilize the patient's airway and prevent tube migration by using a device to prevent rotation, flexion, or extension of patient's head.
11. Efficiently employ post-ETI diagnostic tools to thoroughly assess overall effectiveness of ventilatory support throughout the duration of respiratory management efforts, including:
 - a. Visualize symmetrical rise and fall of the chest with PPV.
 - b. Monitor pulse oximetry – the target SpO₂ is greater than or equal to 94% if spontaneous circulation is present.
 - c. Monitor ETCO₂ for appropriate waveform morphology and target CO₂ levels.
 - 1) The target range for ETCO₂ level is between 30 – 45 mmHg if spontaneous circulation is present.
 - 2) In cardiac arrest, metabolic derangements will significantly alter ETCO₂ values and waveform morphology. Target range for ETCO₂ level is between 15mmHg – 45mmHg during CPR.
 - 3) Recognize that in a patient with traumatic brain injury, ETCO₂ less than 35 mmHg due to hyperventilation may cause harm. Minute volume should be adjusted accordingly while maintaining optimal oxygenation, reserving hyperventilation for those patients showing signs of cerebral herniation only.³
 - d. Monitor ECG for dysrhythmia due to vagal stimulation or other treatable causes.
 - e. Frequent auscultation of lung fields and epigastrium, at a minimum after every patient movement.
 - f. Constant evaluation of ventilatory compliance and resistance during PPV.
12. Immediately identify malfunctioning equipment, ineffective techniques or changes in post-ETI PPV compliance/resistance and employ alternative measures to achieve effective ventilations.
13. Reconfirm correct ETT placement each time the patient is moved and before transfer of care to hospital staff.
 - a. Record and/or print the waveform ETCO₂ strip: after every patient movement, AND just prior to transfer of care to the hospital staff and attach the strip to the completed PCR.
14. Maintain effective ventilation and oxygenation.
15. Accurately document all assessment findings, therapeutic treatments, and the patient's response to therapy.

Critical Success Targets for ETI

1. ETT securely placed in the trachea followed by effective PPV.
2. Chest rise and fall with each ventilation cycle
3. Ventilatory rate and tidal volume appropriate for patient condition and response
4. SpO₂ of greater than 94% in patients with spontaneous circulation
5. Recognition of an esophageal intubation
6. Limited interruption of PPV (30 seconds maximum)
7. Evaluation and documentation of ETCO₂ morphology and values.

System Benchmark

1. ETT securely placed in the trachea within 2 attempts in 85% of the indicated patients
2. Recognition of misplaced or dislodged ETT in 100% of the occurrences
3. Appropriate clinical interpretation of digital waveform capnography with every orotracheal intubation.

Core Competency Requirements to be covered during education/ training on ETI

1. Respiratory A&P and Pathophysiology
2. Assessment of airway and breathing
3. Techniques for PPV
4. Airway pressure secondary to PPV – mean vs. peak
5. Possible complications of PPV – gastric, pulmonary, cerebral, and cardiovascular complications of over-inflation
6. Determination of PPV adequacy and efficacy
7. Differentiation between effective and ineffective patient response to PPV via BLS measures

8. Indications and contraindications for ETI
9. Selection of correct equipment required for ETI (e.g. ET size)
10. Identification of the difficult airway and employment of alternative techniques and tools
11. Laryngoscopy techniques
12. ETT placement techniques
13. Stabilization of patient's head to prevent dislodgement of airway
14. Post-placement ETT monitoring
15. Complications, risks, consequences of failure to complete post-placement ETT monitoring
16. Auscultation and diagnostic differentiation of lung sounds
17. Use of diagnostic tools (*i.e.*, ETCO₂ monitoring)
18. Recognition of complications (**Dislodgement, Obstruction, Pneumothorax, Equipment Failure, or DOPE**)
19. Team leadership and patient safety
20. Documentation

Equipment Requirements

1. Personal Protective Equipment
2. NP/OP Airways
3. BVM
4. Stethoscope
5. Supplemental oxygen
6. Magill forceps
7. Laryngoscope(s)
8. Laryngoscope blades (multiple sizes)
9. Appropriate size ET tubes
10. Stylet(s)
11. Pulse oximeter
12. Waveform capnography
13. Suction device
14. Cardiac monitor
15. Difficult Airway Kit/Rescue Airway Kit (including just in time training aids)

Instructor Resource Materials

1. Prehospital Trauma Life Support
2. AHA CPR and BLS Provider Manual
3. AHA ACLS Provider Manual
4. AHA PALS Provider Manual
5. Current AHA Guidelines for Cardiopulmonary Resuscitation and Emergency Cardiovascular Care
6. NHTSA EMS Educational Instructor Guidelines for EMT and Paramedic

Adult Orotracheal Intubation Validation

PERFORMANCE CRITERIA: 100% accuracy required on all items with an *

Before performing orotracheal intubation, the paramedic must:

successful	unsuccessful	Performance Steps	Additional Information
		Take or verbalize body substance isolation.	Selection: gloves, goggles, mask, gown, booties, N95 PRN
		Determine BLS airway adjuncts are inadequate for effective positive pressure ventilation (PPV) and confirm the need for Endotracheal Intubation (ETI). *	No or inadequate rise and fall of chest, no improvement in patient's color or condition.
		Recognize signs of a difficult airway and select, prepare, and employ the appropriate alternative tools and techniques (e.g., ET Introducing Stylet, Rescue Airway). *	- A difficult airway is defined as the presence of anatomic conditions which preclude direct visualization of the patient's glottic opening. - Signs of a difficult airway include, but are not limited to: 1. Airway edema 2. Arthritis or scoliosis of the spine 3. Significant overbite 4. Small mandible 5. Short neck 6. Morbid obesity 7. C-spine immobilization 8. Face or neck trauma
		Correctly assemble all equipment required for ETI within 60 seconds. *	ETT, stylet, laryngoscope with functioning bulb, Magill forceps, suction, suction catheters (flexible and rigid), 10 mL syringe, stethoscope, Rescue Airways, Toomey Syringe, waveform capnography, pulse oximeter, BVM.
		Provide optimal ventilation and oxygenation to the patient while ETI equipment is prepared. *	
		Test the cuff of the ET Tube by inflating it with air, and ensuring that there is not a leak in the cuff. *	
		Select an appropriate size ET tube	- Adult women will typically require a 6.5 – 7.5 ETT - Adult men will typically require a 7.5 – 8.0 ETT

While performing orotracheal intubation, the paramedic must:

		Consider having a team member apply cricoid pressure during intubation attempts.	Apply gentle pressure to the patient's cricoid cartilage to occlude the esophagus and reduce the patient's chances of aspirating gastric contents.
		Properly position the patient for intubation. *	

		Visualize anatomical structures including the glottic opening (vocal cords) during direct laryngoscopy. *	
		Minimize oral trauma during laryngoscopy by utilizing correct technique. *	Do not use the patient's teeth as a fulcrum.
		Place the appropriately sized ETT securely in the trachea at the correct depth within 30 seconds. *	• • Appropriate depth is ½ -- 1 inch beyond the vocal cords, usually 21 – 23 cm marking at the teeth dependent on tube size
		Inflate the cuff with enough air to seal the trachea, estimating inflation pressure by palpation of the pilot balloon. *	
		Immediately re-establish PPV with the appropriate rate, tidal volume and oxygen at 10 – 15 LPM following ETT placement. *	
		Confirm ETT is in the trachea *	• Direct visualization of the tube passing through the vocal cords • IMMEDIATELY attach waveform capnography and commence gentle bagging while confirming proper airway placement. ○ Confirm appropriate rectangular waveform is present or that the colorimetric detector shows yellow on exhalation • Auscultate over lung fields for confirmation of airflow with PPV ○ If only right lung sounds heard, carefully adjust ETT as necessary by slowly withdrawing ETT and listening for onset of left lung sounds. • Auscultate over the epigastrium for the lack of airflow with PPV. • Print strip of capnogram and retain for documentation. • Observe for appropriate chest rise and fall. • Remove ETT immediately if esophageal placement is suspected.
		Secure the ETT in the trachea at the correct depth with tape or a commercial device and prevent tube migration by securing the patient's head. *	• Stabilize the patient's airway and prevent tube migration by using a device to prevent rotation, flexion, or extension of patient's head.

		Efficiently employ post-ETI diagnostic tools to thoroughly assess overall effectiveness of ventilatory support throughout the duration of respiratory management efforts. *	• Symmetrical rise and fall of the chest with PPV • Monitor pulse oximetry – the target SpO₂ is greater than or equal to 94% if spontaneous circulation is present ○ In patients with COPD/pulmonary disease, it may not be possible or desirable to attain a SpO₂ of 94%. • Monitor ETCO₂ for appropriate waveform morphology and target CO₂ levels. ○ The target range for ETCO₂ level is between 30 – 45 mmHg if spontaneous circulation is present. ○ In cardiac arrest, metabolic derangement will significantly alter ETCO₂ values and waveform morphology. Target range for ETCO₂ levels is between 15 mmHg – 45 mmHg during CPR. ○ Recognize that in a patient with traumatic brain injury, ETCO₂ less than 35 mmHg due to hyperventilation may cause harm. Minute volume should be adjusted accordingly while maintaining optimal oxygenation. • Monitor ECG for dysrhythmia due to vagal stimulation or other treatable causes. • Frequent auscultation of lung fields and epigastrium. • Constant evaluation of ventilatory compliance and resistance during PPV.
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Critical Failure Criteria

- ___ Failure to take or verbalize BSI appropriate to the skill prior to performing the skill
- ___ Failure to initiate ventilations within 30 seconds after applying gloves or interrupts ventilations for greater than 30 seconds
- ___ Failure to ventilate patient at a rate appropriate to patient age
- ___ Failure to provide adequate tidal volume per breath
- ___ Failure to pre-oxygenate patient prior to intubation attempt
- ___ Failure to successfully intubate within 2 attempts
- ___ Failure to disconnect syringe immediately after inflating cuff of ET tube
- ___ Uses teeth as a fulcrum
- ___ Failure to assure proper tube placement by auscultation over lung fields and epigastrium
- ___ Failure to use either a colorimetric end tidal CO₂ cap or waveform capnography
- ___ If used, stylet extends beyond end of tube
- ___ Failure to recognize an esophageal intubation
- ___ Any procedure that would have harmed the patient



Supraglottic Airway		6091B
Effective 7/1/2026		Expires 6/30/2029
Low Frequency Supraglottic Airway	Approval: Medical Director John Beuerle, MD	Signed 
Applies To: Paramedics	Approval: EMS Director Debra Hopgood	Signed 

Performance Objective

Secure placement of a supraglottic airway to facilitate positive pressure ventilation.

Before performing insertion of a supraglottic airway, paramedics must:

1. Determine that BLS airway adjuncts are inadequate for effective positive pressure ventilation (PPV) and confirm the need for alternate means of airway management.
2. Assess the patient for signs of a difficult airway.
 - a. A difficult airway is defined as the presence of anatomic conditions that preclude direct visualization of the patient's glottic opening (e.g., airway edema, arthritis, kyphosis of the spine, significant overbite, small mandible, short neck, morbid obesity, cervical spine immobilization, face or neck trauma).
3. Select and prepare the appropriate supraglottic airway.
4. Within 60 seconds, correctly assemble all equipment required for supraglottic airway insertion.
 - a. Appropriate size iGel:
 - 1) Size 3: For patients 30-60 kg
 - 2) Size 4: For patients 50-90 kg
 - 3) Size 5: For patients > 90 kg
 - b. Lubricant
 - c. Bag-Valve device
 - d. Stethoscope
 - e. Suction
 - f. Monitoring equipment
5. Ensure optimal ventilation and oxygenation of the patient while the supraglottic airway and equipment are being assembled and prepared.

While performing insertion of a supraglottic airway, paramedics must:

1. Minimize oral trauma during insertion by utilizing correct technique.
2. Place the appropriately sized supraglottic airway securely (per the manufacturer's instructions) in the hypopharynx at the correct depth within 30 seconds.
3. Following supraglottic airway placement, immediately re-establish Positive Pressure Ventilation (PPV) with the appropriate rate and tidal volume (minute volume) with supplemental oxygen at 10 – 15 LPM.
4. Confirm correct placement:
 - a. IMMEDIATELY attach waveform capnography and commence gentle bagging while confirming proper airway placement.
 - b. Confirm appropriate rectangular waveform is present.
 - c. Auscultate lung fields and epigastrium.
 - d. Print capnography strip and retain for documentation.
 - e. Observe patient for appropriate chest rise and fall.
5. Secure the supraglottic airway at the correct depth (per the manufacturer's instructions).

6. Stabilize the patient's airway and prevent tube migration by using a device to prevent rotation, flexion, or extension of patient's head.
7. Efficiently employ post supraglottic airway diagnostic tools to thoroughly assess overall effectiveness of ventilatory support throughout the duration of respiratory management efforts, including:
 - a. Visualization of symmetrical rise and fall of the chest with PPV.
 - b. Monitor pulse oximetry. The target SpO₂ is greater than 95% if spontaneous circulation is present.
 - c. Monitor ETCO₂ for appropriate waveform morphology and target CO₂ levels.
 - 1) The target range for ETCO₂ level is between 30 – 45 mmHg if spontaneous circulation is present.
 - 2) In cardiac arrest, metabolic derangements will significantly alter ETCO₂ values and waveform morphology. Target range for ETCO₂ level is between 15 mmHg – 45 mmHg during CPR.
 - 3) Recognize that in a patient with traumatic brain injury, ETCO₂ less than 35 mmHg due to hyperventilation may cause harm to the patient. Minute volume should be adjusted accordingly while maintaining optimal oxygenation, reserving hyperventilation for those patients showing signs of cerebral herniation only.¹
 - d. Monitor ECG for dysrhythmias due to vagal stimulation or other treatable causes.
 - e. Periodically auscultate lung fields and epigastrium.
 - f. Continuously evaluate lung compliance and airway resistance during PPV. Changes in lung compliance or airway resistance may indicate an airway obstruction, pneumothorax, or poor positioning of the supraglottic airway.
8. Re-implement effective PPV within 10 seconds following unsuccessful supraglottic airway placement attempts.
 - a. Rapidly transport patients to the closest Emergency Department when supraglottic airway placement is unsuccessful and airway patency is not secure.
9. Immediately identify malfunctioning equipment, ineffective techniques, or changes in post placement PPV compliance and employ alternative measures to achieve effective ventilations.
10. Reassess supraglottic airway placement each time the patient is moved and before transfer of care to hospital staff.
 - a. Record and print the waveform ETCO₂ strip prior to transfer of care to the hospital staff and attach the strip to the completed PCR.
11. Provide direction to personnel performing PPV following placement of a supraglottic airway.
12. Maintain effective ventilation and oxygenation throughout the entire prehospital treatment period.
13. Maintain calm, and effectively lead a team-based approach to resuscitation under all conditions.
14. Accurately document all assessment findings, therapeutic treatments, and the patient's response to therapy.

Critical Success Targets for use of a Supraglottic Airway

1. Supraglottic airway securely placed in the patient's hypopharynx followed by effective PPV
2. Chest rise and fall with each ventilation cycle
3. Ventilatory rate and tidal volume (minute volume) appropriate for patient condition and response
4. SpO₂ of greater than 95% in patients with spontaneous circulation
5. Limited interruption of PPV (30 seconds maximum)
6. Evaluation and documentation of ETCO₂ morphology and values

System Benchmark

Supraglottic airway securely placed in patient's airway within 2 attempts in 98% of the indicated patients.

Core Competency Requirements to be covered during education/training on Supraglottic Airway

1. Respiratory anatomy, physiology, and pathophysiology
2. Assessment of airway and breathing
3. Techniques for PPV
4. Airway pressure secondary to PPV (mean versus peak airway pressure)
5. Possible complications of PPV (gastric, pulmonary, cerebral, and cardiovascular complications of over-inflation and

¹ The Brain Trauma Foundation's Guidelines for Prehospital Management of Severe Traumatic Brain Injury, Fourth Edition, Section V

over-ventilation)

6. Determination of PPV adequacy and efficacy. Note that greater tidal volume may be necessary due to greater dead space in use of supraglottic airways
7. Differentiation between effective and ineffective patient response to PPV via BLS measures
8. Indications for use of a supraglottic airway
9. Selection of correct equipment required for insertion of a supraglottic airway
10. Identification of the difficult airway and employment of alternative techniques for airway management
11. Supraglottic airway placement techniques
12. Post-placement airway monitoring
13. Auscultation and diagnostic differentiation of lung sounds
14. Use of diagnostic tools (e.g., capnography)
15. Recognition of complications (**Dislodgement, Obstruction, Pneumothorax, Equipment Failure**)
16. Team Leadership and patient safety
17. Documentation

Equipment Requirements

1. Personal protective equipment
2. Adult airway mannequin
3. NPA / OPA airways
4. BVM
5. Supraglottic airway(s)
6. Stethoscope
7. Supplemental oxygen
8. Pulse oximeter
9. Waveform capnography
10. Suction device (both rigid and flexible catheters)
11. Cardiac monitor

Instructor Resource Materials

1. Prehospital Trauma Life Support, 8th Edition
2. AHA CPR and BLS Provider Manual
3. AHA ACLS Provider Manual
4. AHA PALS Provider Manual
5. Current AHA Guidelines for Cardiopulmonary Resuscitation and Emergency Cardiovascular Care
6. NHTSA EMS Educational Instructor Guidelines for EMT and Paramedic
7. Adjunct specific manufacturer guidelines for use

Supraglottic Airway

Successful (y/n)	Performance Steps	Additional Information
	Take or verbalize appropriate body substance isolation precautions	Selection: gloves, goggles, mask, gown, booties, P100 PRN
	Assess whether BLS airway adjuncts are adequate for effective positive pressure ventilation (PPV). Identify the need for ALS airway placement.	
	Recognize signs of a difficult airway. Select and prepare the appropriate supraglottic airway. *	- A difficult airway is defined as the presence of anatomic conditions that preclude direct visualization of the patient's glottic opening. - Signs of a difficult airway include, but are not limited to: - Airway edema - Arthritis or kyphosis of the spine - Significant overbite - Small mandible - Short neck - Morbid obesity - Cervical spine immobilization - Face or neck trauma
	Correctly assemble all equipment required for supraglottic airway insertion within 60 seconds. *	Suction, suction catheters (flexible and rigid), stethoscope, supraglottic airways, waveform capnography, pulse oximeter, BVM
	Ensure optimal ventilation and oxygenation of the patient while supraglottic airway equipment is prepared. *	

While performing insertion of a supraglottic airway, the paramedic must:

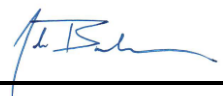
	Minimize oral trauma during insertion by utilizing correct technique. *	
	Place the appropriately sized supraglottic airway securely (per the manufacturer's instructions) in the hypopharynx at the correct depth within 30 seconds. *	
	Immediately re-establish PPV with the appropriate rate and tidal volume (minute volume) with supplemental oxygen at 10 – 15 LPM. *	
	Confirm correct placement. *	- IMMEDIATELY attach waveform capnography and commence gentle bagging while confirming proper airway placement. - Confirm appropriate rectangular waveform is present

		• Auscultate lung fields and epigastrium • Print capnography strip and retain for documentation • Observe the patient for appropriate chest rise and fall
	Secure the supraglottic airway at the correct depth. *	Per manufacturer's instructions
	Stabilize the patient's head to avoid movement and possible supraglottic airway dislodgement.	Stabilize patient's airway and prevent tube migration by using a device to prevent rotation, flexion, or extension of patient's head.
	Efficiently employ post-supraglottic airway diagnostic tools to thoroughly assess overall effectiveness of ventilator support throughout the duration of respiratory management efforts. *	• Visualize symmetrical rise and fall of the chest with PPV • Monitor pulse oximetry. The target is greater than 95% if spontaneous circulation is present. ○ The target range is ETCO2 level is 30 – 45 mmHg if spontaneous circulation is present. ○ In cardiac arrest, metabolic derangements will significantly alter ETCO2 values and waveform morphology. Target range for ETCO2 level is 15 – 45 mmHg during CPR. ○ Recognize that in a patient with traumatic brain injury, ETCO2 less than 35 mmHg due to hyperventilation may cause harm to the patient. Minute volume should be adjusted accordingly while maintaining optimal oxygenation, reserving hyperventilation for those patients showing signs of cerebral herniation only. • Monitor ECG for dysrhythmia due to vagal stimulation or other treatable causes. • Frequent auscultation of lung fields and epigastrium. • Constant evaluation of ventilator compliance and airway resistance during PPV.
	Re-implement effective PPV within 10 seconds following unsuccessful placement attempts. *	Rapidly transport patients to the closest most appropriate hospital when supraglottic airway placement is unsuccessful and airway patency is not secure.
	Immediately identify malfunctioning equipment, ineffective techniques, or changes in post-placement PPV compliance/resistance and employ alternative measures to achieve effective ventilations. *	
	Reassess supraglottic airway placement each time the patient is moved and before transfer of care to hospital staff. *	Record and print the waveform ETCO2 strip prior to transfer of care to the hospital staff. Attach the recording strip to the completed PCR.
	Maintain effective ventilation and oxygenation throughout the entire pre-hospital treatment period. *	Target SpO2 is greater than 95%. Target ETCO2 is 30 – 45 mmHg in a patient with spontaneous circulation.
	Maintain calm, and effectively lead a team-based approach to resuscitation under all conditions	
	Accurately document all assessment findings, therapeutic treatments, and the patient's response to therapy.	

Critical Failure Criteria

- ☐ Failure to take or verbalize BSI appropriate to the skill prior to performing the skill
- ☐ Failure to initiate ventilations within 30 seconds after applying gloves or interrupts ventilations for greater than 30 seconds
- ☐ Failure to ventilate patient at a rate appropriate to patient age
- ☐ Failure to provide adequate tidal volume per breath
- ☐ Failure to pre-oxygenate patient prior to placement of supraglottic airway
- ☐ Failure to successfully place supraglottic airway within 2 attempts
- ☐ Failure to disconnect syringe immediately after inflating cuff of supraglottic airway
- ☐ Failure to assure proper placement by auscultation over lung fields and epigastrium
- ☐ Failure to use waveform capnography
- ☐ Failure to re-check placement after each patient movement and before transfer of care to hospital staff
- ☐ Any procedure that would have harmed the patient



Pleural Decompression		6091-C
Effective 7/1/2026		Expires 6/30/2029
Low Frequency/High Risk: Pleural Decompression	Approval: Medical Director John Beuerle, M.D.	Signed 
Applies To: Paramedics	Approval: EMS Director Debra Hopgood	Signed 

Performance Objective

Relieve intrathoracic pressure due to tension pneumothorax to improve cardiac output, ventilation and oxygenation.

Prior to Needle Pleural Decompression

Assess the patient:

1. Be suspicious of tension pneumothorax in the context of known or suspected torso trauma
2. Be suspicious of spontaneous tension pneumothorax
3. Be suspicious of tension pneumothorax during prolonged artificial ventilation
4. Identify traumatic cardiac arrest with evidence of chest trauma

Recognize and differentiate the signs and symptoms of tension pneumothorax:

1. Hypotension
2. Pleural pain
3. Air hunger
4. Respiratory distress
5. Tachycardia
6. Neck vein distension
7. Tracheal deviation away from the side of the injury
8. Unilateral absence of breath sounds
9. Elevated hemithorax without respiratory movement
10. Cyanosis (late manifestation)

Treat hypoxemia and inadequate ventilation:

1. Position the patient as clinically indicated to meet physiologic requirements
2. Assist breathing as clinically indicated:
 - a. Give only sufficient volume to cause chest rise
3. Use minimum titratable oxygen to reach 94% SpO₂

Recognize and correct confounding factors:

1. Occlusive dressing of open pneumothorax
2. Misplaced endotracheal tube

Confirm the indication for unilateral needle pleural decompression:

1. Signs and symptoms of tension pneumothorax with compromised cardiac output AND rapidly progressing respiratory distress unrelieved by less invasive means

Confirm the indication for bilateral needle pleural decompression:

1. Traumatic cardiac arrest with known / suspected chest trauma

Assemble equipment required for needle pleural decompression

Identify and aseptically mark the appropriate side(s), approach(es), and insertion site(s):

Left, right, or bilateral:

1. Anterior approach: Second intercostal space at the midclavicular line immediately above the third rib
2. Lateral approach: Fourth or fifth intercostal space at the midaxillary line (age 15 and older only)

Note: Inability to positively identify the insertion site is a contraindication to needle pleural decompression.

Prepare the insertion site:

1. Use aseptic technique
2. Swab the site with alcohol, povidone iodine, and/or Chloraprep
3. Confirm use of the clinically indicated personal protective equipment (PPE)
4. Remove the Luer lock or slip tip fitting from the end of the IV needle-catheter set
 - a. Alternatively attach a syringe partially filled with Normal Saline

Needle Pleural Decompression

Perform needle pleural decompression:

1. Firmly but carefully insert the IV Catheter at a 90° angle, just over the superior aspect of the rib, through the skin and pleura until air escapes or a distinct “give” is felt. The undersurface of the rib should be avoided to limit injury to the neurovascular bundle. Air should be freely aspirated, or a rush of bubbles will be in the partially filled syringe.
2. If air is released:
 - a. Withdraw the needle while leaving the catheter in place
 - b. Secure the catheter using bandages and tape
 - c. Reassess signs and symptoms of tension pneumothorax
 - d. Monitor patency of catheter
3. If air is not released:
 - a. Reassess for placement in the pleural space and adjust as needed.
 - b. Withdraw the needle-catheter set and dress the insertion site with an occlusive dressing
 - c. Reassess for signs and symptoms of tension pneumothorax, hemothorax, or other conditions and treat appropriately
 - d. Monitor for iatrogenic pneumothorax

Critical Success Targets

1. Improved cardiac output, including return of pulses, improved skin color, improved BP, improved level of consciousness
2. Chest rise and fall with each breath or ventilation
3. SpO₂ of 94%
4. Improved ETCO₂

Equipment Requirements

1. Personal Protective Equipment
2. Respiratory Protection
3. Medical Exam Gloves
4. Barrier Garment
5. Stethoscope
6. Monitoring and resuscitation equipment
7. Alcohol, Povidone Iodine and/or Chloraprep Swab
8. Syringe and Hypodermic needle
9. Normal Saline 0.9% - 10 mL Prefilled Syringe or Vial
10. Catheter(s) for Needle Pleural Decompression. Options include:
 - a. 3.25-inch 14 g IV Catheter(s)
 - b. 3.25-inch 16 g IV Catheter(s)
 - c. 1.75-2-inch 18 g IV Catheter(s)
11. Portable Sharps Container
12. Gauze Sponge, Pad, Bandage, and/ or Dressing
13. Tape
14. Petrolatum Gauze Dressing
15. Biohazard Bag
16. Waste Bag

Instructor Resource Materials

1. Advanced Trauma Life Support, 11th Edition
2. Prehospital Trauma Life Support, 10th Edition
3. Tactical Emergency Casualty Care, 2nd Edition

Pleural Decompression

Successful (y/n)	Performance Steps	Additional Information
	Use standard, contact, and droplet precautions	Personal protective equipment includes multiple-use eye protection, respiratory protection, barrier garment, and medical exam gloves
	Assess the patient	• Consider tension pneumothorax in the context of known or suspected torso trauma • Consider the possibility of spontaneous tension pneumothorax • Consider tension pneumothorax in traumatic cardiac arrest with chest trauma
	Recognize and differentiate the signs and symptoms of tension pneumothorax *	• Pleural pain • Air hunger • Respiratory distress • Tachycardia • Hypotension • Tracheal deviation away from the side of the injury (late manifestation) • Unilateral absence of breath sounds • Elevated hemithorax without respiratory movement • Neck vein distension • Cyanosis (late manifestation)
	Treat hypoxemia and inadequate ventilation *	• Position the patient as clinically indicated to meet physiologic requirements • Assist breathing as clinically indicated: ○ Give only sufficient volume to cause chest rise • Use minimum titratable oxygen to reach 94% SpO₂
	Recognize and correct confounding factors *	• Tension pneumothorax due to occlusive dressing of open pneumothorax • Misplaced endotracheal tube
	Confirm the indication for needle pleural decompression *	• Unilateral: ○ Signs and symptoms of tension pneumothorax with rapidly progressing respiratory distress unrelieved by less invasive means with s/s compromised cardiac output • Bilateral: ○ Traumatic cardiac arrest with known / suspected chest trauma
	Assemble equipment required for needle pleural decompression	• Personal Protective Equipment • Respiratory Protection • Medical Exam Gloves • Barrier Garment • Stethoscope • Monitoring and resuscitation equipment • Alcohol, Povidone Iodine and/or ChlorPrep Swab • Syringe and Hypodermic needle • Normal Saline 0.9% - 10 mL Prefilled Syringe or Vial • 3.25-inch 14 g IV Catheter(s) for Needle Pleural

		Decompression ○ 3.25-inch 16 g IV Catheter(s) for Needle Pleural Decompression ● 1.75 – 2-inch 18 g IV Catheter(s) for Needle Pleural Decompression ● Portable Sharps Container ● Gauze Sponge, Pad, Bandage, and/ or Dressing ● Tape ● Petrolatum Gauze Dressing ● Biohazard Bag ● Waste Bag
	Identify and aseptically mark the appropriate side(s), approach(es), and insertion site(s) *	● Left, right or bilateral ○ The side(s) requiring needle chest decompression ● Anterior approach: ○ Second intercostal space at the midclavicular line immediately above the third rib ● Lateral approach: (age 15 and older only) ○ Fourth or fifth intercostal space at the midclavicular line <i>The inability to positively identify the insertion site is a contraindication to needle chest decompression.</i>
	Prepare the insertion site *	● Use aseptic technique ● Swab the site with alcohol, povidone iodine, and/or Chloraprep ● Confirm use of the clinically indicated personal protective equipment (PPE) ● Remove the Luer lock or slip tip fitting from the end of the IV needle-catheter set ○ Alternatively, attach a syringe partially filled with Normal Saline
	Perform Needle Pleural Decompression *	● Firmly but carefully insert the IV Catheter at a 90° angle, just over the superior aspect of the rib, through the skin and pleura until air escapes or a distinct “give” is felt. The undersurface of the rib should be avoided to limit injury to the neurovascular bundle. Air should be freely aspirated, or a rush of bubbles will be in the partially filled syringe. ● If air is released: ○ Withdraw the needle while leaving the catheter in place ○ Secure the catheter using bandages and tape ○ Reassess signs and symptoms of tension pneumothorax ○ Monitor patency of catheter ● If air is not released, the catheter is not in the pleural space: ○ Withdraw the needle-catheter set and dress the insertion site with an occlusive dressing ○ Reassess signs and symptoms of tension pneumothorax ○ Monitor for iatrogenic pneumothorax

Critical Failure Criteria

- ☐ **Failure to use standard, contact, and droplet precautions**
- ☐ **Failure to recognize and differentiate the signs and symptoms of tension pneumothorax**
- ☐ **Failure to treat hypoxemia and inadequate ventilation**
- ☐ **Failure to recognize and correct confounding factors**
- ☐ **Failure to confirm the indication for needle pleural decompression**
- ☐ **Failure to identify and aseptically mark the appropriate side(s), approach(es), and insertion site(s)**
- ☐ **Failure to prepare the insertion site**
- ☐ **Failure to perform clinically indicated needle pleural decompression**
- ☐ **Failure to reassess and monitor**
- ☐ **Any procedure that would have harmed the patient**



Transcutaneous Pacing		6091D
Effective 7/1/2026		Expires 6/30/2029
Low Frequency/High Risk: Transcutaneous Pacing	Approval: Medical Director John Beuerle, MD	Signed 
Applies To: Paramedics	Approval: EMS Director Debra Hopgood	Signed 

Performance Objective

Electrical capture and control of the mechanical contraction of the heart resulting in adequate cardiac output and tissue perfusion.

Before performing transcutaneous cardiac pacing (TCP), paramedics must:

- Methodically assess patient's ABC's within 30 seconds.
- Determine the patient is hemodynamically unstable due to bradycardia and is a candidate for immediate TCP by confirming all the following are present:
 - The patient is exhibiting signs and symptoms of systemic poor perfusion, *and*
 - Bradycardia is present on the ECG, *and*
 - Underlying causes of the dysrhythmia have been considered, and reversible causes have been treated (*e.g.*, bradydysrhythmias in children are usually respiratory related), *and*
 - Confirm no contraindications to TCP are present, *and*
 - The cardiac monitoring equipment is placed correctly on patient, and a baseline rhythm strip is printed.
 - Consider performing a 12 Lead ECG prior to initiating TCP if such delay does not cause harm to the patient.
- Identify contraindications for TCP:
 - Children less than or equal to 15 kg
 - Asystolic cardiac arrest, unless approved by base hospital physician.
- Prepare for TCP
 - Administer Atropine, if indicated by heart rhythm, while preparing pacer. Do not delay TCP if there is difficulty establishing an IV.
 - Use TCP without delay for high-degree block (2nd degree type II (Mobitz II) AV block or 3rd degree AV block)
- Explain to patient/family what they can expect to feel and to see but do not delay immediately needed treatment.
- Once capture is obtained with TCP, strongly consider Versed for sedation and amnesic effect of conscious patient.
 - Use IN/IM route for sedation if IV access is poor and would delay TCP.

While performing transcutaneous cardiac pacing (TCP), paramedics must:

- Apply the ECG defibrillation/pacer multifunction pads (MFP) firmly to the patient's clean, bare skin in the correct anatomical locations for maximum electrical current flow through the heart.
 - Proper pad placement on cleaned, dry skin is essential to minimize pain (heat generated from passage of current through the skin) and maximize current conduction. The better the contact, the more effective pacing will be.
- Identify a patient with a pacemaker or automatic internal cardiac defibrillator (AICD) and place the MFP(s) in alternate position(s) to minimize damage to the device(s) and disruption of current flow through the heart.
- Place the ECG monitor into pacing mode.
- Turn Pacer on and set initial Rate and Current values for procedure (60 bpm and 10 mA). Gradually increase current in increments of 10 mA until electrical capture is gained. (*i.e.*, pacer spike generates a QRS complex on the ECG).
- Once capture has been obtained, increase the output by 10%.

6. Determine and utilize minimum electrical current needed to maintain capture (*i.e.*, decrease current by 5 mA increments until pulses capture lost, increase current by 5 mA increments until capture/pulses regained).
7. Confirm mechanical capture by palpating pulses that match pacemaker.
8. Evaluate the effectiveness of TCP by assessing the patient's level of consciousness and vital signs for improvement.
9. Identify continuing signs and symptoms of poor perfusion (including but not limited to hypotension, altered level of consciousness, chest pain, dyspnea/tachypnea, diaphoresis, pale, cool skin) despite effective mechanical capture and increase TCP rate in increments of 10 bpm to a max of 80 bpm to increase cardiac output.
10. Provide Versed per protocol for sedation and amnesic effect as clinically required.
11. Continuously re-assesses the patient's vital signs and level of consciousness throughout the prehospital period of treatment.
12. Contact the base hospital if signs and symptoms of poor perfusion persist.
13. Properly document procedure, printing paced rhythm strip and attach it to the PCR.

Critical Success Targets for TCP

1. Electrical and mechanical capture
2. Resolution of patient's bradycardia related signs and symptoms (hypotension, skin signs, level of consciousness, dyspnea/tachypnea, chest pain).

System Benchmark

Percentage of patients that experience mechanical capture with signs of improved cardiac output (*i.e.* improved level of consciousness/mentation, peripheral pulses, BP, skin signs).

Core Competency Requirements to be covered during education/ training on TCP

1. Assess patient to determine if appropriate indications are present (hemodynamically unstable patients with bradycardia, Mobitz II, or 3rd AV blocks).
2. Assess patient for contraindications (patients 15 kg or less, or patients in asystole)
3. Proper placement of ECG electrodes on patient
4. Proper identification of cardiac dysrhythmia(s) requiring TCP
5. Proper placement of multi-function pads on patient
6. Assessment for and recognition of hemodynamic instability
7. Verbalize possible treatments for hemodynamically unstable bradycardias other than TCP
8. Explain procedure to patient (where applicable)/ pre-medicate patient (where applicable)
9. Demonstrate proper technique for setting rate and current
10. Demonstrate proper technique for gaining electrical capture with minimum required current
11. Describe how to obtain mechanical capture if not gained with initial electrical capture
12. Cardiac monitoring/ rhythm recognition and treatment

Equipment Requirements

1. PPE
2. COR mannequin(s)
3. Stethoscope
4. Cardiac monitor/ECG/Defibrillator
5. ECG Rhythm Generator
6. ECG electrodes
7. Defibrillation/ Multifunction pads
8. Versed
9. Pre-medication equipment (IV access, IN equipment, IM equipment)

Instructor Resource Materials

1. AHA ACLS Provider Manual
2. AHA PALS Provider Manual
3. Current AHA Guidelines for Cardiopulmonary Resuscitation and Emergency Cardiovascular Care
4. NHTSA EMS Educational Instructor Guidelines for EMT and Paramedic

Transcutaneous Cardiac Pacing

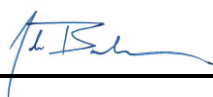
Successful (y/n)	Performance Steps	Additional Information
	Take or verbalize body substance isolation.	Selection: gloves, goggles, mask, booties, gown, N95 PRN
	Methodically assess patient's ABC's within 30 seconds.*	
	Determine the patient is hemodynamically unstable due to bradycardia and is a candidate for immediate Transcutaneous Pacing (TCP)*	Confirm that <i>all</i> of the following are present: • The patient is exhibiting signs and symptoms of systemic poor perfusion; <i>and</i> • Symptomatic bradycardia is present on the ECG; <i>and</i> • Underlying causes of the dysrhythmia have been considered and reversible causes have been treated; <i>and</i> • No contraindications of TCP are present.
	Place cardiac monitoring equipment on the patient correctly and print a baseline rhythm strip.	Consider performing a 12 Lead ECG prior to initiating TCP, if such delay does not cause harm to the patient.
	Identify contraindications for TCP.	• Children less than or equal to 15 kg (bradydysrhythmias in children are usually respiratory related)
	Prepare for TCP.	• Administer Atropine if indicated while preparing pacer. ◦ Do not delay TCP if there is difficulty establishing an IV. • Use TCP without delay for high-degree block (2nd degree type II (Mobitz II) AV block or 3rd degree AV block)
	Explain to patient/family what they can expect to feel and to see.*	Do not delay immediately needed treatment.
	Strongly consider Versed for sedation/amnesic effect while preparing TCP equipment.	Use IN/IM route for Versed for sedation/amnesic effect if IV access is poor and would delay TCP.

	Apply the ECG monitor/pacer multifunction pads (MFP) firmly to the patient's clean, bare skin in the correct anatomical locations for maximum electrical current flow through the heart. *	• Anterior-posterior placement is recommended, if possible. • Proper pad placement on the patient's cleaned, dry skin is essential to minimize pain (heat generated from passage of current through the skin) and maximize current conduction. The better the contact, the more effective pacing will be.
	Identify a patient with a pacemaker or automatic internal cardiac defibrillator (AICD) and place the MFP(s) in alternate position(s) to minimize damage to the device(s) and disruption of current flow through the heart *	
	Correctly place the ECG monitor into pacing mode. *	
	Turn the pacer on and accurately set initial rate and current values for procedure. *	60 beats per minute (bpm) and 10 mA
	Gradually increase current until electrical capture is gained *	i.e., a Pacer spike generates a QRS complex on the ECG
	Confirm mechanical capture by palpating pulses that match pacemaker. *	
	Evaluate the effectiveness of TCP*	Assess the patient's mentation and vital signs for improvement.
	Identify continuing signs and symptoms of poor perfusion despite effective mechanical capture and increase TCP rate in increments of 10 bpm to a maximum of 100 bpm to increase cardiac output.	Signs and symptoms of poor perfusion include but are not limited to: • Hypotension • Altered level of consciousness • Chest pain • Dyspnea/tachypnea • Diaphoresis • Pale/cool skin
	Provide Versed for sedation/amnesic effect and Morphine for pain as clinically indicated per protocol.	
	Accurately document all assessment findings, therapeutic treatments, and the patient's response to therapy.	Print paced rhythm strip and attach it to the PCR.

Critical Failure Criteria

- ☐ **Failure to take or verbalize BSI appropriate to the skill prior to performing the skill**
- ☐ **Failure to identify indications/contraindications for procedure**
- ☐ **Failure to ensure the functionality of cardiac monitor and availability of equipment**
- ☐ **Failure to adjust current and rate appropriately**
- ☐ **Failure to confirm efficacy of intervention – using electrical and mechanical capture**
- ☐ **Any procedure that would have harmed the patient**



Synchronized Cardioversion		6091-E
Effective 7/1/2006		Expires 6/30/2029
Low Frequency/High Risk: Synchronized Cardioversion	Approval: Medical Director John Beuerle, M.D.	Signed 
Applies To: Paramedics	Approval: EMS Director Debra Hopgood	Signed 

Performance Objective

Termination of hemodynamically significant tachycardia resulting in restoration of adequate cardiac output and tissue perfusion.

Before performing synchronized cardioversion, paramedics must:

1. Methodically assess the patient's ABC's within 30 seconds.
2. Determine the patient is hemodynamically unstable due to idiopathic (non-compensatory) tachycardia and is a candidate for immediate cardioversion:
 - a. Confirm the patient is exhibiting signs and symptoms of systemic poor perfusion (including but not limited to hypotension, altered mental status, chest pain, dyspnea/tachypnea, diaphoresis, pale/cool skin).
 - b. Confirm tachycardia (HR greater than 150 in adults, greater than 180 in children, greater than 220 in infants) is present on the ECG.
 - c. Confirm underlying causes of the dysrhythmia have been considered and reversible causes have been treated.
3. Provide supplemental oxygen in high concentration (10 – 15 LPM).
4. Confirm the ECG monitor leads have been placed appropriately.
5. Differentiate between wide and narrow complex tachycardia.
 - a. Print Lead II strip prior to performing any medical treatment as this could appear to be a wide complex rhythm when in fact it is a paced rhythm (some monitors do not show pacer spikes).
 - b. Consider performing a 12 Lead ECG prior to cardioversion if such delay does not cause harm to the patient.
6. Strongly consider midazolam for sedation/amnesic affect for alert patients while preparing cardioversion equipment, but do not delay cardioversion in an unstable patient presenting with signs and symptoms of poor perfusion (hypotension, decreased LOC, chest pain, dyspnea/tachypnea, diaphoresis, pale/cool skin).
 - a. If IV access is delayed, consider faster alternate routes of administration for midazolam (IN/IM).
7. Explain to patient/family what they can expect to feel and to see while avoiding delays in treatment.

While performing synchronized cardioversion, paramedics must:

1. Select and prepare the appropriate sites for application of the ECG monitor/defibrillator multifunction pads.
 - a. Proper pad placement on cleaned, dry skin is essential to minimize pain (heat generated from passage of current through the skin) and maximize current conduction. The better the contact, the more effective attempts at cardioversion will be.
2. Apply the ECG monitor/defibrillator multifunction pads (MFP) firmly to the patient's clean, bare skin in the correct anatomical locations for maximum electrical current flow through the heart.
3. Identify a patient with a pacemaker or automatic internal cardiac defibrillator (AICD) and place the MFP(s) in alternate position(s) to minimize damage to the device(s) and to avoid disruption of current flow through the heart.
4. Place the ECG monitor/defibrillator in synchronize mode.
5. Confirm the monitor is tracking the R wave for delivery of synchronized current.

6. Select the correct energy setting on the ECG monitor/defibrillator per the Monterey County EMS agency protocols
7. Assure everyone is clear from the patient and all possible energy conducting surfaces/contacts.
8. Discharge the defibrillator for synchronized delivery of electrical current.
9. Immediately re-assess the patient.
10. Perform and print a 12 Lead ECG and attach to PCR.
11. Provide treatment based upon re-assessment findings.

Critical Success Targets for Synchronized Cardioversion

1. Improvement in patient level of consciousness
2. Improved signs of perfusion
3. Resolution of patient's tachycardia-related signs and symptoms (chest pain)
4. ECG return to normal sinus rhythm or sinus tachycardia
5. Proficient use of the ECG monitor/defibrillator including lead and MFP placement

System Benchmark

Percentage of patients receiving cardioversion with restoration of a stable perfusing rhythm

Core Competency Requirements to be covered during education/training on synchronized cardioversion

1. Cardiovascular A & P
2. Cardiology – Pathophysiology of tachycardias
3. Assessment of circulation and recognition of hemodynamic instability
4. Identification and contraindications for synchronized cardioversion
5. Proper placement of ECG electrodes on patient
6. Proper placement of multi-function pads on patient
7. Patient communication techniques
8. Pre-cardioversion midazolam for sedation and amnesic effect
9. Demonstrates proper technique for use of the ECG monitor/defibrillator for cardioversion
10. Post-cardioversion cardiac monitoring/ rhythm recognition and treatment
11. Reassessment of patient

Equipment Requirements

1. PPE
2. CPR mannequin(s)
3. Stethoscope
4. Cardiac monitor/ECG/Defibrillator
5. ECG Rhythm Generator
6. ECG electrodes
7. Defibrillation/ Multifunction pads
8. Midazolam
9. Pre-medication equipment (IV access, IN equipment, IM equipment)

Instructor Resource Materials

1. AHA ACLS Provider Manual
2. AHA PALS Provider Manual
3. Current AHA Guidelines for Cardiopulmonary Resuscitation and Emergency Cardiovascular Care
4. NHTSA EMS Educational Instructor Guidelines for EMT and Paramedic Synchronized Cardioversion Validation

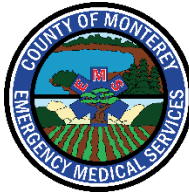
Synchronized Cardioversion

Successful (y/n)	Performance Steps	Additional Information
	Take or verbalize appropriate body substance isolation.	
	Methodically assess the patient's ABC's within 30 seconds. *	
	Determine the patient is hemodynamically unstable due to idiopathic (non-compensatory) tachycardia and is a candidate for immediate cardioversion. *	• Confirm the patient is exhibiting signs and symptoms of systemic poor perfusion: ○ Hypotension ○ Altered mental status ○ Chest pain ○ Dyspnea/tachypnea ○ Diaphoresis ○ Pale/cool skin • Confirm tachycardia is present on the ECG ○ Heart rate greater than 150 in adults ○ Heart rate greater than 180 in children ○ Heart rate greater than 220 in infants • Confirm underlying causes of the dysrhythmia have been considered and reversible causes have been treated.
	Provide supplemental oxygen	
	Confirm the ECG monitor leads have been placed appropriately.	
	Differentiate between wide and narrow complex tachycardia.	• Print a Lead II strip prior to performing any medical treatment as this could appear to be wide complex rhythm when in fact it is a paced rhythm (some monitors do not show pacer spikes) • Consider performing a 12 Lead ECG prior to cardioversion if such delay does not cause harm to the patient.
	Consider midazolam for sedation/amnesic effect while preparing cardioversion equipment.	• Do not delay cardioversion in an unstable patient presenting with signs and symptoms of poor perfusion. • Use IN/IM route midazolam for sedation/amnesic effect if IV access is poor.
	Explain to the patient/family what they can expect to feel and to see.	Do not delay immediately needed treatment.
	Apply the ECG monitor/defibrillator multifunction pads (MFP) firmly to the patient's clean, bare skin in the correct anatomical locations for maximum electrical current flow through the heart. *	• Anterior-posterior placement is recommended, if possible. • Proper pad placement on cleaned, dry skin is essential to minimize pain (heat generated from passage of current through the skin) and maximize current conduction. The better the contact, the more effective conduction will be. • Identify if patient with a pacemaker or automatic internal cardiac defibrillator (AICD) and place the MFP(s) in alternate position(s) to minimize

		damage to the device(s) and to avoid disruption of current flow through the heart
1	Correctly place the ECG monitor/defibrillator in synchronize mode. *	
1	Confirm the monitor is tracking the R wave for delivery of synchronized current *	
1	Select the correct energy setting on the ECG monitor/defibrillator.	
1	Assure everyone is clear from the patient and all possible energy conducting surfaces/contacts. *	
1	Discharge the defibrillator for synchronized delivery of electrical current. *	
	Immediately reassess the patient.	
	Perform a 12 Lead ECG and print a rhythm strip	Attach the rhythm strip to your PCR.
	Provide treatment based upon reassessment findings.	

Critical Failure Criteria

- ___ Failure to take or verbalize BSI appropriate to the skill prior to performing the skill
- ___ Failure to identify indications for procedure
- ___ Failure to ensure the functionality of cardiac monitor and availability of equipment
- ___ Failure to ensure the cardiac monitor/defibrillator is in synch mode, resulting in delivery of a non-synchronized defibrillation shock
- ___ Failure to assure that everyone is clear from the patient and all possible energy conducting surfaces/contacts.
- ___ Failure to confirm efficacy of intervention
- ___ Any procedure that would have harmed the patient



EZ-IO		6091-F
Low Frequency/High Risk: EZ-IO	Effective 7/1/2026	Expires 6/30/2029
	Approval: Medical Director John Beuerle, M.D.	Signed 
Applies To: Paramedics	Approval: EMS Director Debra Hopgood	Signed 

Performance Objective

To establish, maintain, and ensure intraosseous vascular access for the delivery of drugs and fluids.

Before establishing intraosseous access, paramedics must:

1. Confirm that intraosseous access is clinically indicated and required for emergency stabilization.
2. Do not place if any of these contraindications to intraosseous access are present:
 - a. Fracture of insertion site bone
 - b. Inability to clearly identify insertion site (absence of anatomical landmarks / excess tissue / other problem)
 - c. Infection at insertion site
 - d. Previous orthopedic procedure at insertion site (IO attempt within last 48 hours / joint replacement / prosthetic)

While establishing intraosseous access, paramedics must:

1. Select the most appropriate insertion site based on clinical needs, operational requirements, and the available device:
 - 1) EZ-IO Power Driver may be used by paramedics while following a standing order at these insertion sites: Proximal tibia in adults & pediatrics and proximal humerus in adults.
2. Identify the insertion site using anatomical landmarks.
3. Select the size of needle set or device based on patient size, the selected insertion site, and tissue depth.
4. Cleanse and prepare the insertion site using aseptic technique.
5. Assemble and prepare equipment using aseptic technique.
6. Establish IO access by following the manufacturer's instructions for the available device:
 - a. EZ-IO with pink, blue, or yellow needle sets:
 - 1) https://www.teleflex.com/usa/en/product-areas/emergency-medicine/intraosseous-access/arrow-ez-io-system/literature/VA_IOS_EZ-IO_Pocket_Guide.pdf
 - 2) <http://www.ezioapp.com>
 - 3) <https://www.youtube.com/watch?v=Wu-KVibUGNM>
7. Verify Intraosseous placement:
 - a. Needle is free standing, without support
 - b. Aspiration of blood/marrow
 - c. Infusion without extravasation
8. Attach IV set and adjust infusion rate as clinically indicated.
 - a. Despite proper placement, pressure infusion may be needed to achieve the required flow rates.
 - b. Give Lidocaine 2% as ordered by the applicable treatment protocol or a base hospital order for infusion pain in the conscious patient.
9. Secure the insertion site and splint the limb to maintain intraosseous vascular access despite scene activity, patient movement, and transport.
10. Routinely reassess the insertion site to ensure vascular access.
11. Document the procedure.

Critical Success Targets for IO

1. Aspiration of blood/marrow from the intraosseous access site
2. Infusion without extravasation

Core Competency Requirements to be covered during education/ training on EZ-IO

1. Assessment
2. Anatomy and physiology
3. Differences between neonatal, pediatric, adult, and bariatric patients
4. Indications and contraindications
5. Equipment and site selection
6. Precautions and complications

Equipment Requirements

1. Personal protective equipment
2. An intraosseous access device and EZ-IO with pink (15g / 15 mm), blue (15g / 25 mm), and yellow (15 g/45 mm) needle sets
3. 10 mL syringe containing 3 to 5 mL of normal saline
4. Normal saline
5. IV administration set
6. Tape
7. Antiseptic
8. Gauze/Securing Device

Instructor Resource Materials

1. Paramedic Care Principles & Practice, Third Edition, Bledsoe, Porter, Cherry

Intraosseous Infusion

Score	Performance Steps	Additional Information
	Take or verbalize body substance isolation. *	Selection: gloves, goggles, mask, gown, booties, P100 PRN
	Confirm that intraosseous access is clinically indicated. *	
	Do not place if contraindications to intraosseous access are present. *	Fracture of insertion site bone Inability to clearly identify insertion site (absence of anatomical landmarks / excess tissue / other problem) Infection at insertion site Previous orthopedic procedure at insertion site (IO attempt within last 48 hours / joint replacement / prosthetic)
	Select the most appropriate insertion site based on clinical needs, operational requirements, and the available device.	EZ-IO Power Driver may be used while following a standing order at these insertion sites: Proximal tibia in adults & pediatrics Proximal humerus in adults only
	Identify the insertion site using anatomical landmarks.	
	Select the size of needle set or device based on patient size, the selected insertion site, and tissue depth.	EZ-IO with pink (15g / 15 mm), blue (15g / 25 mm), and yellow (15g / 45 mm) needle sets
	Cleanse and prepare the insertion site using aseptic technique. *	
	Assemble and prepare equipment using aseptic technique. *	

	Establish intraosseous access by following the manufacturer's instructions for the available device.	
	Verify Intraosseous placement.	1. Needle is free standing, without support 2. Aspiration of blood/marrow 3. Infusion without extravasation
	Attach IV set and adjust infusion rate as clinically indicated.	Despite proper placement, pressure infusion may be needed to achieve the required flow rates.
	Secure the insertion site	
	Routinely reassess the insertion site to ensure vascular access. *	
	Document the procedure.	

Critical Failure Criteria

- ___ Failure to take or verbalize BSI appropriate to the skill prior to performing the skill.
- ___ Failure to confirm that intraosseous access is clinically indicated.
- ___ Placement when any contraindication to intraosseous access is present.
- ___ Failure to identify the insertion site using anatomical landmarks.
- ___ Failure to select the size of needle set or device based on patient size, the selected insertion site, and tissue depth.
- ___ Failure to cleanse and prepare the insertion site using aseptic technique.
- ___ Failure to assemble and prepare equipment using aseptic technique.
- ___ Failure to follow the manufacturer's instructions for the available device.
- ___ Failure to attach IV set and adjust infusion rate as clinically indicated.
- ___ Failure to secure the insertion site and splint the limb.
- ___ Failure to routinely reassess the insertion site to ensure vascular access.
- ___ Any procedure that would have harmed the patient

County of Monterey EMS System Policy



Policy Number: 6170

Effective Date:

Review Date:

PILOT PROGRAMS

I. PURPOSE

- A. To define the process by which the County of Monterey EMS Agency plans, develops, implements, and monitors Pilot Programs.

II. POLICY

- A. The EMS Agency Director and EMS Medical Director must review and approve all Pilot Programs prior to implementation.
- B. Pilot Program studies are typically small scale and short term for the purpose of evaluating quality indicators and/or operational improvements for local EMS policies and/or protocols.
- C. Projects involving any of the following will be considered research and require approval according to the County of Monterey EMS System Policy #6160 (Research Studies):
 - 1. Changes in the State EMS Authority Paramedic Scope of Practice or an untested intervention.
 - 2. The goal of the project is to test a hypothesis.
 - 3. The investigators intend to submit the results for publication in a professional journal.
 - 4. When questions arise as whether a project is quality improvement versus research, the investigator will consult with the Institutional Review Board.
- D. The Pilot Program Investigator(s) shall submit a Pilot Program Proposal to the EMS Agency at least three (3) months prior to the planned implementation date in order to allow time for the Pilot Program Review Process.
 - 1. The EMS Agency shall review the Pilot Program Proposal and solicit EMS Stakeholder input through the most appropriate EMS Advisory Committee, either the Clinical Care Committee (CCC) or the Operations Working Group (OPS), or both. Recommendations from the EMS Agency and the EMS Advisory Committee(s) shall be presented to the Emergency Medical Care Committee (EMCC) for review, discussion, and recommendations.
 - 2. The Pilot Program Proposal shall include the following:
 - a) Background/Significance: Describe the rationale for the Pilot Program, citing relevant research. Identify what questions remain and how the

proposed Pilot Program will address these questions. Specify how the Pilot Program may improve the quality of care in the EMS System.

- b) Objectives: List the objectives upon which the outcome of the Pilot Program will be based. Identify the predictor and outcome variables and the expected outcome of the Pilot Program.
- c) Design/Methods: Identify the type of study, the outcome variables to be measured, and how each outcome variable is an indicator of quality of care. Describe the methods used to collect the data and avoid bias.
- d) Evaluation: Describe the data management and statistical methods that will be used to evaluate the data. Include methods used to minimize bias and evaluate standards or benchmarks proposed to accept the conclusions.
- e) Conflicts of Interest: Identify and describe any proprietary, personal, familial, social, professional, or financial (*e.g.*, salary, consultant payments, honoraria, royalty payments, dividends, loans, or any other payments or remunerations) conflicts of interest.
- f) References: Provide copies of references cited in the proposal.

III. PILOT PROGRAM REVIEW PROCESS

- A. The EMS Agency Director and EMS Medical Director shall review the Pilot Program Proposal and will present the Pilot Program to the most appropriate EMS Advisory Committee(s) (CCC and/or OPS) for review and discussion.
- B. Feedback from the reviewing body will be incorporated into the EMS Agency's presentation of the Pilot Program to EMCC, which shall review the proposal, provide feedback, and vote to recommend approval, modifications, or disapproval of the proposed Pilot Program. Recommendations from EMCC and the reviewing body shall be considered by the EMS Medical Director, who will make the final determination on whether to approve or deny the implementation of the Pilot Program.
- C. If approved, the data collection period is one (1) year or as determined by the EMS Medical Director.

IV. REPORTING PROCESS

- A. Monthly Reports: The Pilot Program Investigator(s) shall submit monthly reports to the EMS Agency on the 15th day of the following month for the duration of the Pilot Program. These reports must summarize the progress of the program and evaluate available outcome data from the previous month. The need for quarterly reports in addition to or instead of monthly reports will be determined by the EMS Medical Director.
- B. Final Report: At the end of the data-collection period, the Investigator(s) must submit a final report to the EMS Agency Medical Director. The final report must include a summary of the Pilot Program including the objectives, methods, data analysis of the outcome variables, limitations of the study, and conclusions. The final report is due no

later than the 15th day of the 3rd month following completion of the data-collection period.

V. EARLY DISCONTINUATION

- A. If, under the judgment of the EMS Medical Director, patient safety concerns or preliminary data demonstrate compelling evidence warranting suspension or early discontinuation of the Pilot Program, the EMS Agency Medical Director may suspend or discontinue the Pilot Program at any time.

VI. COMPLETION OF THE PILOT PROGRAM

- A. The EMS Agency will present the results of the Pilot Program to EMCC and the most appropriate EMS Advisory Committee(s) (CCC and/or OPS). Members may offer recommendations pertaining to continuation, modification, or discontinuation of the program.
- B. The EMS Medical Director will assign one of the following designations to the Pilot Program:
 - 1. Approved – The Pilot Program demonstrates improved quality of care or benefits the EMS System and its patients, as determined by the EMS Medical Director. The program may be modified or continued under a Local Optional Scope of Practice or revision to EMS policies and/or treatment protocols, pursuant to Monterey County EMS System Policy #1000 (Policy and Protocol Development Process).
 - 2. Extended (up to one year) – There is insufficient data to evaluate the impact of the Pilot Program on quality of care or the EMS System. The data-collection period shall be extended for a defined period of time not to exceed an additional one year from completion of the initial data-collection period.
 - 3. Discontinued – The Pilot Program does not demonstrate improved quality of care or benefit the EMS System and its patients, as determined by the EMS Medical Director. The Pilot Program will be discontinued.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 6190

Effective Date:

Review Date:

COLLECTION AND SUBMISSION OF EMS DATA

I. PURPOSE

To establish requirements for the collection and submission of data to the County of Monterey EMS Data System by EMS provider agencies..

II. POLICY

- A. All County of Monterey EMS provider agencies, including first responder agencies and ambulance transport providers, shall utilize either the designated electronic patient care report (ePCR) ePCR system provided by the County of Monterey EMS Agency or an ePCR system capable of integrating seamlessly with the EMS Agency's Data System. All data shall meet California Emergency Medical Services Information System (CEMSIS) and National Emergency Medical Services Information System (NEMSIS) standards.
- B. The designated Monterey County EMS Data System is the primary system for the collection and submission of EMS data in Monterey County and is the only authorized data system for the submission of data to CEMSIS and NEMSIS.

III. EMS PROVIDER AGENCIES UTILIZING COUNTY ePCR SYSTEM

- A. EMS provider agencies shall use the designated ePCR system to document all EMS responses/calls/patient information as required by EMS System Policies, Protocols, and procedures.
- B. EMS provider agencies shall maintain hardware/software as identified in manufacturers' and vendors' recommendations, including all system and security updates.
- C. EMS provider agencies are responsible for all costs of modules not related to the designated EMS Data System, such as fire reporting, staffing, scheduling, and inventory management.
- D. EMS provider agencies shall use the designated County of Monterey EMS Data System to collect, analyze, and report provider-specific data as required by EMS System Policies, system quality improvement efforts, and applicable state requirements such as CEMSIS.
- E. EMS provider agencies shall have access to their organization's data and other data, as provided by the EMS Agency for system quality improvement efforts.
- F. In the event of an incident where an EMS provider agency notices that the reporting system is not operating properly, that agency shall file a support claim to the vendor. Afterwards, they shall notify the EMS Agency with a copy of the submitted support claim.

- G. EMS provider agencies shall participate in the Continuous Quality Improvement Technical Advisory Group (CQI TAG), where all EMS system participants will work collaboratively on system-wide issues and changes. EMS provider agencies shall not change the EMS data component of the designated County system without going through a collaborative county-wide process.
- H. EMS provider agencies will enter a data sharing agreement(s) with the EMS Agency and will only utilize the system once that agreement(s) has been finalized between the provider agency and the EMS Agency.
- I. EMS provider agencies will develop internal policies and procedures, provide training, and take other necessary steps to assure contract compliance and compliance with all data security and patient privacy laws, including Health Insurance Portability and Accountability Act, Health Information Technology for Economic and Clinical Health Act, and California Confidentiality of Medical Information Act.
- J. EMS provider agencies and hospitals will collaborate with the EMS Agency to institute and maintain a Health Data Exchange (HDE) program.

IV. EMS PROVIDER AGENCIES NOT UTILIZING COUNTY ePCR SYSTEM

- A. EMS provider agencies not using the designated County of Monterey EMS Data System and their vendors must maintain a data system that:
 - 1. Exports data to the County of Monterey's Data System in a format compliant with the EMS Data System, EMS Agency's guidelines and CEMSIS and NEMSIS standards. This transfer of data must be seamless and require no effort on the part of the EMS Agency to integrate the exported data.
 - 2. Includes all data elements required by the designated Monterey County EMS Data System.
 - 3. Attaches all pertinent records and documents to the ePCR shall be included in submissions. This includes, but is not limited to, electrocardiograms (ECG/EKGs), capnography waveforms, Physician Certification Statement (PCS) forms, face sheets, and PDF copies of the electronic patient care report.
 - 4. Contains provisions for the electronic transfer of patient care information between EMS providers and hospitals at the time of transfer of care or within timeframes specified in EMS System Policies and Procedures. All shared information must be compatible with the EMS Agency's Data System.
 - 5. Integrate real time Search, Alert, File and Reconcile capabilities consistent with the Monterey County EMS Data System.
 - 6. Bi-directionally moves data in real time into and from the designated Monterey County EMS Data System
 - 7. Provides bi-directional data movement and real time Search, Alert, File and Reconcile capabilities that is automatic and in real time and does not require any effort on the part of the EMS Agency to seamlessly integrate data with the rest of the Monterey County EMS Data System.

8. Continues seamless integration and operation with the designated Monterey County EMS Data System, even if material changes are made to the Monterey County EMS Data System.
- B. EMS Provider organizations utilizing their own system must notify the EMS Agency via the EMS Duty Officer of any system outages lasting over sixty minutes.
- C. EMS Provider organizations utilizing their own system must provide the EMS agency with a list of all data elements and field values currently active in their system. This documentation must show the relationship between data elements and field values in the provider's system and the EMS Agency's Data System
- D. EMS Provider organizations utilizing their own system are responsible for making any changes or updates to their system required to keep their system compatible with the EMS Agency's system and are responsible for the costs of those changes/updates. Non-users of the EMS Agency's system are also responsible for the costs to the County of processing, testing, and ensuring compatibility between the provider's system and the EMS Agency's system.
- E. EMS Provider organizations shall develop internal policies and procedures, provide training, and engage in all necessary steps to ensure compliance with all data security and patient privacy laws. This includes, but is not limited to, Health Insurance Portability and Accountability Act, Health Information Technology for Economic and Clinical Health Act, and California Confidentiality of Medical Information Act.
- F. EMS provider agencies will collaborate with the EMS Agency and hospitals to institute and maintain a HDE program.

V. HOSPITAL ROLES/RESPONSIBILITIES

- A. Hospitals shall be responsible for working with EMS providers and the EMS Agency to ensure the integration of prehospital and hospital patient records as specified in EMS System Policies and Procedures.
- B. Hospitals will provide outcome data to the EMS data system. At a minimum, that data will include Emergency Department and hospital discharge diagnosis information as well as other data points required by the EMS Agency.
- C. Hospitals shall participate in the CQI TAG and work collaboratively on system-wide issues.
- D. Hospital data submission shall be consistent with CEMSIS and NEMSIS hospital data elements.
- E. Hospitals will collaborate with the EMS Agency and EMS Provider organizations in the HDE as appropriate.
- F. Hospitals shall enter into a data sharing agreement with the County of Monterey EMS Agency and will only utilize the system once that agreement has been finalized.

EMS AGENCY ROLES/RESPONSIBILITIES

- A. The EMS Agency will coordinate the designated ePCR/Data System including implementation, training, and issue resolution. The Agency will work with the ePCR vendor on system coordination.
- B. The EMS Agency will pay for the initial implementation and ongoing costs related to the EMS Data system as specified in the agreement(s) between provider agencies and the EMS Agency.
- C. The EMS Agency will promulgate EMS System Policies and Procedures to prescribe use of the designated Monterey County EMS Data System.
- D. The EMS Agency will have access to provider-specific and system-wide data. Some operational data from EMS provider organizations, such as response times, may be shared as a part of the public record. Clinically oriented data may be shared in appropriate confidential quality improvement processes.
- E. The EMS Agency will analyze and report on system-wide data to EMS system stakeholders and other interested parties as appropriate and in concert with system-wide quality improvement efforts. The Agency may also work with individual provider agencies on provider specific quality improvement efforts as requested and appropriate.
- F. The EMS Agency will be responsible for the submission of data to the CEMSIS database. This will include the submission of Core Measures and any other data as required by the state.
- G. The EMS Agency will enter a data sharing agreement(s) with provider agencies and hospitals and will only utilize the system once that agreement(s) has been finalized between the Agency, provider agencies, and hospitals.
- H. The EMS Agency will work in good faith to resolve problems with individual provider agencies and hospitals as they arise and will strive for a collaborative relationship with all system stakeholders

VI. DISPATCH CENTERS USING COMPUTER-AIDED DISPATCH (CAD) ROLES/RESPONSIBILITIES

- A. Any dispatch center dispatching EMS providers in the County of Monterey must work with the EMS Agency, system providers, and vendor(s) to ensure CAD data can populate the appropriate fields of the ePCR and that this same data can be integrated into the EMS Agency's Data System.
- B. All data must be consistent with current CEMSIS and NEMSIS data standards, and with the EMS Agency's Data System.
- C. EMS Dispatch Centers shall participate in the CQI TAG to work collaboratively on system-wide issues.
- D. As appropriate and deemed necessary by the EMS Agency, Dispatch Centers shall enter a data sharing agreement with the EMS Agency.
- E. Dispatch Centers shall participate in system-wide QI efforts based on CAD and Medical Priority Dispatch System (MPDS) data.

VII. VENDOR RESPONSIBILITIES

- A. Vendor responsibilities will be described in detail in an agreement between the vendor and the County of Monterey. That agreement is a public document and will be shared with system stakeholders upon request.

VIII. TECHNICAL PROBLEMS/OUTAGES

- A. Device Failure – In the event of a device failure (e.g. iPad, tablet, laptop), provider agencies should contact their internal support person, document all pertinent PCR information on paper, and enter the ePCR information into the electronic system as soon as possible. Electronic device failure is not an exception for completing an ePCR.
- B. Connectivity Failure – In the event of connectivity failure, the provider shall attempt to find another appropriate means to submit ePCR data. In the event the provider cannot they shall document all patient information either on paper or on the device and save it until connectivity is restored. Patient information should be uploaded into the system as soon as possible. Local support personnel should be contacted for local issues.
- C. System failure – In the event of system-wide failure, document all patient care information on paper or a device for future submission. Patient information should be uploaded into the system as soon as the system is back up. The EMS Agency will work with the vendor to resolve system failure issues as quickly as possible and will communicate with system stakeholders when the system is back up.
- D. The EMS Agency shall be notified of downtime or transmission difficulties lasting more than 1 hour. The EMS Duty Officer shall be contacted for issues arising after normal business hours.
- E. Any system upgrades or system maintenance shall be reviewed and approved in writing by the EMS Agency prior to implementation. Any planned issue, such as system maintenance, that could cause a delay in data transmission shall be reported to the EMS Agency at least 24 hours in advance.

IX. PRIVACY AND PROTECTION OF HEALTH INFORMATION

- A. Maintaining confidentiality is an essential part of all health care, including prehospital care. The confidentiality of protected health information (PHI) and protected personal information (PPI) is covered by numerous state and federal statutes. These include: Health Insurance Portability and Accountability Act (HIPAA) of 1996; California Confidentiality of Medical Information Act (CCMIA) including California Civil Code Section 56.36, Division 109, Section 130200; and California Health and Safety Code Sections 1280.1, 1280.15, and 1280.3
 - 1. Providing ePCR information to receiving hospitals and other providers giving care in an EMS response system and to the EMS Agency for quality improvement efforts does not constitute a violation of the above regulations.
 - 2. Provider agencies and hospitals are responsible for training their employees on the initiation, completion, and distribution of patient care records, and on all applicable state and federal information privacy statutes, regulations, and policies and procedures. This is a priority as the unintended (or intentional) release of

protected information can have serious consequences for the provider agency or hospital and may violate HIPPA and other applicable laws. EMS and provider agency and facility staff must protect the security, confidentiality, and privacy of all patient medical records in their custody at all times.

3. Detailed requirements for the sharing of data within the EMS system and for the protection of personal health information is contained in the agreements between individual provider agencies, hospitals, and the county.
4. The above referenced statutes and regulations also allow for the sharing of information in HIEs/HDEs. All the entities involved in an HIE/HDE are responsible for maintaining patient confidentiality. That includes the vendor of the various electronic patient care reporting systems and the HIE/HDE entity itself.
5. The EMS Agency will facilitate and coordinate, as appropriate and deemed necessary, training opportunities on protecting confidential patient care information.
6. All patient records containing protected health and personal information must be kept secure at all times. These records must not be visible to the public. This includes appropriate security measures, such as user identification and login passwords, for computer workstations. Workstations must be locked when unattended.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 7020

Effective Date:

Review Date:

NALOXONE LEAVE BEHIND PROGRAM

I. PURPOSE

- A. To establish a policy to allow EMS personnel to provide an intranasal naloxone delivery device to patients who are at high risk for fatal opioid overdose.
- B. To help mitigate the impact of the opioid crisis by increasing the availability of naloxone to the public.

II. POLICY

A Naloxone Leave Behind Kit may be offered to an at-risk person, friend, or family member in the event of a suspected overdose following treatment of the patient. Kits may be provided to patients or bystanders regardless of whether the patient is transported to the hospital.

III. PROCEDURE

- A. EMS units may be stocked with naloxone intranasal delivery devices intended for layperson utilization in the event of an opioid overdose. EMS shall not distribute naloxone to patients or bystanders from the regular EMS patient care supply.
- B. Administration of naloxone by EMS providers at the scene of an incident will be performed in accordance with existing County of Monterey EMS protocols.
- C. Patients at-risk for opioid overdose may include:
 - 1. Opioid overdose requiring naloxone administration or supportive care and monitoring.
 - 2. History or physical exam with evidence of illicit substance use or paraphernalia (e.g. history of intravenous drug use, needle marks, abscesses at injection sites, needles present in belongings).
 - 3. History or physical exam with prescription opioid use (prescribed or recreational).
 - 4. Physical environment with multiple or high-dose prescription opioids present.
 - 5. Self-reported dependence on opioids.
- D. A Naloxone Leave Behind Kit may be distributed on each call where there is a primary impression of overdose or reasonable provider suspicion of opioid abuse.
- E. If a Naloxone Leave Behind Kit is left with the patient or other responsible adult, EMS personnel will instruct the recipient on the proper indications and technique for usage of the device and shall leave an instructional pamphlet.

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- F. EMS personnel shall document in the patient care report (PCR) that a Naloxone Leave Behind Kit was distributed. If a Kit is left with a friend or family member and not the patient, EMS personnel do not need to create an additional PCR as it is related to the emergency scene of the initial response. In addition to the standard requirements, documentation shall include:
1. A description of the instructions given to the recipient.
 2. The administration of naloxone in the Medication section of the Flow Chart with “Other/miscellaneous” as the Route, the Lot Number of the Kit, and “Leave Behind” in the Comments.
- G. EMS personnel may, at their discretion, leave a Naloxone Leave Behind Kit with other individuals whom EMS personnel deem to be at risk for unintentional opioid overdose (e.g., patients whose medication regimen includes high doses of narcotics or at-risk family and bystanders), even if the call to which EMS personnel are responding does not specifically involve an opioid overdose. In such cases, EMS personnel will document on the initial patient’s PCR, that a Kit was provided to a separate individual, and include the naloxone lot number. No demographic information pertaining to the individual shall be recorded on the patient’s PCR. The EMS personnel will instruct the recipient or a responsible adult on the proper indications and technique for usage of the device and leave an instructional pamphlet.

END OF POLICY

County of Monterey EMS System Policy



Policy Number: 8080

Effective Date:

Review Date:

COUNT OF MONTEREY MHOAC NOTIFICATION/ACTIVATION

I. PURPOSE

To provide guidance to the EMS Duty Officer, Medical Health Operational Area Coordinator (MHOAC) program, and operational lead and/or supporting agencies responsible for ensuring that medical and health preparedness and response activities are completed. Within the Operational Area (OA) various departments and programs may be responsible for one or more of the 17 MHOAC program functions outlined in Health and Safety Code §1797.153.

II. POLICY

The Board of Supervisors has designated the EMS Director as the County of Monterey MHOAC. The MHOAC program will be facilitated through a collaborative effort between the County of Monterey EMS Agency, the County of Monterey Health Officer, and the County of Monterey Health Department.

III. PROCEDURE

A. Notification

1. Any agency that is impacted by a trigger event or situation (as outlined below) shall contact the EMS Duty Officer by calling the County of Monterey EMS Communications Center at **831-796-6444**.
2. The County of Monterey EMS Communications Center will contact the EMS Duty Officer, based upon internal notification policies.
3. If the County of Monterey EMS Communications Center becomes aware of or identifies an event or situation that meets the notification/activation triggers they shall notify the EMS Duty Officer.
4. The EMS Duty Officer will notify the MHOAC, as appropriate.

B. Triggers for Notification of the MHOAC

Any of the following conditions may trigger the notification of the MHOAC:

1. An incident that significantly impacts or is anticipated to significantly impact public health, the medical/health system, behavioral health, environmental health, or emergency medical services;
2. An incident where resources are needed or are anticipated to be needed beyond the capabilities of the OA, including those resources available through existing

agreements;

3. An incident that leads to a regional or state request for information or mutual aid; and/or
4. An incident in which increased information flow from the OA to the region and the state will assist in the management or mitigation of the incident's impact.

C. Role of the EMS Duty Officer

1. Day to day EMS system operational issues will be managed by the EMS Duty Officer.
2. If the situation/event meets a notification/activation trigger as described above, the EMS Duty Officer shall contact the MHOAC.

D. Role of the MHOAC

1. Identify resources and coordinate the procurement and allocation of public and private medical, health and other resources required to support disaster medical and health operations in affected areas.
2. Request and respond to situation status reporting and resource requests generated by OA hospitals, Health Care Facilities and medical care entities and providers.
3. Communicate the medical and health status and needs to local, regional, and state governmental agencies and officials inside and outside of the OA.
4. Coordinate with the Regional Disaster Medical Coordinator/Specialist (RDMHC/S) program for medical and/or health mutual aid support from outside the OA as needed.
5. Coordinate with OA Supporting Agencies, and with regional and state entities as appropriate.
6. Ensure the completion and submission of a Situation Status Report inclusive of all Medical and Health awareness information within the OA as per guidance in the California Public Health and Medical Emergency Operations Manual.
7. Coordination and support of the specific MHOAC Program functions with the Operational Area Lead and/or Support Agencies.

E. The Operational Area Lead and/or Support Agency is responsible for:

1. Coordination of notifications within their respective agency, with the MHOAC, other OA agencies and with regional and state entities.
2. Ensuring situational awareness information is shared with the MHOAC for inclusion in any notifications and/or situation status reports.
3. Coordination and support of the specific MHOAC Program functions in which they are participating.

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F. MHOAC Program Function Matrix

MHOAC Program Response Function	Operational Area Lead	Operational Support Agency
1. Assessment of immediate medical needs	EMS	PH
2. Coordination of disaster medical and health resources	MHOAC	HO, PH, and BH
3. Coordination of patient distribution and medical evaluations	EMS	PH
4. Coordination with inpatient and emergency care providers	EMS	PH
5. Coordination of out-of-hospital medical care providers	MHOAC	EMS and PH (Outpatient Clinics, Skilled Nursing, Long Term Care, Dialysis Centers, Urgent Care and Surgery Centers)
6. Coordination and integration with fire agencies personnel, resources, and emergency fire prehospital medical services	EMS	OES
7. Coordination of providers of non-fire based prehospital emergency medical services	EMS	OES
8. Coordination of the establishment of temporary field treatment sites	EMS	PH and OES
9. Health surveillance and epidemiological analyses of community health status	CDU	PH

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MHOAC Program Response Function	Operational Area Lead	Operational Support Agency
10. Assurance of food safety	EH	HO
11. Management of exposure to hazardous agents	EH	HO/Regional HazMat Team
12. Provision or coordination of mental health services	BH	PH
13. Provision of medical and health public information protective action recommendations	HO and PH PIO	HO and OES PIO
14. Provision or coordination of vector control services	EH	HO / PH
15. Assurance of drinking water safety	EH	HO / PH
16. Assurance of the safe management of liquid, solid and hazardous wastes	EH	PH
17. Investigation and control of communicable disease	CDU	PH

Matrix Key:

AG	Agriculture Commissioner
BH	Behavioral Health
CDU	Communicable Disease Unit
EH	Environmental Health
EMS	EMS Agency
HO	Health Officer
MHOAC	Medical Health Operational Area Coordinator
OES	Office of Emergency Services
PH	Public Health
PIO	Public Information Officer (Health Department)

G. Sustaining the MHOAC program in a long-term event

The MHOAC Program will be sustained through a collaborative effort among the County of Monterey EMS Agency, the County of Monterey Health Officer, and the County of Monterey Public Health Bureau for the duration of the event and through the recovery process. For sustained events requiring substantial expansion of the ICS/MAC structure, mutual aid may be necessary.

1. The Public Health Bureau and EMS Agency will ensure that they can adequately supply qualified staff to perform as the MHOAC designee.
2. The Public Health Bureau and EMS Agency will create a MHOAC staffing schedule to be utilized in the event of a long-term event, to include the activation of the DOC or the Operational Area EOC in order to fill the Medical/ Health Branch.

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3. The Public Health Bureau and EMS Agency will identify the Lead Agency referencing the MHOAC Program Function Matrix
- H. Triggers to sustain the MHOAC program in a long-term event include:
1. Event lasting longer than 48 hours; or
 2. Operational Area EOC activation; or
 3. Public Health Officer directive; or
 4. Activation of Public Health DOC

END OF POLICY